Answer Key & Rationales · Form 8

Quick key. **P** = pretest (unscored; do not count). Bank 2026.10.22-0834.

#	Key	Area	Sub-area	P
1	**B**	II	II.A	
2	**C**	III	III.B	
3	**A**	III	III.B	
4	**A**	II	II.A	
5	**A**	I	I.B	
6	**C**	II	II.A	
7	**B**	I	I.B	
8	**A**	II	II.A	
9	**B**	I	I.A	
10	**A**	III	III.B	**P**
11	**A**	II	II.C	
12	**C**	I	I.B	
13	**A**	II	II.B	
14	**A**	I	I.A	
15	**D**	I	I.B	**P**
16	**C**	III	III.B	**P**
17	**B**	I	I.C	
18	**A**	III	III.B	
19	**B**	I	I.C	
20	**B**	I	I.C	
21	**C**	III	III.D	
22	**B**	I	I.C	
23	**C**	I	I.C	
24	**B**	I	I.A	
25	**B**	III	III.A	
26	**D**	I	I.B	
27	**B**	III	III.A	
28	**C**	II	II.B	
29	**B**	III	III.C	
30	**B**	II	II.B	**P**
31	**C**	II	II.B	
32	**C**	III	III.B	
33	**D**	I	I.B	
34	**A**	II	II.C	
35	**B**	III	III.B	
36	**A**	III	III.B	
37	**B**	I	I.B	
38	**A**	I	I.A	
39	**A**	I	I.B	
40	**D**	I	I.A	**P**
41	**C**	II	II.B	
42	**A**	III	III.B	
43	**A**	I	I.B	
44	**B**	II	II.B	
45	**A**	II	II.A	
46	**B**	I	I.A	
47	**B**	II	II.A	
48	**A**	II	II.A	
49	**A**	II	II.B	**P**
50	**A**	II	II.C	
51	**C**	III	III.A	
52	**B**	III	III.B	
53	**C**	III	III.D	
54	**C**	III	III.B	
55	**D**	III	III.C	
56	**C**	II	II.A	
57	**B**	II	II.B	
58	**D**	II	II.A	
59	**C**	III	III.B	
60	**C**	I	I.A	
61	**C**	I	I.B	
62	**A**	II	II.B	
63	**D**	III	III.D	
64	**A**	III	III.B	
65	**B**	I	I.B	
66	**C**	I	I.A	
67	**A**	I	I.A	**P**
68	**A**	III	III.B	
69	**C**	III	III.B	
70	**A**	III	III.A	
71	**B**	II	II.B	
72	**D**	I	I.B	
73	**B**	III	III.B	**P**
74	**A**	III	III.C	
75	**B**	III	III.B	
76	**C**	I	I.A	
77	**A**	I	I.B	
78	**A**	II	II.C	
79	**C**	III	III.C	
80	**B**	II	II.A	
81	**C**	I	I.B	
82	**D**	I	I.A	
83	**B**	II	II.B	
84	**D**	III	III.B	
85	**C**	III	III.B	
86	**D**	I	I.A	
87	**D**	I	I.C	
88	**C**	I	I.B	
89	**A**	III	III.C	
90	**C**	II	II.A	
91	**A**	III	III.B	
92	**A**	I	I.B	
93	**A**	III	III.A	
94	**B**	III	III.B	**P**
95	**A**	II	II.A	
96	**B**	II	II.B	
97	**C**	I	I.C	
98	**B**	III	III.B	
99	**D**	II	II.B	**P**
100	**A**	II	II.A	
101	**B**	II	II.A	
102	**C**	I	I.C	
103	**A**	I	I.A	
104	**B**	II	II.A	
105	**C**	I	I.C	
106	**D**	II	II.A	
107	**C**	II	II.B	
108	**C**	III	III.B	
109	**B**	II	II.A	
110	**B**	II	II.A	
111	**A**	III	III.B	
112	**C**	I	I.C	
113	**D**	I	I.B	
114	**B**	II	II.C	
115	**A**	I	I.B	**P**
116	**A**	III	III.A	
117	**A**	II	II.B	**P**
118	**D**	I	I.C	
119	**C**	III	III.D	
120	**B**	I	I.B	
121	**C**	II	II.B	
122	**B**	I	I.A	

Scoring worksheet

Count correct answers on **scored items only**. Item numbers for each sub-area are listed; pretest items are left out.

Area / sub-area	Scored item numbers	Items	Correct
I. Values and Ethics		**40**	**___ / 40**
I.A Ethical Principles and Responsibilities	9, 14, 24, 38, 46, 60, 66, 76, 82, 86, 103, 122	12	____
I.B Ethical Service Delivery	5, 7, 12, 26, 33, 37, 39, 43, 61, 65, 72, 77, 81, 88, 92, 113, 120	17	____
I.C Diversity and Social Justice	17, 19, 20, 22, 23, 87, 97, 102, 105, 112, 118	11	____
II. Assessment and Planning		**35**	**___ / 35**
II.A Assessment Concepts	1, 4, 6, 8, 45, 47, 48, 56, 58, 80, 90, 95, 100, 101, 104, 106, 109, 110	18	____
II.B Assessment Methods and Techniques	13, 28, 31, 41, 44, 57, 62, 71, 83, 96, 107, 121	12	____
II.C Assessment Practices	11, 34, 50, 78, 114	5	____
III. Intervention and Practice		**35**	**___ / 35**
III.A Practice Concepts	25, 27, 51, 70, 93, 116	6	____
III.B Intervention Methods and Techniques	2, 3, 18, 32, 35, 36, 42, 52, 54, 59, 64, 68, 69, 75, 84, 85, 91, 98, 108, 111	20	____
III.C Practice Evaluation and Research	29, 55, 74, 79, 89	5	____
III.D Supervision and Administration	21, 53, 63, 119	4	____

Pretest items (skip when scoring): 10, 15, 16, 30, 40, 49, 67, 73, 94, 99, 115, 117

Total correct (scored items)	_____ / 110
Practice pass line	77 / 110 (70%)
Correct needed to reach the line	77 − your total = _____

The real exam sets its passing score separately for each form (it is reported as a scaled score). 77 of 110 is a deliberately cautious practice target that builds a margin; it is not a prediction of your real result.

Score conversion (raw out of 110 → percent)

The boxed row is the practice pass line: **77 / 110 = 70%**. Any score at or above it meets the line.

Raw	%	Raw	%	Raw	%	Raw	%	Raw	%	Raw	%
110	100%	91	83%	72	65%	53	48%	34	31%	15	14%
109	99%	90	82%	71	65%	52	47%	33	30%	14	13%
108	98%	89	81%	70	64%	51	46%	32	29%	13	12%
107	97%	88	80%	69	63%	50	45%	31	28%	12	11%
106	96%	87	79%	68	62%	49	45%	30	27%	11	10%
105	95%	86	78%	67	61%	48	44%	29	26%	10	9%
104	95%	85	77%	66	60%	47	43%	28	25%	9	8%
103	94%	84	76%	65	59%	46	42%	27	25%	8	7%
102	93%	83	75%	64	58%	45	41%	26	24%	7	6%
101	92%	82	75%	63	57%	44	40%	25	23%	6	5%
100	91%	81	74%	62	56%	43	39%	24	22%	5	5%
99	90%	80	73%	61	55%	42	38%	23	21%	4	4%
98	89%	79	72%	60	55%	41	37%	22	20%	3	3%
97	88%	78	71%	59	54%	40	36%	21	19%	2	2%
96	87%	**77**	**70%**	58	53%	39	35%	20	18%	1	1%
95	86%	76	69%	57	52%	38	35%	19	17%	0	0%
94	85%	75	68%	56	51%	37	34%	18	16%		
93	85%	74	67%	55	50%	36	33%	17	15%		
92	84%	73	66%	54	49%	35	32%	16	15%		

Per-area practice guide: aim for at least 70% in each area as well as overall (Area I 28 of 40; Areas II and III 25 of 35 each).

Rationales for every item

Section 1

1. ASM-1633 (rev 1) · Best answer: B

A 74-year-old widowed client plans to remarry. The client's adult children request a family session. They say the new partner is replacing their late parent, and they worry about the family home and their inheritance. The client listens, then says the decision is made but that the client wants the children's blessing. Which response by the social worker is **BEST**?

A. Suggest the client delay the wedding until the children adjust
B. Help the children tell the client their worries directly
C. Request an evaluation of the client's decision-making capacity

Why each option is right or wrong

A **Incorrect** (trap: Overrides the client's choice). Postponing might ease the children's distress, but the client has made the decision and shows no wish to wait. Steering a competent older adult to delay a personal choice for others' comfort undercuts self-determination.

B **Correct.** Later-life transitions such as widowhood and repartnering require the whole family to reorganize, and they can stir loyalty to the late parent and worries about inheritance. The client has decided and wants the children's blessing, so helping the children put their fears into words with the client serves both the family bond and the client's right to choose. It beats A because asking a competent adult to postpone a personal decision to soothe others places the children's wishes above the client's self-determination (McGoldrick et al., 2016; NASW Code 1.02).

C **Incorrect** (trap: Skips assessment). A capacity evaluation fits when there are signs of impaired understanding or reasoning about a decision. Nothing in the client's statements suggests impairment, and family disagreement alone is not a reason to question an older adult's capacity.

Takeaway: Later-life repartnering can stir loyalty and inheritance fears; help the family voice them while respecting the older adult's choice.

Sources: McGoldrick, M., Garcia Preto, N., & Carter, B. (2016). The Expanding Family Life Cycle: Individual, Family, and Social Perspectives (5th ed.). Pearson, families in later life; NASW Code of Ethics (2021), 1.02
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.18 Aging's impact on individuals and family systems · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

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2. INT-1108 (rev 1) · Best answer: C

In Carl Rogers' person-centered approach, which term describes a helper whose outward responses fit what the helper is actually experiencing?

A. Unconditional positive regard
B. Accurate empathy
C. Congruence
D. Immediacy

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Unconditional positive regard is warm acceptance of the client as a person, without conditions of worth. It describes the helper's attitude toward the client, not whether the helper's expression fits their own inner experience.

B **Incorrect** (trap: Wrong model or concept). Accurate empathy is the helper's ability to sense the client's inner world and communicate that understanding. It centers on understanding the client, while the stem describes the helper being genuine about their own experience.

C **Correct.** Congruence, also called genuineness, is the helper's state of being integrated and authentic, so that what the helper communicates fits what the helper actually feels and thinks. Rogers (1957) named it one of the core conditions for therapeutic change, alongside unconditional positive regard and empathic understanding. It beats D because immediacy is a specific skill of discussing what is happening between client and worker in the moment; congruence is the broader quality of being genuine in every response.

D **Incorrect** (trap: Wrong model or concept). Immediacy is a skill of discussing the here-and-now relationship between client and worker, such as tension in the session. It draws on genuineness, but it is a specific technique, not one of Rogers' core conditions.

Takeaway: Rogers' core conditions are congruence (genuineness), unconditional positive regard and empathic understanding, as perceived by the client.

Sources: Rogers, C. R. (1957). The necessary and sufficient conditions of therapeutic personality change. Journal of Consulting Psychology, 21(2), 95-103, core conditions: congruence
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.2 Verbal and nonverbal communication; congruence, body language · **Qualifier** none · **Cognitive** recall · **Difficulty** 1 of 3

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3. INT-1652 (rev 1) · Best answer: A

A client with schizoaffective disorder receives a lease warning after neighbors complain of loud talking at night. The client links it to voices that worsened during a medication change, which the prescriber is now adjusting. The client wants to keep the apartment and agrees to let the social worker help. What is the **MOST** appropriate way for the social worker to assist?

A. Help the client ask the landlord for a reasonable accommodation
B. Help the client look for a unit with fewer shared walls
C. Help the client file a fair housing complaint right away
D. Send the landlord a letter describing the client's diagnosis

Why each option is right or wrong

A **Correct.** The Fair Housing Act makes it unlawful to refuse reasonable accommodations in rules, policies or practices when a person with a disability needs them to use and enjoy a home (42 U.S.C. 3604(f)(3)(B); 24 CFR 100.204). The noise is tied to symptoms that treatment is now addressing, and the client wants to stay, so a written request, such as time for the medication adjustment before the lease is enforced, uses the client's right through the proper first channel (Hepworth et al.). It beats C because a complaint fits after a request is refused, and no request has been made yet.

B **Incorrect** (trap: Good but not best). Moving might end the complaints, but the client wants to keep this apartment and has a legal route to do so. Starting with a move gives up an entitlement before using it. A move becomes an option if the client later prefers it.

C **Incorrect** (trap: Skips assessment). A fair housing complaint is the right step if the landlord refuses a reasonable request. The landlord has not yet been asked, so filing now skips the direct request that may settle the matter and that a complaint would usually build on.

D **Incorrect** (trap: Misreads law or ethics). A request can describe the disability-related need and the change sought without naming a diagnosis or treatment details. Sending the diagnosis shares more than the purpose requires; disclosure should be limited to what is needed (NASW Code 1.07(c)).

Takeaway: When disability symptoms threaten housing, help the client ask for a reasonable accommodation before moving or filing a complaint.

Sources: Fair Housing Act, 42 U.S.C. 3604(f)(3)(B); Fair Housing Act regulations, 24 CFR 100.204; Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, Advocacy and social action: case and cause advocacy; NASW Code of Ethics (2021), 1.07(c)

Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.13 Advocating for needed services and resources · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 2 of 3

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4. ASM-1551 (rev 2) · Best answer: A

A community mental health client with depression has missed 3 of 5 appointments, and clinic policy allows closure after 3 missed visits. When reached, the client explains that their hourly job posts shifts only 2 days ahead and their phone is often shut off for unpaid bills, so reminder calls fail. The client says they want to keep coming. What is the **MOST** appropriate response?

A. Offer short-notice scheduling and a workable contact method
B. Explore the client's ambivalence about continuing treatment
C. Close the case and invite the client to reapply later

Why each option is right or wrong

A **Correct.** Poverty shapes service use: unpredictable hourly shifts and unstable phone service are economic barriers, not a lack of motivation (ODPHP, Healthy People 2030). Because the client wants care, the agency adapts how it delivers services. Short-notice slots fit a schedule posted 2 days ahead, and an agreed contact method replaces reminder calls that never arrive. It beats B because nothing in the client's account suggests mixed feelings about therapy; exploring ambivalence would treat a structural barrier as a motivational one.

B **Incorrect** (trap: Good but not best). Missed visits can signal ambivalence, and exploring it fits when a client seems unsure about therapy. This client named concrete scheduling and phone barriers and wants to continue, so the problem is access rather than motivation.

C **Incorrect** (trap: Hands off too soon). Closing the case penalizes the client for the effects of low-wage, unstable work. Closure could follow if the client chose to stop or barriers remained after real accommodations, but no adjustment has been tried yet.

Takeaway: When missed appointments stem from poverty-related barriers, adapt how services are delivered before treating attendance as a motivation problem.

Sources: Office of Disease Prevention and Health Promotion (ODPHP) (2020). Healthy People 2030: Social Determinants of Health, SDOH domain: economic stability
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.9 Poverty's impact; social determinants of health · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 1 of 3

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5. ETH-1163 (rev 2) · Best answer: A

A social worker in private practice plans to raise session fees by 20%. One client, who pays out of pocket and lives on a fixed income, is midway through a planned course of trauma treatment. How should the social worker **BEST** handle the change with this client?

A. Give early notice and discuss a reduced fee if needed
B. Announce the new fee to clients with 30 days' notice
C. Refer the client to a lower-cost clinic before the increase

Why each option is right or wrong

A **Correct.** Fees should be fair, reasonable and in line with the services, with attention to the client's ability to pay (NASW Code 1.13(a)), and clients should understand costs as part of informed consent (1.03(a)). Giving notice well ahead and discussing options, such as keeping the current rate or a sliding fee, protects a client on a fixed income who is midway through trauma treatment. It beats B because notice alone treats fairness as identical fees for everyone, while the Code ties fees to the client's ability to pay.

B **Incorrect** (trap: Good but not best). Advance notice and the same rule for every client sound fair, which makes this tempting. Applying the full increase without discussing options ignores this client's ability to pay, which the Code asks social workers to consider, and risks ending trauma work midway.

C **Incorrect** (trap: Hands off too soon). Referral can be right if the client cannot afford care even with adjustments. As a first move, it ends a trauma course midway, when notice and a reduced fee could keep the work intact.

Takeaway: Before raising fees, give advance notice and weigh each client's ability to pay, offering adjustments before referral.

Sources: NASW Code of Ethics (2021), 1.13(a); NASW Code of Ethics (2021), 1.03(a)
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.5 Ethical billing: managed care, insurance, incident-to billing · **Qualifier** BEST · **Cognitive** application · **Difficulty** 2 of 3

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6. ASM-0104 (rev 1) · Best answer: C

At a community mental health agency, a client has had major depressive disorder since adolescence and began drinking heavily 2 years ago. The client now meets criteria for moderate alcohol use disorder and denies suicidal thoughts. An intake staff member suggests the client get sober before starting mental health care. What is the **BEST** treatment approach?

A. Address the drinking first, then reassess the depression
B. Refer to separate agencies for each disorder at once
C. Treat both disorders together in one coordinated plan

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Requiring sobriety before mental health care is the sequential model. A period of abstinence can help clarify a diagnosis when depression may be alcohol-induced, but this client's depression predates the drinking by years. Waiting leaves an independent disorder untreated.

B **Incorrect** (trap: Good but not best). Parallel treatment addresses both disorders, but in separate systems that may not coordinate. Clients can receive conflicting messages and drop out between programs. It is better than no care, but it falls short of integrated treatment.

C **Correct.** Integrated treatment, which addresses both disorders at the same time through one team or coordinated plan, is the preferred model for co-occurring disorders (SAMHSA TIP 42, Ch. 2). Here the depression began years before the drinking, so it is an independent disorder rather than a result of alcohol use, and delaying its care risks worsening both. It beats B because parallel care at separate agencies splits the plan, sends mixed messages and often loses clients between systems.

Takeaway: For co-occurring mental health and substance use disorders, integrated treatment of both at once is preferred over sequential or parallel care.

Sources: SAMHSA TIP 42 (2020), Ch. 2: Guiding Principles for Working With People Who Have Co-Occurring Disorders (Achieving Integrated Treatment); DSM-5-TR (APA, 2022), Depressive Disorders: Substance/Medication-Induced Depressive Disorder
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.6 Comorbid (co-occurring) disorders and conditions · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

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7. ETH-0111 (rev 1) · Best answer: B

A social worker supervises a staff member who is working toward clinical licensure. The supervisee invites the supervisor to invest in a weekend wellness business the supervisee is starting. The supervisor will submit an evaluation of the supervisee's hours to the licensing board next month. What should the supervisor do **FIRST**?

A. Invest a small amount after submitting the evaluation
B. Decline and explain the conflict with the supervisory role
C. Record the invitation in the supervision notes

Why each option is right or wrong

- **A** **Incorrect** (trap: Boundary slip or collusion). Waiting until after the evaluation does not remove the conflict. The supervisor's power and influence over the supervisee's career continue, and a business tie would compromise future supervision and references.
- **B** **Correct.** Supervisors must avoid dual relationships with supervisees that risk exploitation or harm (NASW Code 3.01(c)). A shared business investment would tie the supervisor's money to the supervisee's success while the supervisor holds evaluative power, including a licensing evaluation next month. Declining clearly and explaining why models the boundary the supervisee will need with clients (NASW & ASWB, 2013). It beats C because documenting the request is good practice, but it comes after the supervisor has responded; recording it first leaves the invitation open.
- **C** **Incorrect** (trap: Right action, wrong time). Documenting the invitation and the response belongs in the supervision record. Doing it before responding delays the clear answer the supervisee needs and leaves the boundary question unsettled.

Takeaway: Supervisors hold evaluative power, so they decline business or social ties with supervisees and explain the boundary directly.

Sources: NASW Code of Ethics (2021), 3.01(c); NASW & ASWB Best Practice Standards in Social Work Supervision (2013), Standard 4 (Ethical Issues): Boundaries
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.7 Ethics of supervising and managing · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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8. ASM-1044 (rev 1) · Best answer: A

A client with bipolar I disorder, stable for 2 years, meets the social worker by video. The client reports sleeping 4 hours a night for 3 nights, feeling 'better than ever' and planning a new business. The relapse plan the client wrote while well lists short sleep as a warning sign that means calling the prescriber. The client now wants to wait for next month's appointment. What should the social worker do **FIRST**?

- **A.** Help the client contact their medication provider today
- **B.** Explore the client's concerns about medication changes
- **C.** Agree to track sleep together until the next appointment

Why each option is right or wrong

- **A** **Correct.** Less sleep with rising mood is a common early warning sign of manic relapse, and teaching clients to act on early signs through a prearranged plan reduces manic relapses (Perry et al., 1999). The client wrote this plan while well, so helping the client follow it supports, rather than overrides, self-determination (NASW Code 1.02). It beats B because worries about medication deserve attention, but exploring them first delays a time-sensitive step; they can be discussed while the call is made.
- **B** **Incorrect** (trap: Right action, wrong time). Ambivalence about medication is common and worth exploring. With 3 nights of short sleep and rising mood, exploration comes after the client reaches the provider, since early warning signs can escalate within days.
- **C** **Incorrect** (trap: Ignores risk). Tracking sleep is useful alongside treatment, but waiting a month ignores the warning sign the client's own plan names. Early manic symptoms can progress quickly.

Takeaway: Early warning signs named in a client's relapse plan call for prompt contact with the treating provider, honoring choices the client made while well.

Sources: Perry, A., Tarrier, N., Morriss, R., McCarthy, E., & Limb, K. (1999). Randomised controlled trial of efficacy of teaching patients with bipolar disorder to identify early symptoms of relapse and obtain treatment. BMJ, 318(7177), 149-153, whole article; NASW Code of Ethics (2021), 1.02
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.5 Signs of mental and emotional illness across the lifespan · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 2 of 3

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9. ETH-1133 (rev 2) · Best answer: B

A hospital social work department receives a disability insurer's request for a former patient's psychosocial assessment. The attached release is signed and dated by the patient, but the lines for the purpose of the disclosure and the expiration date are blank. What should the social worker do **FIRST**?

A. Send the assessment, since the patient signed the release
B. Contact the patient to complete a valid authorization
C. Send a brief summary instead of the full assessment
D. Tell the insurer that the records require a subpoena

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). Sending the assessment becomes appropriate once the patient completes a valid authorization. With the purpose and expiration blank, the form is defective, so releasing records now is not yet authorized.

B **Correct.** HIPAA treats an authorization as defective when a required core element, such as the purpose or an expiration date or event, is left blank (45 CFR 164.508(b)(2), 164.508(c)(1)). A defective form is not valid consent (NASW Code 1.07(b)). Contacting the patient lets them decide what to release, to whom and for how long. B beats A because a signature alone does not make an incomplete form valid; sending now would be an unauthorized disclosure.

C **Incorrect** (trap: Misuses the technique). Sending less is the right instinct about minimum disclosure, but it does not cure the problem. Any disclosure, even a summary, needs a valid authorization, and this one is missing required elements.

D **Incorrect** (trap: Misreads law or ethics). Insisting on a subpoena misreads the rules: a valid authorization from the patient is enough for this disclosure. Telling the insurer otherwise delays a claim the patient likely wants processed.

Takeaway: A signed release missing required elements is not valid; have the client complete it before disclosing anything.

Sources: HIPAA Privacy Rule, 45 CFR 164.508(b)(2); HIPAA Privacy Rule, 45 CFR 164.508(c)(1); NASW Code of Ethics (2021), 1.07(b)
Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.1 Confidentiality law and ethics, its limits, HIPAA/PHI privacy · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 1 of 3

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10. INT-1041 (rev 1) · Best answer: A

PRETEST — unscored on this form; do not count it

Parents of a 16-year-old report that the teen has stopped doing homework. Every evening they remind, lecture, and check the teen's work, and each week the teen does less. Which intervention is **MOST** consistent with the Mental Research Institute (MRI) brief therapy model?

A. Help the parents stop the nightly reminders and try something new
B. Build a three-generation genogram of the family's school achievement
C. Have the parents and teen enact a homework argument in session
D. Ask the teen when homework gets done without any reminders

Why each option is right or wrong

A **Correct.** The MRI model holds that problems persist because of the attempted solution: the more the family does what isn't working, the more the problem grows (Watzlawick, Weakland & Fisch, 1974). Nightly reminding, lecturing, and checking are followed by less homework, so the parents' solution has become part of the problem. Interrupting it and trying a different response is the MRI move. It beats D because asking about exceptions is a solution-focused technique; it looks for times the problem is absent rather than changing the sequence that keeps it going.

B **Incorrect** (trap: Wrong model or concept). A multigenerational genogram is a Bowen tool for tracing patterns across generations. MRI brief therapy focuses on the present interaction that keeps the problem going, not on family history.

C **Incorrect** (trap: Wrong model or concept). Enactment belongs to structural family therapy, which uses in-session interaction to restructure hierarchy and boundaries. MRI work targets the attempted solution itself rather than staging the conflict.

D **Incorrect** (trap: Wrong model or concept). Exception questions come from solution-focused brief therapy, which grew partly out of MRI work. They search for times the problem is absent instead of interrupting the parents' attempted solution.

Takeaway: MRI brief therapy targets the attempted solution: when what the family keeps doing maintains the problem, interrupt it and try something different.

Sources: Watzlawick, P., Weakland, J., & Fisch, R. (1974). Change: Principles of Problem Formation and Problem Resolution. W. W. Norton, attempted solutions; more of the same
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.21 Family therapy models and interventions · **Qualifier** MOST · **Cognitive** application · **Difficulty** 2 of 3

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11. ASM-1069 (rev 2) · Best answer: A

An adult with schizophrenia who has been stable on clozapine moves into a smoke-free group residence and stops smoking 1.5 packs a day all at once. Five days later, staff report that the client is much sleepier, drooling more and walking unsteadily. What is the **MOST** important action for the social worker?

A. Tell the prescriber today that the client quit tobacco
B. Arrange nicotine replacement to ease the client's withdrawal
C. Assess for withdrawal symptoms since the client quit smoking

Why each option is right or wrong

A **Correct.** Tobacco smoke speeds the breakdown of clozapine by inducing the liver enzyme CYP1A2, so stopping smoking suddenly can raise clozapine blood levels sharply within days. Increased sleepiness, drooling and unsteadiness after quitting suggest rising levels, which can progress to seizures. The prescriber needs to know today to check levels and adjust the dose. It beats B because nicotine replacement eases withdrawal but does not affect the enzyme, so it would not lower the clozapine level (Lucas & Martin, 2013).

B **Incorrect** (trap: Good but not best). Nicotine replacement is reasonable support for quitting, but nicotine itself does not induce the enzyme that smoke does. It would not correct a rising clozapine level, the likely cause of these signs.

C **Incorrect** (trap: Near-miss diagnosis). Nicotine withdrawal causes irritability, restlessness, poor sleep and craving, not heavy sleepiness, drooling and unsteady gait. Assessing withdrawal alone delays telling the prescriber about a likely drug-level change.

Takeaway: Abruptly stopping smoking can raise clozapine levels; report quitting and new sedation to the prescriber promptly.

Sources: Lucas, C., & Martin, J. (2013). Smoking and drug interactions. Australian Prescriber, 36(3), 102-104, Antipsychotics: clozapine

Area II Assessment and Planning · **Sub-area** II.C Assessment Practices · **Topic** II.C.5 Prescription, OTC, alternative medications: uses and side effects · **Qualifier** MOST · **Cognitive** application · **Difficulty** 3 of 3

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12. ETH-0112 (rev 1) · Best answer: C

A social worker accepts a job at another agency and will leave in 6 weeks. The supervisor has approved a plan for telling clients this week. One client has worked on trauma symptoms with the social worker for 8 months. What should the social worker do **NEXT** with this client?

A. Refer the client to a trauma specialist outside the agency

B. Wait until the last 2 sessions to tell the client of the move

C. Tell the client about the move and discuss options for care

Why each option is right or wrong

A **Incorrect** (trap: Hands off too soon). An outside referral may suit the client, but making it before the client hears about the departure takes away the client's say. A discussion of options, including staying at the agency with a new worker, comes first.

B **Incorrect** (trap: Right action, wrong time). Delaying the news shortens the time to work through the ending, which matters after a long trauma-focused relationship. Late notice can feel like abandonment and leaves little room for a careful transfer.

C **Correct.** Clients should get prompt notice of an anticipated interruption, and a social worker leaving an agency should explain the options for continuing services, with the benefits and risks of each (NASW Code 1.17(e), 1.17(f)). With 6 weeks left and an 8-month trauma relationship, early notice gives time to process the ending and choose the next step together. It beats A because a referral may be one of the options, but deciding it before the client knows about the move removes the client's choice. Any transfer then proceeds with consent (NASW Code 1.16(b)).

Takeaway: When leaving an agency, tell clients early and lay out their options for continuing care so they can choose.

Sources: NASW Code of Ethics (2021), 1.17(e); NASW Code of Ethics (2021), 1.17(f); NASW Code of Ethics (2021), 1.16(b)
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.9 Law and ethics of ending services · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 2 of 3

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13. ASM-1087 (rev 1) · Best answer: A

Which finding is a warning sign of near-term suicide risk rather than a long-term risk factor?

A. Searching online for ways to die
B. A family history of death by suicide
C. Childhood physical abuse
D. A chronic pain condition

Why each option is right or wrong

A **Correct.** Warning signs are current behaviors or states that signal suicide risk in the coming minutes to days, such as looking for a way to kill oneself or talking about wanting to die (Rudd et al., 2006). Risk factors are more stable traits or history that raise risk over a lifetime. Searching for methods is a preparatory behavior that calls for immediate assessment. It beats B because a family history of suicide raises long-term risk but says nothing about whether danger is rising now.

B **Incorrect** (trap: Wrong model or concept). Family history of suicide is a well-established risk factor. It is a fixed part of a person's history and does not show that risk is rising at this moment.

C **Incorrect** (trap: Wrong model or concept). Childhood abuse raises lifetime suicide risk. As a historical factor, it does not mark near-term danger the way current preparatory behavior does.

D **Incorrect** (trap: Wrong model or concept). Chronic pain is a recognized risk factor, especially in later life. It is an ongoing condition rather than a sign that a crisis is building now.

Takeaway: Warning signs, such as seeking methods, signal near-term danger; risk factors, such as family history, raise long-term risk.

Sources: Rudd, M. D., et al. (2006). Warning signs for suicide: Theory, research, and clinical applications. Suicide and Life-Threatening Behavior, 36(3), 255-262, whole article
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.1 Assessing risk of harm to self or others · **Qualifier** none · **Cognitive** recall · **Difficulty** 1 of 3

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14. ETH-1181 (rev 1) · Best answer: A

A social worker at a county child welfare agency drafts a letter to the local newspaper opposing a proposed state cut to foster care funding. The letter draws on the social worker's general experience and names no families. The draft is signed with the social worker's name, job title, and agency. The agency has not taken a position on the cut. What is the **BEST** action for the social worker?

A. Sign as a private citizen and note the views are personal
B. Withhold the letter, since county employees should stay neutral
C. Ask foster parents on the caseload to co-sign the letter
D. Send the letter as drafted, since advocacy is an ethical duty

Why each option is right or wrong

A **Correct.** The Code encourages social and political action and advocacy for policy changes that meet basic needs (6.04(a)), and it asks social workers to distinguish statements made as private individuals from those made for their employer (4.06(a)). Signing with the agency name implies an agency position the agency has not taken. Signing as a private citizen keeps the advocacy and removes the misrepresentation. It beats D because the duty to advocate does not include speaking for an employer without authorization.

B **Incorrect** (trap: Good but not best). Public employees generally may speak on public issues as private citizens, and the Code encourages policy advocacy. Dropping the letter gives up legitimate advocacy when a simple change in how it is signed solves the problem.

C **Incorrect** (trap: Boundary slip or collusion). Recruiting people the social worker serves to back the worker's cause uses the professional relationship for the worker's goals. Foster parents may feel unable to refuse someone with influence over their case.

D **Incorrect** (trap: Misreads law or ethics). Advocacy is encouraged, but signing with the job title and agency name implies the agency endorses the letter. The Code asks social workers to make clear when they speak only for themselves.

Takeaway: Advocate on policy freely, but make clear when you speak as a private individual rather than for your employer or the profession.

Sources: NASW Code of Ethics (2021), 6.04(a); NASW Code of Ethics (2021), 4.06(a)
Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.2 Legal-ethical duties to clients, colleagues, profession, and society · **Qualifier** BEST · **Cognitive** application · **Difficulty** 1 of 3

15. ETH-1531 (rev 2) · Best answer: D

PRETEST — unscored on this form; do not count it

A hospital patient with advanced major neurocognitive disorder can no longer communicate. The patient's living will declines a feeding tube. The patient's health care agent, an adult child, insists on placing one, saying, 'I can't let my parent starve.' The team met with the agent yesterday to review the directive, but the agent's position has not changed, and the agent wants the tube placed this week. What should the social worker do **NEXT**?

A. Follow the agent's instructions as the legal decision-maker
B. Schedule another meeting to review the directive with the agent
C. Explore the agent's grief about the parent's decline
D. Request an ethics consultation on the conflict

Why each option is right or wrong

A **Incorrect** (trap: Misreads law or ethics). The agent does hold decision authority, which is why this option tempts. But that authority is to decide as the patient would have chosen. When the agent's choice contradicts a clear written directive, simply following it sets aside the patient's own wishes.

B **Incorrect** (trap: Right action, wrong time). Another meeting was the right step before, and continued communication matters. The team has already reviewed the directive with the agent, who did not change position, so repeating it delays a fair resolution while a decision is pending.

C **Incorrect** (trap: Good but not best). The agent's grief is real and deserves ongoing support, which may ease the conflict over time. It cannot settle a decision being pressed this week that contradicts the patient's documented wishes; that now needs a formal, impartial review. Grief support can continue alongside the consultation.

D **Correct.** Social workers seek to ensure that a third party making decisions acts consistently with the client's wishes (NASW Code 1.03(c)). The living will clearly declines a feeding tube, a team meeting has failed, and placement is being pressed. An ethics consultation gives a structured, impartial review of the patient's wishes, the agent's role and the options. It beats C because grief support, though needed, cannot settle a pending decision that contradicts the directive. It beats A because an agent's authority is to carry out the patient's known wishes, not replace them (Beauchamp & Childress, 2019).

Takeaway: When a surrogate overrides a patient's clear advance directive and team discussion fails, seek an ethics consultation.

Sources: NASW Code of Ethics (2021), 1.03(c); Beauchamp, T. L., & Childress, J. F. (2019). Principles of Biomedical Ethics (8th ed.). Oxford University Press, Ch. 5: standards of surrogate decision making
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.6 Law and ethics around death and dying · **Qualifier** NEXT · **Cognitive** reasoning · **Difficulty** 3 of 3

16. INT-1027 (rev 1) · Best answer: C

PRETEST — unscored on this form; do not count it

In Meichenbaum's stress inoculation training, during which phase does the client practice coping skills under graded imagined and real-life stressors?

A. Conceptualization
B. Skills acquisition and rehearsal
C. Application and follow-through

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Conceptualization is the first phase. The client and social worker build a shared understanding of how stress works for this client, and coping skills have not yet been taught or tested.

B **Incorrect** (trap: Wrong model or concept). Skills are learned and rehearsed in this phase, so the word 'practice' makes it tempting. Exposure to graded stressors, imagined and then real, comes in the next phase, after the skills are in place.

C **Correct.** Stress inoculation training has three phases. Conceptualization builds a shared understanding of stress and the client's reactions. Skills acquisition and rehearsal teaches and practices coping skills such as relaxation and self-instruction. Application and follow-through has the client use those skills against graded stressors, first in imagery and role-play, then in real life, with relapse prevention (Meichenbaum, 1985). It beats B because the second phase rehearses skills within the session before they are tested against stress.

Takeaway: Stress inoculation moves from understanding stress, to learning and rehearsing coping skills, to applying them under graded stressors.

Sources: Meichenbaum, D. (1985). Stress Inoculation Training. Pergamon Press, three phases of stress inoculation training
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.9 Managing stress: client techniques · **Qualifier** none · **Cognitive** recall · **Difficulty** 3 of 3

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17. ETH-1641 (rev 1) · Best answer: B

Which term describes a person who entered the United States on a student visa and is now applying for protection from persecution in the home country?

A. Refugee
B. Asylum seeker
C. Temporary protected status holder

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Refugee status is granted abroad through overseas processing before the person travels to the United States. Someone who applies after arriving is seeking asylum, even though the persecution standard is the same.

B **Correct.** An asylum seeker requests protection from persecution from inside the United States or at a port of entry; a refugee is screened and approved abroad before arrival (8 U.S.C. 1158; 8 U.S.C. 1157). Both rest on the same definition of persecution (8 U.S.C. 1101(a)(42)), but the status shapes services: refugees arrive with

resettlement agency support, while asylum seekers awaiting a decision generally lack it. It beats A because the person applied after arriving, which makes them an asylum seeker rather than a refugee.

C **Incorrect** (trap: Wrong model or concept). Temporary protected status covers nationals of countries designated because of armed conflict or disaster, whatever their personal history of persecution. It is a separate humanitarian status, not the label for a pending persecution claim.

Takeaway: Refugees are approved abroad before arrival; asylum seekers apply from inside the country, often without resettlement supports while they wait.

Sources: Immigration and Nationality Act, 8 U.S.C. 1101(a)(42), 1157, 1158
Area I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.9 Globalization, immigration, refugee status effects on clients and services · **Qualifier** none · **Cognitive** recall · **Difficulty** 2 of 3

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18. INT-1104 (rev 1) · Best answer: A

Which treatment has the **MOST** consistent research support for helping adults stop using cocaine or methamphetamine?

A. Contingency management plus community reinforcement
B. Cognitive-behavioral relapse prevention therapy
C. Twelve-step facilitation with peer sponsorship
D. Supportive-expressive psychodynamic therapy

Why each option is right or wrong

A **Correct.** Contingency management gives tangible rewards, such as vouchers or prize draws, for drug-free urine samples; the community reinforcement approach builds rewarding non-using activities. In a network meta-analysis of psychosocial treatments for cocaine and amphetamine use, this combination was the only one that raised abstinence compared with usual treatment, and it outperformed CBT and 12-step approaches (De Crescenzo et al., 2018). With no FDA-approved medication for stimulant use disorders (SAMHSA, 2020), it is the leading option. It beats B because CBT is widely used and helpful, but it was less effective on abstinence at the end of treatment.

B **Incorrect** (trap: Wrong model or concept). CBT relapse prevention teaches coping skills for high-risk situations and is widely used. In the network meta-analysis, though, contingency management plus community reinforcement produced more abstinence than CBT at the end of treatment.

C **Incorrect** (trap: Wrong model or concept). Twelve-step approaches offer peer support and help many people, but in the network meta-analysis they produced less abstinence than contingency management plus community reinforcement.

D **Incorrect** (trap: Wrong model or concept). Supportive-expressive psychodynamic therapy has been tested with cocaine use, but the community reinforcement approach outperformed it at longest follow-up. It is not the best-supported option for stimulant abstinence.

Takeaway: For stimulant use disorders, contingency management, best with community reinforcement, has the strongest abstinence evidence; no medication is FDA-approved.

Sources: De Crescenzo, F., Ciabattini, M., D'Alo, G. L., De Giorgi, R., Del Giovane, C., Cassar, C., et al. (2018). Comparative efficacy and acceptability of psychosocial interventions for individuals with cocaine and amphetamine addiction: A systematic review and network meta-analysis. PLoS Medicine, 15(12), e1002715, whole article; SAMHSA (2020). Treatment of Stimulant Use Disorders. Evidence-Based Resource Guide Series (PEP20-06-01-001), medications: none FDA-approved

Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.17 Intervening with addiction · **Qualifier** MOST · **Cognitive** recall · **Difficulty** 2 of 3

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19. ETH-1713 (rev 1) · Best answer: B

A parent's child welfare case plan requires a 10-week parenting class run by a private nonprofit. The parent has a documented reading disability and turns in blank worksheets but takes an active part in discussions. The facilitator plans to report the parent as noncompliant, and the parent asks the caseworker, a social worker, for help. What should the social worker do **FIRST**?

A. Help the parent complete the worksheets after each class

B. Request oral or recorded worksheet responses

C. Find a different parenting program with less written work

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Extra help can support a parent with a reading disability and may serve as a fallback. It leaves the inaccessible format in place, adds to the parent's load, and can blur whether the answers show the parent's own learning.

B **Correct.** Programs open to the public must make reasonable changes in practices when needed to avoid excluding people with disabilities, unless the change would alter the nature of the service (28 CFR 36.302(a)). The parent's active discussion shows learning; the barrier is the written format. Asking for oral or recorded responses fixes the barrier at its source and lets the facilitator judge the parent's real progress. Disability is part of the diversity social workers must understand (NASW Code 1.05(d)). It beats A because worksheets finished with the caseworker's help leave the format unchanged and may raise doubts about whose work they are.

C **Incorrect** (trap: Right action, wrong time). Changing programs is a later step, needed only if the provider refuses a reasonable change. Starting there delays the case plan and leaves the current program's barrier in place for other parents.

Takeaway: When a disability blocks a required program's format, request a reasonable change in the format rather than workarounds or a new program.

Sources: ADA Title III regulations, 28 CFR 36.302(a); NASW Code of Ethics (2021), 1.05(d)

Area I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.4 Access: language, interpretation, ASL, physical, cultural, neurodiversity · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 3 of 3

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20. ETH-1682 (rev 1) · Best answer: B

After one session with a Black social worker, a white client at a community mental health center asks the intake supervisor for a new worker. The client says, 'Nothing personal, but I'd do better with someone more like me.' The supervisor has already talked with the social worker, who is comfortable continuing. What is the supervisor's **BEST** response?

A. Reassign the client to a white social worker as requested
B. Meet with the client about the request and affirm the assignment
C. Ask the social worker again how continuing would feel

Why each option is right or wrong

A **Incorrect** (trap: Misreads law or ethics). Honoring client preferences is usually good practice, but this request asks the agency to remove a qualified worker because of race. Granting it would make the agency part of the discrimination. Reassignment could be reconsidered if a specific clinical problem unrelated to race emerges.

B **Correct.** Social workers should not facilitate discrimination based on race, including in an agency's work assignments (NASW Code 4.02; 3.09(e)). Granting this request would let a preference about the worker's race decide staffing. Meeting with the client lets the supervisor hear any real clinical concern while explaining that assignments rest on qualifications and fit. It beats A because self-determination covers a client's goals and choices in treatment; it does not require the agency to discriminate against its staff.

C **Incorrect** (trap: Good but not best). Checking the worker's wishes was the right step, and it has been done: the worker wants to continue. Asking again shifts the burden back to the worker and delays the response the client needs to hear.

Takeaway: Agencies should not honor a client's request to remove a staff member because of race; address the request directly while protecting the assignment.

Sources: NASW Code of Ethics (2021), 4.02; NASW Code of Ethics (2021), 3.09(e)
Area I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.5 Power differentials, racial privilege, internalized inferiority/superiority · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 3 of 3

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21. INT-1603 (rev 1) · Best answer: C

A social worker with 6 years of independent practice in adult outpatient therapy transfers to a pediatric oncology unit. In supervision, the social worker asks for scripts for explaining procedures to children and wants every family meeting plan reviewed. With adult caregivers, the social worker works confidently and needs little input. Which supervisory approach is **MOST** appropriate?

A. Keep the autonomy-focused approach that suits the worker's experience
B. Explore with the worker why confidence has dropped since the move
C. Give more structure and direction on the child-focused work
D. Give close direction across the worker's whole caseload

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Years of experience do not transfer automatically to a new population. A hands-off approach ignores the worker's clear requests for help with children. It does fit the adult caregiver work.

B **Incorrect** (trap: Good but not best). The stem already shows why the worker wants help: child-focused work is new. Exploring the drop in confidence adds little and delays the structure the worker needs now. Exploration fits when a change in functioning has no clear cause.

C **Correct.** Stoltenberg's Integrated Developmental Model holds that development is domain-specific: a clinician can work autonomously in one area and function like a beginner in another, and supervision should match the level in each domain (Stoltenberg & McNeill, 2010). This worker needs structure for child-focused work but not for adult caregiver work. It beats D, which treats the worker as a beginner overall and undercuts autonomy where the worker is competent, and A, which treats experience as one global level.

D **Incorrect** (trap: Misuses the technique). Close direction on every case treats the worker as a beginner across the board. That can erode motivation and autonomy in the adult caregiver work, where the worker already functions independently.

Takeaway: Supervisee development is domain-specific: give more structure in new areas while keeping autonomy where the supervisee is already competent.

Sources: Stoltenberg, C. D., & McNeill, B. W. (2010). IDM Supervision: An Integrative Developmental Model for Supervising Counselors and Therapists (3rd ed.). Routledge, Domains of development; supervision environments by level

Area III Intervention and Practice · **Sub-area** III.D Supervision and Administration · **Topic** III.D.2 Supervision and consultation models: individual, peer, group · **Qualifier** MOST APPROPRIATE · **Cognitive** reasoning · **Difficulty** 2 of 3

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22. ETH-1634 (rev 1) · Best answer: B

A juvenile diversion program refers a 15-year-old for a restorative conference with a store owner whose shop window was broken. At the preparation meeting, the youth tells the social worker, 'I wasn't even there. They just blame me because of my friends.' The store owner is eager to meet. What is the **BEST** way to proceed?

A. Hold the conference so the youth hears the owner's losses
B. Put the conference on hold while the youth denies involvement
C. Let the store owner decide whether the conference goes ahead
D. Urge the youth to admit the act to stay in diversion

Why each option is right or wrong

A **Incorrect** (trap: Misuses the technique). Hearing the impact can prompt accountability when a youth minimizes harm. A flat denial is different: the meeting could force an innocent youth to absorb blame, and the owner may be hurt again by hearing a denial in person.

B **Correct.** A restorative conference requires the person who caused harm to accept at least some responsibility; without that, the meeting risks hurting the harmed person again and pressuring a youth who may be innocent (Zehr, 2015). The youth flatly denies being there, so preparation shows the conference is not ready. Pausing it and returning the question to the referral source protects both parties. It beats C because the owner's willingness is necessary but cannot supply the responsibility the process needs. Pressing for an admission would also undercut the youth's free choice (NASW Code 1.02).

C **Incorrect** (trap: Good but not best). The harmed person's needs are central, and the owner's willingness is required for a conference. It is not enough on its own: the youth must also accept responsibility, so the owner's eagerness cannot decide the question.

D **Incorrect** (trap: Overrides the client's choice). Tying diversion to a confession coerces the youth and can produce a false admission. Responsibility in restorative work must be freely accepted; if the youth maintains innocence, the case returns to the usual process.

Takeaway: A restorative conference needs the wrongdoer's acknowledgment of responsibility; when the person denies the act, pause rather than proceed or pressure.

Sources: Zehr, H. (2015). The Little Book of Restorative Justice (revised and updated ed.). Good Books, Restorative justice is not mediation; offender responsibility; NASW Code of Ethics (2021), 1.02
Area I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.8 Restorative practice, truth and reconciliation with historically oppressed groups · **Qualifier** BEST · **Cognitive** application · **Difficulty** 2 of 3

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23. ETH-1618 (rev 2) · Best answer: C

A Deaf client at a nonprofit counseling agency asks for an ASL interpreter for weekly sessions. The program manager tells the social worker to use written notes because interpreters are costly. When the social worker lays out the client's request and the access concern, the manager repeats the decision. What should the social worker do **NEXT**?

A. Use written notes as directed and record the client's request
B. Ask the client to bring a relative who signs to sessions
C. Seek guidance from the agency's director or compliance officer
D. Refer the client to an agency that provides interpreters

Why each option is right or wrong

A **Incorrect** (trap: Misreads law or ethics). Recording the request is good practice, but following a directive that likely denies effective communication puts cost ahead of the client's access rights. Written notes suit brief, simple exchanges, not weekly counseling.

B **Incorrect** (trap: Misreads law or ethics). Agencies may not require a client to bring someone to interpret, and relatives raise privacy and accuracy concerns in counseling. Providing a qualified interpreter is the agency's responsibility.

C **Correct.** A private agency must provide auxiliary aids, including qualified interpreters, when needed for effective communication, and may not require the client to bring someone to interpret (28 CFR 36.303(b)(1), (c)). Weekly counseling is unlikely to work through written notes. Social workers should not let agency directives interfere with ethical practice (NASW Code 3.09(d)). The social worker has raised the issue with the manager without success, so the director or compliance officer is the next channel. It beats D because referring out shifts the agency's duty onto the client.

D **Incorrect** (trap: Hands off too soon). Referral would fit if the client wanted a program that specializes in serving Deaf clients. Here the client chose this agency, and referring out because of interpreter costs shifts the agency's legal duty onto the client.

Takeaway: When a manager refuses a required accommodation after it is raised, take the concern to the next level, such as the director or compliance officer.

Sources: ADA Title III regulations, 28 CFR 36.303(b)(1), (c); NASW Code of Ethics (2021), 3.09(d)
Area I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.4 Access: language, interpretation, ASL, physical, cultural, neurodiversity · **Qualifier** NEXT · **Cognitive** reasoning · **Difficulty** 3 of 3

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24. ETH-1191 (rev 1) · Best answer: B

Two weeks after a child on the unit's caseload died from abuse, a child welfare supervisor notices that a supervisee who worked the case startles easily, avoids that neighborhood, and snaps at coworkers. The supervisee has not mentioned any of this. What should the supervisor do **FIRST**?

A. Refer the supervisee to the employee assistance program for help
B. Meet privately to ask how the case is affecting the supervisee
C. Reassign the supervisee's cases in that neighborhood

Why each option is right or wrong

A **Incorrect** (trap: Hands off too soon). An employee assistance referral may well follow, but sending the supervisee elsewhere before talking can feel like a brush-off. It also skips the supervisor's own role in recognizing and responding to the reaction.

B **Correct.** Startle, avoidance and irritability after a traumatic case event fit secondary traumatic stress, and reflective supervision is a primary way to recognize and address it (NCTSN, 2011). The NASW Code also encourages agencies to support workers' self-care (Purpose). The supervisor has seen signs but has not heard from the supervisee, so a private conversation about how the case is affecting them comes first and shows what support fits. It beats A because a referral before that conversation hands off the concern without understanding it.

C **Incorrect** (trap: Skips assessment). Changing assignments may help, but deciding for the supervisee before talking assumes what they need and can feel punitive. Workload changes work best when planned with the supervisee after the conversation.

Takeaway: When a supervisee shows trauma-related stress after a case event, start with a private supportive conversation before referring or reassigning.

Sources: National Child Traumatic Stress Network (2011). Secondary Traumatic Stress: A Fact Sheet for Child-Serving Professionals, Supervision and organizational strategies; NASW Code of Ethics (2021), Purpose
Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.6 Burnout, compassion fatigue, and secondary traumatic stress · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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25. INT-1513 (rev 2) · Best answer: B

At a homeless shelter, a client cannot apply for jobs because a stolen wallet held the client's ID and birth certificate. The social worker and client have agreed that replacing the ID is the problem to work on first. What should the social worker do **NEXT**?

A. Pick the quickest way to replace the ID and tell the client
B. List possible ways to replace the ID with the client
C. Request a shelter residency letter to take to the ID office
D. Explore how losing the wallet has affected the client

Why each option is right or wrong

A **Incorrect** (trap: Too directive). Choosing for the client is efficient, but it skips generating and weighing options and puts the worker in charge of the client's problem. Choosing comes after alternatives are listed and compared with the client.

B **Correct.** The problem-solving sequence runs from defining the problem to generating alternatives, deciding, acting, and checking results (D'Zurilla & Goldfried, 1971). The problem is now defined and agreed, so the next step is brainstorming options with the client, such as a shelter letter, a vital records request, or a legal aid clinic. The worker's role is to support the client's own problem solving. It beats A because picking the route for the client skips the alternatives step and takes the decision away from the client.

C **Incorrect** (trap: Right action, wrong time). A shelter residency letter may well become part of the plan, but acting on one route now skips generating and weighing alternatives with the client. It fits once the client has compared the options and chosen that route.

D **Incorrect** (trap: Right action, wrong time). Exploring the impact of the theft is a worthwhile engagement move, but the pair have already defined the problem together. Returning to feelings now delays the concrete work the client needs.

Takeaway: In problem solving, once the problem is defined, generate alternatives with the client before choosing, acting, or evaluating.

Sources: D'Zurilla, T. J., & Goldfried, M. R. (1971). Problem solving and behavior modification. Journal of Abnormal Psychology, 78(1), 107-126, stages of problem solving
Area III Intervention and Practice · **Sub-area** III.A Practice Concepts · **Topic** III.A.4 What the social worker does in problem solving · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 2 of 3

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26. ETH-1157 (rev 1) · Best answer: D

An outpatient substance use program sends automatic text reminders before appointments. A new client says they often forget appointments and would like reminders. However, everyone in the household shares one phone, and relatives read the messages on it. Which plan for reminders is **MOST** appropriate?

A. Send the standard reminder, since the client asked for one
B. Skip reminders to keep the treatment private
C. Mail written reminders to the client's home instead
D. Agree with the client on a discreet reminder method

Why each option is right or wrong

A **Incorrect** (trap: Misreads law or ethics). The client wants reminders, but a standard message from a substance use program would show relatives where the client is being treated. Honoring the request without adjusting the method ignores the privacy risk the client described.

B **Incorrect** (trap: Good but not best). Skipping reminders removes the privacy risk but also the help the client asked for with forgetting appointments. Missed sessions could undermine treatment when a discreet method could serve both needs.

C **Incorrect** (trap: Misuses the technique). Household mail is another shared channel that relatives may open. Switching to letters without asking the client trades one exposure for another and is slower than the reminder the client needs.

D **Correct.** Providers must accommodate reasonable requests to receive communications by alternative means (HIPAA, 45 CFR 164.522(b)), and social workers should protect the confidentiality of electronic communications (NASW Code 1.07(m)). This client needs reminders to keep appointments yet faces a real privacy risk on a shared phone. Working out a method together, such as a call at a set time or a generic text that names no program, meets both needs and leaves the client in control. It beats B because dropping reminders protects privacy at the cost of the support the client asked for.

Takeaway: Tailor reminders and other electronic contact to each client's privacy situation, agreeing on a method that meets needs without revealing treatment.

Sources: HIPAA Privacy Rule, 45 CFR 164.522(b); NASW Code of Ethics (2021), 1.07(m)
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.10 Ethics of electronic and technology-mediated practice · **Qualifier** MOST APPROPRIATE · **Cognitive** reasoning · **Difficulty** 1 of 3

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27. INT-1502 (rev 1) · Best answer: B

A 70-year-old client, widowed for 2 years, has met with the social worker weekly for 4 months. Near the end of a session, the client says, 'I think I've fallen in love with you. You're the only one who understands me.' What is the social worker's **BEST** response?

A. Arrange a transfer to a colleague before the next session
B. Explore the feelings in sessions while holding a clear role
C. Remind the client that the relationship is strictly professional

Why each option is right or wrong

A **Incorrect** (trap: Hands off too soon). Transferring at the first disclosure treats a workable transference as a failure and can feel like rejection to a grieving client. Transfer becomes appropriate if the feelings cannot be managed in the work or the worker's own reactions interfere.

B **Correct.** Romantic feelings toward a social worker are a common form of transference and are treated as clinical material, not as a crisis. Exploring them, while the worker keeps the role clear and never reciprocates (NASW Code 1.09(a)), can show what the client longs for, here connection after losing a spouse. It beats C because a bare reminder of the rules is accurate but can leave the client ashamed and closes off useful material; the boundary is held while the meaning is explored (Hepworth et al.).

C **Incorrect** (trap: Good but not best). Stating the professional boundary is necessary, but stopping there can shame the client and lose the chance to understand the feelings. The boundary works best when it is paired with exploration of what the feelings mean.

Takeaway: Treat a client's romantic feelings as transference to explore within a clear professional role, not as grounds for immediate transfer.

Sources: Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, Relational reactions: transference; NASW Code of Ethics (2021), 1.09(a)
Area III Intervention and Practice · **Sub-area** III.A Practice Concepts · **Topic** III.A.2 Building and sustaining the helping relationship (empathy, rapport, acceptance) · **Qualifier** BEST · **Cognitive** application · **Difficulty** 2 of 3

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28. ASM-1168 (rev 1) · Best answer: C

In a pediatric emergency department, a 15-year-old being treated for an asthma attack completes the hospital's suicide risk screen with a nurse. The teen answers yes when asked about having thoughts of killing themselves right now. The parent is in the waiting room. What should the social worker do **NEXT**?

A. Schedule an outpatient mental health visit for this week
B. Ask the parent whether the teen has talked about suicide before
C. Ensure the teen is watched until an urgent safety evaluation

Why each option is right or wrong

A **Incorrect** (trap: Ignores risk). Outpatient follow-up suits a non-acute positive screen. A yes to thoughts of suicide right now signals possible imminent risk, which calls for same-visit evaluation and observation, not a later appointment.

B **Incorrect** (trap: Right action, wrong time). Collateral history helps judge risk and will be part of the full evaluation. Seeking it first, before the teen is under observation and an urgent evaluation is requested, puts information ahead of safety.

C **Correct.** A yes to current suicidal thoughts on a screen such as the NIMH Ask Suicide-Screening Questions (ASQ) marks an acute positive result. The toolkit calls for keeping the patient in sight, removing dangerous objects and getting an urgent full safety evaluation before the patient leaves (NIMH ASQ). It beats B because history from the parent is valuable and will be gathered during the evaluation, but protection comes first once the screen has flagged imminent risk.

Takeaway: An acute positive suicide screen calls for continuous observation and an urgent full safety evaluation before the patient leaves.

Sources: National Institute of Mental Health (NIMH). Ask Suicide-Screening Questions (ASQ) Toolkit: Suicide Risk Screening Tool, Acute positive screen
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.1 Assessing risk of harm to self or others · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 2 of 3

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29. INT-1675 (rev 1) · Best answer: B

Which needs assessment approach uses public data, such as census and health figures, to compare needs across neighborhoods?

A. Key informant approach
B. Social indicators approach
C. Community forum approach
D. Rates-under-treatment approach

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Key informant work gathers the views of people who know the community well, such as clergy, principals or physicians, usually through interviews. The approach described relies on public statistics rather than the judgments of informed individuals.

B **Correct.** The social indicators approach estimates need from existing public data, such as census, housing and health statistics, and compares areas or tracks change over time. Its strengths are low cost and broad coverage; its weakness is that the figures are

indirect measures of need, so they are often paired with surveys or forums. It beats D because rates under treatment count people already receiving services, which misses residents whose needs are unmet (Royse, Thyer & Padgett, 2016; Kettner, Moroney & Martin, 2022).

C **Incorrect** (trap: Wrong model or concept). A community forum invites residents to an open meeting to describe needs in their own words. It captures lived experience but tends to draw the most vocal residents. The approach described analyzes existing data instead of holding a meeting.

D **Incorrect** (trap: Wrong model or concept). Rates under treatment use service statistics, such as caseloads and waitlists, to infer need. It is tempting because it also relies on existing numbers, but those come from agency records of people already served, not population-wide public statistics.

Takeaway: Needs assessment from existing public statistics compared across areas is the social indicators approach; counts of people served are rates under treatment.

Sources: Royse, D., Thyer, B. A., & Padgett, D. K. (2016). Program Evaluation: An Introduction to an Evidence-Based Approach (6th ed.). Cengage, Ch. 3: Needs Assessment; Kettner, P. M., Moroney, R. M., & Martin, L. L. (2022). Designing and Managing Programs: An Effectiveness-Based Approach (6th ed.). SAGE, Ch. 4-5: Needs Assessment

Area III Intervention and Practice · **Sub-area** III.C Practice Evaluation and Research · **Topic** III.C.4 Agency program evaluation: needs assessment, formative/summative, cost analyses · **Qualifier** none · **Cognitive** recall · **Difficulty** 2 of 3

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30. ASM-1666 (rev 1) · Best answer: B

PRETEST — unscored on this form; do not count it

At a community clinic, a client returns intake forms with most sections blank. When the social worker points to the consent page, the client says they left their reading glasses at home and asks to take everything home to finish. The client missed two earlier appointments that were scheduled by letter. What should the social worker do **FIRST**?

A. Ask the client to read the first paragraph aloud
B. Offer to review the forms together aloud
C. Let the client take the forms home to finish
D. Refer the client to an adult literacy program

Why each option is right or wrong

A **Incorrect** (trap: Misuses the technique). Asking someone to read aloud amounts to an on-the-spot test. It risks shame and may lead the client to avoid the clinic. Universal precautions favor offering help to everyone over testing individuals.

B **Correct.** Health literacy universal precautions call for assuming that anyone may struggle with written information and offering help without testing or embarrassing the person (AHRQ). Blank forms, a forgotten-glasses explanation and missed appointments scheduled by letter are common signs of limited reading skills. Offering to go over the forms together, reading them aloud as a routine courtesy, gets the needed information and protects the client's dignity. It beats D because a literacy referral may help later if the client wants it, but raising it now singles the client out and leaves today's forms and consent unfinished.

C **Incorrect** (trap: Good but not best). Taking the forms home accepts the client's way of saving face, but the same reading barrier will be there. Missed letters suggest the forms may not come back complete, and consent needs to be understood.

D **Incorrect** (trap: Right action, wrong time). An adult literacy program may be a welcome option later, offered privately if the client raises reading. Suggesting it first labels the client and does nothing for today's forms and consent.

Takeaway: When signs suggest limited literacy, offer routine help with written material, such as reviewing it together aloud, rather than testing the client.

Sources: Agency for Healthcare Research and Quality (AHRQ) (2024). Health Literacy Universal Precautions Toolkit (3rd ed.), Introduction: health literacy universal precautions

Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.8 Evaluating a client's communication skills · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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31. ASM-1595 (rev 1) · Best answer: C

At intake in a community clinic, a client whose preferred language is Tagalog reports low mood and poor sleep. The clinic routinely gives a standardized depression questionnaire, and its publisher offers versions in many languages, including Tagalog. A trained interpreter is on the call. Which approach is **BEST**?

A. Skip the questionnaire and rely on the clinical interview

B. Have the interpreter read the English version aloud in Tagalog

C. Give the client the published Tagalog version of the questionnaire

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). The clinical interview remains essential, but dropping the measure denies this client the standardized screening other clients receive. A properly translated version makes the tool usable, so skipping it is unnecessary.

B **Incorrect** (trap: Misuses the technique). Translating items on the spot changes wording from one administration to the next, so scores may no longer mean what the norms assume. Trained interpreters are essential for the interview, but not as a substitute for a validated translation of a standardized measure.

C **Correct.** Standardized measures give meaningful scores only when given in the form that was developed and tested, and a properly adapted translation preserves that (Standards for Educational and Psychological Testing, Ch. 3). Using the published Tagalog version gives the client equal access to screening in their own language (NASW Cultural Competence Standard 9). It beats B because translating items on the spot alters their meaning and makes the score hard to interpret.

Takeaway: With clients who prefer another language, use a properly translated and tested version of a standardized measure, not on-the-spot translation.

Sources: Standards for Educational and Psychological Testing (2014). American Educational Research Association, American Psychological Association, & National Council on Measurement in Education, Ch. 3: Fairness in Testing; NASW Standards and Indicators for Cultural Competence in Social Work Practice (2015), Standard 9: Language and Communication

Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.6 Tools for assessing strengths, resources, needs, challenges · **Qualifier** BEST · **Cognitive** application · **Difficulty** 2 of 3

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32. INT-1040 (rev 1) · Best answer: C

A family is referred for in-home therapy after a 15-year-old's school suspension. At the first visit, a parent sits with arms crossed and says, 'The last provider told us everything we were doing wrong.' The social worker uses structural family therapy. What should the social worker do **FIRST**?

A. Ask the family to act out a recent argument in session
B. Seat the parents together and the teen across the room
C. Learn the family's routines and language and follow their lead

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). Enactment, having the family show a problem interaction in the session, is the signature structural technique. It works once the social worker has joined the family; here it would come too early.

B **Incorrect** (trap: Right action, wrong time). Changing seating is a way to mark boundaries and hierarchy, a later structural move. Rearranging a guarded family at the first visit, before joining, risks looking like another provider taking over.

C **Correct.** In structural family therapy, joining comes first: the social worker accommodates to the family's style, tracks their story, and uses their language to build the alliance that later challenges require (Minuchin & Fishman, 1981). The parent's guarded stance and bad experience with a critical provider make joining essential before any restructuring. It beats A because enactment is a core structural technique, but asking a wary family to perform a conflict before the worker has standing with them is likely to deepen their defensiveness.

Takeaway: Join first: accommodate to and track a guarded family before challenging how it is organized.

Sources: Minuchin, S., & Fishman, H. C. (1981). Family Therapy Techniques. Harvard University Press, joining **Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.21 Family therapy models and interventions · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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33. ETH-1658 (rev 3) · Best answer: D

A 79-year-old hospital patient recovering from a hip fracture wants to go home alone instead of to a rehabilitation facility. A psychiatric consultant found the patient able to make this decision, and the patient knows another fall could happen. Worried, the team asks the social worker to hold off on discharge planning until the patient agrees to rehab. What should the social worker do **NEXT**?

A. Request a second capacity evaluation from another consultant
B. Ask the patient's adult child to urge the move to rehab
C. Have the patient sign a discharge-against-advice form first
D. Plan a home discharge with services the patient accepts

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). A repeat evaluation fits when capacity seems to fluctuate or the first assessment was incomplete. Here a consultant has assessed this specific decision; seeking another opinion because the team disagrees treats disagreement as incapacity.

B **Incorrect** (trap: Overrides the client's choice). Family can help when the patient invites them in. Asking a relative to push for rehab enlists others to change a capable adult's decision, which undermines the patient's choice and trust in the team.

C **Incorrect** (trap: Misreads law or ethics). Against-advice forms are meant for patients who leave before the team considers any discharge appropriate. This patient is ready for discharge and, with capacity, has chosen among settings; the label treats a lawful choice as defiance, can strain trust and follow-up, and does nothing to make the home plan safer.

D **Correct.** A capable adult may choose a riskier option; the Code permits limiting self-determination when a client's actions pose a serious, foreseeable and imminent risk (NASW Code 1.02), which the possibility of a future fall does not. Capacity is decision-specific; it has been assessed for this choice, and the patient understands the fall risk (Appelbaum, 2007). The social worker's task now is to make the chosen plan as safe as possible, with home health, therapy, a medical alert device and home changes. It beats A because a second opinion sought only because the team dislikes the answer turns assessment into a tool for overriding the patient.

Takeaway: Once a patient is found capable of a decision, help make the chosen plan as safe as possible instead of looking for ways around it.

Sources: NASW Code of Ethics (2021), 1.02; Appelbaum, P. S. (2007). Assessment of patients' competence to consent to treatment. New England Journal of Medicine, 357(18), 1834-1840, decision-specific capacity
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.1 Protecting and promoting client self-determination, including right to refuse · **Qualifier** NEXT · **Cognitive** reasoning · **Difficulty** 2 of 3

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34. ASM-1690 (rev 2) · Best answer: A

A 31-year-old veteran who completed two combat deployments tells a community agency social worker about nightmares and growing distance from their partner. The veteran refuses to go to a VA hospital, saying it 'feels like being back on base,' and cannot afford private fees. Which referral is **MOST** appropriate?

A. A Vet Center offering individual and couples readjustment counseling
B. The outpatient mental health clinic at the VA medical center
C. A trauma specialist in private practice near the client's home

Why each option is right or wrong

A **Correct.** A fitting resource must be available, acceptable and affordable for this client. Vet Centers are community-based counseling centers located apart from VA medical facilities, offering readjustment counseling at no cost to eligible veterans who served in a combat theater, including couples and family counseling (U.S. Department of Veterans Affairs, 2025). That addresses the nightmares, the strain with the partner, the setting the veteran rejected and the cost barrier. Referral fits when another provider's expertise is needed (NASW Code 1.16(a)). It beats B because the VA clinic is eligible care but is the hospital setting the veteran refused, so follow-through is unlikely.

B **Incorrect** (trap: Overrides the client's choice). The VA clinic offers strong trauma treatment, and the veteran is likely eligible. But the veteran has clearly refused the hospital setting, and pressing that option disregards the client's stated preference.

C **Incorrect** (trap: Good but not best). A trauma specialist in private practice could be clinically suitable, but the veteran has said private fees are unaffordable. A referral the client cannot pay for is unlikely to be used.

Takeaway: Match referrals to what the client will accept and can access; community-based Vet Centers serve eligible combat veterans and families at no cost.

Sources: U.S. Department of Veterans Affairs (2025). Vet Center Services, VA Welcome Kit quick start guide, services, eligibility, location and cost; NASW Code of Ethics (2021), 1.16(a)
Area II Assessment and Planning · **Sub-area** II.C Assessment Practices · **Topic** II.C.1 Identifying available, appropriate community resources · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 1 of 3

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35. INT-1534 (rev 2) · Best answer: B

A case manager on a community mental health team makes a home visit to a client with schizophrenia. The client says the neighbors are poisoning the tap water. Six sealed water jugs are stacked by the door, and the refrigerator is empty. Which progress-note entry **BEST** documents the visit?

A. Client appeared paranoid about the water and hoarded jugs; refrigerator empty, likely off medications
B. Client said neighbors are poisoning the tap water; six jugs by door; refrigerator empty
C. Client's fear that neighbors use poison seemed worse than last month; self-care looked poor

Why each option is right or wrong

A **Incorrect** (trap: Skips assessment). This labels the client and guesses at a cause no one confirmed. Medication status should be checked and recorded as fact, not assumed. An interpretation belongs in a marked assessment section, tied to observations.

B **Correct.** Records should be accurate and specific enough to support continuity of care (NASW Code 3.04(a), 3.04(b)). The best entry records what the client said and what the worker saw, without labels or guesses, so the next reader can judge the situation and follow up on the empty refrigerator. It beats C because 'seemed worse' and 'looked poor' are impressions with no observable facts behind them; a later reader cannot tell what changed or how serious it is (Kagle & Kopels, 2008).

C **Incorrect** (trap: Misuses the technique). This is an impression without observable facts. A reader cannot tell what the client said, what the worker saw, or how serious the food situation is. Specific description is what makes a note useful.

Takeaway: Document observable facts and the client's own words; labels and guesses without supporting observations make records inaccurate and less useful.

Sources: NASW Code of Ethics (2021), 3.04(a)-(b); Kagle, J. D., & Kopels, S. (2008). Social Work Records (3rd ed.). Waveland Press, content and language of records
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.18 Case management: recording, documentation, case presentation · **Qualifier** BEST · **Cognitive** application · **Difficulty** 1 of 3

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36. INT-1066 (rev 2) · Best answer: A

In a closed school group for 9th graders whose parents are divorcing, three members who share a lunch period start sitting together. During sessions, they trade glances and laugh quietly when others speak, and two members who used to share have gone quiet. Which response is **BEST**?

A. Name the pattern aloud and invite members to discuss it
B. Meet with the three members privately and ask them to stop
C. Wait a session to see whether the pattern fades

Why each option is right or wrong

A **Correct.** Subgroups form in most groups; the leader's task is to make covert alliances overt so the group can examine how they affect trust and participation (SAMHSA TIP 41). Naming the glances and laughter without blame, and asking how members experience them, lets the quieted members be heard and turns the pattern into group work. It beats B because a private talk may stop the behavior for a while, but it keeps the dynamic hidden, leaves the silenced members' reactions unaddressed, and can deepen the split between insiders and outsiders.

B **Incorrect** (trap: Good but not best). Talking privately with the three may quiet the behavior, but it keeps the dynamic secret from the group it affects. A private meeting fits if the subgroup's behavior turns harmful after it has been discussed openly.

C **Incorrect** (trap: Good but not best). Some subgroup behavior does fade, and waiting avoids overreacting to a new friendship. Here two members have already gone quiet, so the alliance is shaping participation now; leaving it unnamed lets the split harden in a closed group with a limited number of sessions.

Takeaway: Make covert subgroups overt: name the pattern in the group so members can examine its effect on trust and participation.

Sources: SAMHSA TIP 41 (2005), Ch. 6: Group Leadership, Concepts, and Techniques (subgroups)
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.23 Group work: managing process and building cohesion · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 3 of 3

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37. ETH-1558 (rev 2) · Best answer: B

A hospital opens an outpatient eating disorders program and asks an experienced clinical supervisor to supervise its 4 newly hired social workers. The supervisor has strong skills with mood and anxiety disorders but no training in eating disorders. Several program clients have serious medical complications. What is the **BEST** response by the supervisor?

A. Accept and learn the specialty alongside the new staff
B. Ask program leaders to secure an eating disorders specialist to supervise
C. Accept and meet monthly with an outside eating disorders expert
D. Decline and let the program find someone else to supervise

Why each option is right or wrong

A **Incorrect** (trap: Outside the social work role). Learning alongside new staff means supervising high-risk work without the knowledge to catch errors. A new specialty is built through one's own training and supervision, not while serving as the team's supervisor.

B **Correct.** Supervisors work only within their knowledge and competence (NASW Code 3.01(a)). When a specialty is unfamiliar, the supervision standards direct supervisors to obtain assistance or refer supervisees to an appropriate source (Supervision Standards, Standard 2: Competency). With new staff and medically fragile clients, the team needs a supervisor already trained in eating disorders, and administrators are responsible for adequate supervision resources (NASW Code 3.07(c)). It beats C because monthly outside consultation leaves week-to-week supervision of high-risk cases with someone who lacks the needed skill.

C **Incorrect** (trap: Good but not best). The supervision standards do endorse getting expert help when a specialty is unfamiliar, and it would help here. But monthly contact cannot make a supervisor with no training competent to guide new staff week to week on medically complicated cases.

D **Incorrect** (trap: Good but not best). Declining is honest about the limits of competence, but simply stepping back leaves new staff and fragile clients without a plan. The stronger step names the need and helps secure qualified supervision.

Takeaway: Supervise only within competence; when a specialty is unfamiliar, help secure qualified supervision instead of learning on the job.

Sources: NASW Code of Ethics (2021), 3.01(a); NASW Code of Ethics (2021), 3.07(c); NASW & ASWB Best Practice Standards in Social Work Supervision (2013), Standard 2 (Conduct of Supervision): Competency
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.7 Ethics of supervising and managing · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 3 of 3

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38. ETH-1094 (rev 1) · Best answer: A

A social worker co-leads a skills group with a newly hired colleague. In the last two sessions, the colleague taught distress-tolerance skills inaccurately, and members seemed confused. The colleague mentions never having received DBT training and says feedback is welcome. What should the social worker do **FIRST**?

A. Talk with the colleague and share DBT training resources
B. Tell the program manager the colleague lacks DBT training
C. Correct the colleague's teaching during the next group

Why each option is right or wrong

A **Correct.** With direct knowledge of a colleague's incompetence, social workers consult that colleague when feasible and help them take remedial action (NASW Code 2.09(a)). The colleague is open to feedback and the problem is a training gap, so a direct conversation with resources is feasible and likely to work. A beats B because the manager becomes the next step only if the colleague does not take adequate steps to address the problem (2.09(b)).

B **Incorrect** (trap: Right action, wrong time). Going through appropriate channels is the second step, used when the colleague does not act. This colleague welcomes feedback, so starting with the manager skips the collegial step.

C **Incorrect** (trap: Misuses the technique). Members need accurate skills, but correcting a co-leader in front of the group undermines the colleague and can confuse members about who leads. Feedback belongs in a private conversation first, offered with respect (2.01(a)).

Takeaway: For a colleague's correctable incompetence, start with a direct conversation and help, before going to a manager.

Sources: NASW Code of Ethics (2021), 2.09(a); NASW Code of Ethics (2021), 2.09(b); NASW Code of Ethics (2021), 2.01(a)
Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.2 Legal-ethical duties to clients, colleagues, profession, and society · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 1 of 3

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39. ETH-1159 (rev 1) · Best answer: A

While scrolling a community events page on a personal account, a social worker sees a friend's recent party photo that shows a client holding a drink. The client is in treatment for alcohol use disorder and said at the last session that they have been sober for 4 months. What is the **BEST** action for the social worker?

A. Tell the client what was seen and discuss it
B. Record a possible relapse in the client's chart
C. Keep it private to protect the client's trust

Why each option is right or wrong

A **Correct.** When a social worker comes across client information online by accident, the technology standards advise telling the client and discussing what the worker now knows and what it may mean for the relationship (Tech Standard 3.09). Information found online must also be verified before it is used, and a photo of a drink may not show what it seems. Raising it openly lets the client explain and keeps the relationship honest. It beats C because silence leaves the social worker holding unspoken information that can shape the work, which undermines trust more than a transparent conversation.

B **Incorrect** (trap: Skips assessment). Charting a relapse from one unverified photo records speculation as fact. Online information must be checked with the client or the original source before it guides care or enters the record (3.04(a)).

C **Incorrect** (trap: Good but not best). Saying nothing can feel like protecting the client, but the social worker now knows something that may affect treatment. The standards favor telling the client how it was found and discussing it, rather than keeping a secret within the relationship.

Takeaway: Client information found online by accident should be disclosed to the client and discussed, and verified before it shapes care or the record.

Sources: NASW, ASWB, CSWE & CSWA Standards for Technology in Social Work Practice (2017), Standard 3.09; NASW Code of Ethics (2021), 3.04(a)
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.10 Ethics of electronic and technology-mediated practice · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 3 of 3

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40. ETH-1114 (rev 1) · Best answer: D

PRETEST — unscored on this form; do not count it

A pre-licensure associate social worker at an outpatient clinic learns that the clinical supervisor will be on medical leave for 6 weeks. The supervisor suggests the associate keep seeing clients and save questions until the leave ends. The associate replies that two clients have recent self-harm and waiting is not workable, but the supervisor repeats the suggestion. What should the associate do **NEXT**?

A. Raise the concern with the supervisor once more
B. Continue sessions and keep a list of questions
C. Pause client sessions until the supervisor returns
D. Ask the program director to arrange interim supervision

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). Talking with the supervisor first was right, and it has been done. Repeating it after a clear answer delays action while clients with recent self-harm go without timely oversight.

B **Incorrect** (trap: Misreads law or ethics). Saving questions for 6 weeks leaves an associate practicing without the oversight the license requires and delays guidance on high-risk clients. Interim supervision is needed now.

C **Incorrect** (trap: Ignores risk). Stopping all sessions would leave clients, including two with recent self-harm, without care. The gap is in supervision, which can be filled without interrupting services (1.17(b)).

D **Correct.** Pre-licensure practitioners provide clinical services under the supervision their regulations require (NASW & ASWB, 2013, Standard 3), and administrators should ensure adequate resources for staff supervision (NASW Code 3.07(c)). Clients with recent self-harm need timely clinical guidance (2.05(a)). The associate has already raised the problem with the supervisor without result, so the next step is the program director, who can assign a qualified interim supervisor. D beats A because repeating the same request to the same person has already failed and delays help for clients at risk.

Takeaway: Pre-licensure practice requires ongoing supervision; if the supervisor cannot provide it, go up the agency's chain to arrange interim coverage.

Sources: NASW & ASWB Best Practice Standards in Social Work Supervision (2013), Standard 3: Legal and Regulatory Issues; NASW Code of Ethics (2021), 3.07(c); NASW Code of Ethics (2021), 2.05(a); NASW Code of Ethics (2021), 1.17(b)
Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.8 Keeping knowledge current: training, licensing CE, supervision, consultation · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 1 of 3

41. ASM-1576 (rev 1) · Best answer: C

In a cardiac rehabilitation program, a 68-year-old client mentions having 'a few glasses of wine' most evenings. The social worker needs an accurate picture of the client's drinking to share with the care team. Which question is **BEST**?

A. 'You don't drink much more than that, do you?'
B. 'Would you say your drinking is a problem?'
C. 'On a typical drinking day, how much do you have?'

Why each option is right or wrong

A **Incorrect** (trap: Misuses the technique). This leading question signals the answer the worker expects and makes it awkward for the client to report heavier drinking. It invites a quick 'no' rather than an accurate amount.

B **Incorrect** (trap: Misuses the technique). Asking the client to judge their own drinking as a 'problem' is a labeling, closed question. Many people who drink at risky levels answer no, so it yields an opinion rather than the amount the team needs.

C **Correct.** Sensitive questions about substance use work best when they are specific, nonjudgmental and assume the behavior may occur. Asking how much the client has on a typical drinking day mirrors the consumption items of the AUDIT screen (Babor et al., WHO) and turns a vague 'a few glasses' into a usable amount. It beats B because asking whether drinking is a 'problem' invites denial and does not measure quantity (Hepworth et al.).

Takeaway: To learn about substance use, ask specific, nonjudgmental quantity and frequency questions; avoid leading or labeling questions.

Sources: Babor, T. F., Higgins-Biddle, J. C., Saunders, J. B., & Monteiro, M. G. (2001). AUDIT: The Alcohol Use Disorders Identification Test: Guidelines for Use in Primary Care (2nd ed.). World Health Organization, AUDIT items 1-3: consumption questions; Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, Avoiding leading questions

Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.7 Eliciting sensitive or confidential information (substance use, sexual abuse) · **Qualifier** BEST · **Cognitive** application · **Difficulty** 1 of 3

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42. INT-1031 (rev 4) · Best answer: A

A client in an outpatient anger management group has started taking time-outs during arguments by walking out of the room. The client reports that the adult sibling they live with follows them, shouting, 'Don't walk away from me,' and the argument gets worse. The client says the sibling would be willing to talk about it at a calm time. How can the social worker **BEST** help the client handle these arguments?

A. Agree on a time-out signal and return plan with the sibling
B. Announce a return time to the sibling while walking out
C. Leave without a word to the sibling until fully calm

Why each option is right or wrong

A **Correct.** A formal time-out works best when it is agreed on in advance with the people involved: a signal that means 'I need a break,' how long the break will last, and a commitment to come back and finish or reschedule the talk (Reilly & Shopshire, 2019). The sibling reads the walk-out as being shut out, so they pursue and the argument escalates. Planning the time-out together removes that threat. It beats B because a return time announced mid-argument is a promise the sibling never agreed to, so the exit can still feel like being shut out; planning the signal and return together ahead of time removes that threat.

B **Incorrect** (trap: Misuses the technique). Naming a return time is better than a silent exit and moves toward a real time-out. Announced in the heat of the argument, though, it is a surprise the sibling has not agreed to, so the pursuit may continue. Time-outs work best when the signal and return are planned with the other person in advance.

C **Incorrect** (trap: Misuses the technique). Leaving can be part of a time-out, but an unexplained exit is what already sets off the sibling's pursuit. Without a signal and a plan to return, the break looks like shutting the sibling out.

Takeaway: Time-outs work best when agreed on in advance: a signal, a set break, and a commitment to return to the discussion.

Sources: Reilly, P. M., & Shopshire, M. S. (2019). Anger Management for Substance Use Disorder and Mental Health Clients: A Cognitive-Behavioral Therapy Manual. SAMHSA (PEP19-02-01-001), Session 3: anger control plans (formal and informal timeouts)

Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.20 Helping clients manage anger · **Qualifier** BEST · **Cognitive** application · **Difficulty** 2 of 3

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43. ETH-1078 (rev 2) · Best answer: A

A researcher designs an online survey that collects no names, contact details, IP addresses, or linking codes. Which protection does this design provide?

A. Anonymity
B. Confidentiality
C. De-identification

Why each option is right or wrong

A **Correct.** Anonymity means no one, including the researcher, can link responses to an individual, because identifying information is never collected. That matches a survey with no names, contact details, IP addresses or codes. NASW Code 5.02(m) asks researchers to protect participants' anonymity or confidentiality. A beats C because de-identification strips identifiers from data that were collected with them, whereas here identifiers never existed. Data without identifiers are not identifiable private information (45 CFR 46.102(e)(5)).

B **Incorrect** (trap: Wrong model or concept). Confidentiality means the researcher can identify participants but promises not to reveal who said what. This survey cannot identify anyone, so the stronger protection, anonymity, applies.

C **Incorrect** (trap: Wrong model or concept). De-identification removes identifiers after data are collected, so the original data could once be linked to people. Here identifiers are never gathered at all.

Takeaway: Anonymity means no one can link data to a person; confidentiality means the researcher can but protects the link.

Sources: NASW Code of Ethics (2021), 5.02(m); 45 CFR Part 46 (Common Rule), 45 CFR 46.102(e)(5)
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.11 Research ethics: IRBs, human subjects protections, consent · **Qualifier** none · **Cognitive** recall · **Difficulty** 1 of 3

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44. ASM-1020 (rev 2) · Best answer: B

A school social worker completes an assessment of a 15-year-old student who identifies as nonbinary and uses they/them pronouns. The student has close friends, steady grades and says they feel comfortable with who they are. The student's parent asks the social worker for a diagnosis 'so we know what is wrong.' What is the **MOST** appropriate response?

A. Diagnose gender dysphoria based on the gender incongruence
B. Explain that a diagnosis requires distress or impairment
C. Refer the student to a gender clinic for a diagnosis

Why each option is right or wrong

A **Incorrect** (trap: Near-miss diagnosis). Incongruence between experienced and assigned gender is only part of the picture. Gender dysphoria also requires clinically significant distress or impairment, which this assessment did not find, so the diagnosis would mislabel a student who is doing well.

B **Correct.** Gender dysphoria requires a marked incongruence between experienced and assigned gender lasting at least 6 months, with clinically significant distress or impairment (DSM-5-TR). DSM-5-TR notes that gender nonconformity is not in itself a mental disorder. This student has friends, steady grades and comfort with their identity, so no diagnosis applies, and explaining this to the parent is accurate and respects the student. It beats C because referring for diagnosis implies a disorder the assessment did not find; a gender clinic referral fits if the student wants support or later reports distress.

C **Incorrect** (trap: Hands off too soon). Referring for diagnosis implies the student has a disorder the assessment did not find. Specialized gender services fit if the student wants them or later reports distress.

Takeaway: Gender diversity is not a disorder; gender dysphoria requires clinically significant distress or impairment.

Sources: DSM-5-TR (APA, 2022), Gender Dysphoria: Gender Dysphoria
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.9 DSM use in assessment; common mental and brain-related condition indicators · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 1 of 3

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45. ASM-1118 (rev 2) · Best answer: A

A hospital patient with advanced major neurocognitive disorder needs a decision about a feeding tube. The team's assessment finds the patient cannot describe the choice or its risks after repeated simple explanations. The patient's health care agent, named in an advance directive, reports that the patient often said they never wanted 'to be kept alive by machines.' A nurse suggests seeking a court-appointed guardian. What should the social worker do **NEXT**?

A. Help the agent decide based on the patient's stated wishes
B. Support seeking a court guardian to replace the agent
C. Request an ethics consultation to settle the patient's care

Why each option is right or wrong

A **Correct.** When a patient lacks capacity for a decision, a valid health care agent decides using substituted judgment: what the patient would choose, based on known wishes and values (Beauchamp & Childress, 2019). The team found the patient cannot understand the choice, and the agent reports a clear, repeated wish to avoid life support by machines. Helping the agent apply that wish honors the patient's self-determination through the person the patient chose. It beats B because guardianship is meant for patients with no valid or suitable surrogate, and this patient chose an agent who knows their wishes. A best-interest standard would apply only if those wishes were unknown.

B **Incorrect** (trap: Misreads law or ethics). Guardianship is a court process for people with no valid surrogate or with a surrogate who is unsuitable. This patient named an agent in an advance directive, so a petition adds delay and cost and sidelines the patient's own choice of decision-maker.

C **Incorrect** (trap: Good but not best). Ethics consultation helps when surrogates and teams disagree or a patient's wishes are unclear. Here a valid agent reports clear, repeated wishes and no one disputes the agent's authority, so sending the decision to a committee adds delay and sidelines the decision-maker the patient chose.

Takeaway: A valid health care agent decides for a patient without capacity by applying the patient's known wishes; guardianship is then unnecessary.

Sources: Beauchamp, T. L., & Childress, J. F. (2019). Principles of Biomedical Ethics (8th ed.). Oxford University Press, Surrogate decision making: substituted judgment and best interests
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.23 Client capacity, self-determination, self-monitoring (consent age, emancipation, permanency) · **Qualifier** NEXT · **Cognitive** reasoning · **Difficulty** 3 of 3

46. ETH-1183 (rev 2) · Best answer: B

A colleague at a family service agency has filed a formal grievance over a demotion. The director tells the social worker that the colleague's former supervisory post is open and asks whether the social worker knows 'anything that would help the agency's case.' The social worker wants the post. Which response is **MOST** consistent with the social worker's ethical responsibilities?

A. Limit any comments to facts the social worker saw firsthand
B. Apply on own merits without commenting on the dispute
C. Withdraw from consideration out of loyalty to the colleague

Why each option is right or wrong

A **Incorrect** (trap: Boundary slip or collusion). Sticking to firsthand facts sounds fair and cooperative. Offered to the decision maker in a job conversation while the social worker wants the post, it still trades on the dispute (2.04(a)) and can shade into unwarranted criticism of a colleague (2.01(b)). If called as a witness in the formal grievance, truthful testimony is appropriate.

B **Correct.** The Code states that social workers should not take advantage of a dispute between a colleague and an employer to obtain a position or advance their own interests (2.04(a)). Applying for an open post is legitimate; supplying damaging information to the side that controls the post, while wanting it, is the exploitation the standard describes. Applying on merit and staying out of the grievance meets both interests. It beats A because even firsthand facts, handed to the person who controls the post while the worker seeks it, use the dispute for advantage; facts the grievance needs can come through its formal process.

C **Incorrect** (trap: Good but not best). Loyalty is admirable, but the Code does not bar applying for an open position. Withdrawing is unnecessary when the social worker can pursue the job fairly and simply decline to take part in the dispute.

Takeaway: Pursue opportunities on your own merits; never use a colleague's dispute with an employer to advance your own interests.

Sources: NASW Code of Ethics (2021), 2.04(a); NASW Code of Ethics (2021), 2.01(b)
Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.2 Legal-ethical duties to clients, colleagues, profession, and society · **Qualifier** MOST · **Cognitive** application · **Difficulty** 2 of 3

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47. ASM-1186 (rev 2) · Best answer: B

Two neighboring families lose their homes in the same river flood. One family starts rebuilding within weeks. The other family received the larger insurance payout, and relatives nearby offer it rooms, yet its daily routines collapse. The parents tell a disaster social worker, 'This proves we were always going to lose everything.' In Hill's ABC-X model, which factor **MOST** likely explains the difference?

A. The family's existing resources
B. The family's definition of the event
C. The pile-up of later stressors

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Resources (the B factor) often explain different outcomes, and they are the strongest alternative here. But the family in crisis has the larger insurance payout and relatives offering rooms, so a lack of resources cannot explain why it is the one that falls apart.

B **Correct.** Hill's (1949) ABC-X model holds that a crisis (X) results from the stressor (A), the family's resources (B) and the family's definition of the event (C). With the same flood and ample resources, the family that reads the loss as proof of lifelong failure is the one in crisis, so the C factor explains the difference. It beats A because the family in crisis has the larger payout and relatives' help, so resources do not explain its collapse.

C **Incorrect** (trap: Wrong model or concept). Pile-up of later stressors is the double A factor added in McCubbin and Patterson's (1983) Double ABCX model. It describes demands that build up over time after a crisis, and no later stressors are described here.

Takeaway: In the ABC-X model, how a family defines a stressor helps decide whether the same event becomes a crisis.

Sources: Hill, R. (1949). Families Under Stress. Harper & Brothers, ABC-X model; McCubbin, H. I., & Patterson, J. M. (1983). The family stress process: The Double ABCX model of adjustment and adaptation. Marriage & Family Review, 6(1-2), 7-37, whole article

Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.1 Trauma, stress, violence, crisis: signs and impact (historical, complex) · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 1 of 3

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48. ASM-1635 (rev 1) · Best answer: A

A hospital social worker is helping an 8-year-old patient get ready for a scheduled surgery. The child asks many questions and wants to know exactly what will happen. Which approach is **BEST** suited to the child's stage of thinking?

A. Show the real mask and gown and walk through each step

B. Discuss possible risks and what happens if complications occur

C. Tell a story about a brave animal cured by magic medicine

Why each option is right or wrong

A **Correct.** Children of about 7 to 11 are typically in Piaget's concrete operational stage: they reason logically about real, observable things but struggle with abstract and hypothetical ideas. Handling the actual equipment and walking through the sequence gives the 8-year-old concrete, accurate information. Children's ideas about illness develop along the same stages (Bibace & Walsh, 1980). It beats B because talking through possible complications asks the child to reason about hypothetical events, a formal operational skill, and may raise fear without adding understanding (Piaget & Inhelder, 1969).

B **Incorrect** (trap: Wrong model or concept). Adolescents and adults who reason abstractly may want to discuss risks and contingencies. For an 8-year-old, hypothetical complications are hard to process and can raise anxiety without improving understanding.

C **Incorrect** (trap: Wrong model or concept). Stories with magical solutions fit the preoperational thinking of preschoolers. An 8-year-old who asks exact questions wants factual, concrete answers, and a magic story can leave the child confused or mistrustful.

Takeaway: Match explanations to cognitive stage: concrete, hands-on, step-by-step information for school-age children; abstract risk discussion suits formal operational thinkers.

Sources: Piaget, J., & Inhelder, B. (1969). The Psychology of the Child. Basic Books, concrete operations; Bibace, R., & Walsh, M. E. (1980). Development of children's concepts of illness. Pediatrics, 66(6), 912-917, whole article
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.16 Typical vs. atypical development across the lifespan · **Qualifier** BEST · **Cognitive** application · **Difficulty** 1 of 3

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49. ASM-1142 (rev 1) · Best answer: A

PRETEST — unscored on this form; do not count it

A housing inspector refers a 52-year-old tenant to a community agency. Stacks of newspapers, bags and boxes fill every room, and the tenant sleeps in a chair because the bed is buried. The tenant becomes very upset at the idea of discarding anything, saying each item 'might be needed someday.' Which diagnosis is **MOST** likely?

A. Hoarding disorder
B. Obsessive-compulsive personality disorder
C. Obsessive-compulsive disorder

Why each option is right or wrong

A **Correct.** Hoarding disorder involves persistent difficulty discarding possessions, driven by a perceived need to save them and distress at parting with them, so that clutter blocks the use of living areas. The tenant's distress at discarding, the 'might be needed' reasoning and a buried bed fit this pattern. It beats C because in OCD, saving follows typical obsessions such as fear of harm or contamination, whereas this tenant saves items for their possible usefulness (DSM-5-TR).

B **Incorrect** (trap: Near-miss diagnosis). OCPD can include reluctance to throw out worn-out objects, but it is a broad pattern of perfectionism, rigidity and control. Saving with clutter that fills the home, without that wider pattern, points to hoarding disorder.

C **Incorrect** (trap: Near-miss diagnosis). In OCD, saving happens as part of typical obsessions or compulsions, such as fear that discarding will cause harm. The tenant saves because items may be useful, the reasoning typical of hoarding disorder.

Takeaway: Distress at discarding and saving 'just in case' until living space is unusable define hoarding disorder, separate from OCD and OCPD.

Sources: DSM-5-TR (APA, 2022), Obsessive-Compulsive and Related Disorders: Hoarding Disorder
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.9 DSM use in assessment; common mental and brain-related condition indicators · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 1 of 3

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50. ASM-1686 (rev 1) · Best answer: A

A 20-month-old was present during several violent incidents between caregivers before the nonoffending caregiver moved with the toddler to a relative's home. Since then, the toddler startles at loud voices, wakes screaming most nights, and clings constantly. The caregiver is also on edge and struggles to soothe the toddler. Which treatment model is the **BEST** fit?

A. Child-parent psychotherapy
B. Parent-child interaction therapy
C. Trauma-focused cognitive behavioral therapy
D. Nondirective individual play therapy

Why each option is right or wrong

A **Correct.** Child-parent psychotherapy is a relationship-based treatment for children from birth to about age 5 who have faced trauma such as family violence (Lieberman & Van Horn, 2008). It treats the toddler and caregiver together, helping the caregiver understand and respond to the child's fear while attending to the caregiver's own trauma. The age, the shared exposure and the caregiver's trouble soothing all point here. It beats B, the strongest competitor, because parent-child interaction therapy coaches parenting skills for disruptive behavior in somewhat older children rather than working through trauma in the relationship (McNeil & Hembree-Kigin, 2010).

B **Incorrect** (trap: Wrong model or concept). This model coaches caregivers live to strengthen the relationship and manage disruptive behavior, usually with preschoolers (McNeil & Hembree-Kigin, 2010). It does not center on trauma or the caregiver's own reactions, and this toddler is younger than its usual range.

C **Incorrect** (trap: Wrong model or concept). Trauma-focused CBT is a strong choice for traumatized children from about age 3 (Cohen, Mannarino & Deblinger, 2017). It relies on coping skills and a trauma narrative that a 20-month-old cannot yet use, even with caregiver involvement.

D **Incorrect** (trap: Wrong model or concept). Individual play therapy works with the child alone and leaves out the caregiver relationship, which is the toddler's main source of safety. At this age, treating the caregiver and child together fits better.

Takeaway: For infants and toddlers exposed to trauma, choose a dyadic treatment such as child-parent psychotherapy that works through the caregiver relationship.

Sources: Lieberman, A. F., & Van Horn, P. (2008). Psychotherapy with Infants and Young Children: Repairing the Effects of Stress and Trauma on Early Attachment. Guilford Press, child-parent psychotherapy: population and approach; McNeil, C. B., & Hembree-Kigin, T. L. (2010). Parent-Child Interaction Therapy (2nd ed.). Springer, overview of PCIT; Cohen, Mannarino & Deblinger (2017). Treating Trauma and Traumatic Grief in Children and Adolescents (2nd ed.). Guilford Press, TF-CBT: appropriate ages

Area II Assessment and Planning · **Sub-area** II.C Assessment Practices · **Topic** II.C.3 Choosing intervention modality by ability, development, culture, life stage · **Qualifier** BEST · **Cognitive** recall · **Difficulty** 3 of 3

51. INT-1504 (rev 1) · Best answer: C

In a juvenile detention center, a 16-year-old describes assaulting another youth. The social worker says plainly that the assault caused real harm. In the same meeting and every meeting after, the worker treats the youth as a person of worth who deserves care. Which element of Rogers' person-centered approach does the worker's stance toward the youth **MOST** likely show?

A. Congruence
B. Accurate empathy
C. Unconditional positive regard

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Congruence is genuineness: the worker's outward responses match inner experience. Naming the harm plainly is congruent, which makes this option tempting, but the stance the question asks about is the steady valuing of the youth, not the worker's transparency.

B **Incorrect** (trap: Wrong model or concept). Accurate empathy is understanding the client's inner world and conveying that understanding. The stem describes how the worker values the youth, not how the worker grasps the youth's feelings.

C **Correct.** Rogers (1957) named unconditional positive regard as a core condition: the worker prizes the client as a person without making that acceptance depend on behavior. The worker condemns the act yet keeps treating the youth as worthy of care, which separates the person from the behavior. It beats A because naming the harm honestly reflects congruence, but the continuing valuing of the youth is the defining feature here.

Takeaway: Unconditional positive regard values the client as a person while the worker can still name harmful behavior honestly.

Sources: Rogers, C. R. (1957). The necessary and sufficient conditions of therapeutic personality change. Journal of Consulting Psychology, 21(2), 95-103, core conditions
Area III Intervention and Practice · **Sub-area** III.A Practice Concepts · **Topic** III.A.2 Building and sustaining the helping relationship (empathy, rapport, acceptance) · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 2 of 3

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52. INT-1650 (rev 1) · Best answer: B

A client with schizophrenia was discharged from an inpatient unit 4 days ago to supported housing and missed the first clinic appointment yesterday. The social worker has called twice this afternoon with no answer. Housing staff report the client has not left the room in 2 days, has refused meals, and can be heard shouting when no one else is there. What should the social worker do **NEXT**?

A. Notify the outpatient prescriber about the missed appointment
B. Request a mobile crisis team visit to the client today
C. Ask housing staff to urge the client to call the clinic
D. Schedule a home visit with the client for later this week

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). The prescriber should hear about the missed visit and the staff report, and the social worker will share them. Notifying the prescriber does not, by itself, get anyone to the client today, and two days of refused meals and shouting at no one need same-day eyes on the client.

B **Correct.** The weeks after psychiatric discharge carry sharply raised risk, so failed outreach plus signs of decompensation calls for prompt in-person evaluation (Chung et al., 2017). Here the phone attempts have failed, and staff describe isolation, refused meals, and apparent responses to voices. A mobile crisis team can assess the client where they live today, a less restrictive response than police or an ambulance (SAMHSA, 2020). It beats C because asking a client who appears to be decompensating to make the call leaves the next step to the person least able to take it.

C **Incorrect** (trap: Ignores risk). Staff encouragement suits a stable client who simply forgot an appointment. This client is isolating, refusing food, and appears to be responding to voices. Waiting for that client to call puts the next step on the person least able to take it.

D **Incorrect** (trap: Ignores risk). An in-person check is the right kind of response, but later this week is too slow. Refused meals and signs of psychosis during the high-risk period after discharge call for evaluation today, not a routine visit.

Takeaway: When outreach after discharge fails and signs of decompensation appear, arrange same-day in-person crisis evaluation rather than more routine contact.

Sources: Chung, D. T., Ryan, C. J., Hadzi-Pavlovic, D., Singh, S. P., Stanton, C., & Large, M. M. (2017). Suicide rates after discharge from psychiatric facilities: A systematic review and meta-analysis. JAMA Psychiatry, 74(7), 694-702, whole article; SAMHSA (2020). National Guidelines for Behavioral Health Crisis Care: Best Practice Toolkit, Mobile crisis team services

Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.22 Discharge planning, aftercare, wraparound, follow-up · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 2 of 3

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53. INT-0009 (rev 1) · Best answer: C

A supervisee who is usually confident becomes hesitant in supervision and repeatedly asks the supervisor to say exactly what to do. The supervisee is working with a client who feels helpless and keeps asking the supervisee for answers. What is the supervisor's **BEST** response?

A. Give step-by-step directions for the next session

B. Recommend personal therapy for the supervisee's dependence

C. Explore whether the case dynamic is being replayed between them

D. Note a performance concern in the supervisee's evaluation

Why each option is right or wrong

A **Incorrect** (trap: Boundary slip or collusion). Detailed directions may solve the next session but repeat the dynamic the client has set up: a helpless person being rescued. They also bypass the supervisee's own clinical thinking.

B **Incorrect** (trap: Hands off too soon). Personal therapy may help if a lasting personal issue emerges. Recommending it now treats a common, case-related supervisory dynamic as a personal problem before it has been explored.

C **Correct.** The supervisee is reenacting with the supervisor the same helpless, answer-seeking stance the client takes with the supervisee. This is parallel process (Searles, 1955). Naming and exploring the parallel helps the supervisee understand the client's experience and regain footing, and it uses the supervisory relationship as information (Kadushin & Harkness, 2014). It beats A because step-by-step answers repeat the rescuing dynamic and leave the supervisee as dependent on the supervisor as the client is on the supervisee.

D **Incorrect** (trap: Skips assessment). Recording a performance concern assumes a skill deficit before the supervisor understands the change. A sudden shift in a usually confident supervisee points to a case dynamic worth exploring first.

Takeaway: When a supervisee re-enacts the client's dynamic with the supervisor, name and explore the parallel process rather than acting it out.

Sources: Searles, H. F. (1955). The informational value of the supervisor's emotional experiences. Psychiatry, 18(2), 135-146, parallel process; Kadushin, A., & Harkness, D. (2014). Supervision in Social Work (5th ed.). Columbia University Press, educational supervision

Area III Intervention and Practice · **Sub-area** III.D Supervision and Administration · **Topic** III.D.3 Transference and countertransference in supervision · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

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54. INT-1050 (rev 2) · Best answer: C

At the first family session, the parents of a 16-year-old who is skipping school say, 'You're the expert. Just tell us what to do and fix our kid.' The social worker replies that only the family can decide what they want to change and how hard they will work at it. This response **MOST** likely reflects which concept?

A. Battle for structure
B. Restraining change
C. Battle for initiative
D. Positive connotation

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). The battle for structure is Whitaker's other early battle: the social worker sets the terms of therapy, such as who attends and when sessions occur (Napier & Whitaker, 1978). This response concerns who owns the motivation for change, not the ground rules of therapy.

B **Incorrect** (trap: Wrong model or concept). Restraining change is a strategic technique in which the worker warns the family to go slowly or names the dangers of improving, expecting them to push back. Here the worker does not discourage change; they place responsibility for it with the family.

C **Correct.** Whitaker's battle for initiative holds that the family, not the social worker, must own the motivation and energy for change; a therapist who supplies the drive leaves the family passive. Declining to 'fix our kid' and handing the question of what they want back to the parents is this stance (Nichols & Davis, 2020). It beats B because restraining change is a strategic paradox that cautions a family to go slow or warns of the risks of change. Here the worker is not discouraging change but insisting that the family lead it.

D **Incorrect** (trap: Wrong model or concept). Positive connotation is a Milan technique that gives a positive, system-serving meaning to every member's behavior, including the symptom. The worker here offers no new meaning for the school skipping; they decline to take over the family's role in changing it.

Takeaway: In Whitaker's model, the therapist wins the battle for structure but leaves the battle for initiative to the family, who must own the drive to change.

Sources: Nichols, M. P., & Davis, S. D. (2020). Family Therapy: Concepts and Methods (12th ed.). Pearson, symbolic-experiential family therapy: battle for structure and battle for initiative; Napier, A. Y., & Whitaker, C. A. (1978). The Family Crucible. Harper & Row, the battle for structure and the battle for initiative

Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.21 Family therapy models and interventions · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 2 of 3

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55. INT-1577 (rev 1) · Best answer: D

Two direct support staff at a group home count one resident's aggressive episodes during the same evening shifts. On the same evenings, one records 2 episodes and the other records 7. When asked, one counts only hitting, while the other also counts shouting and slamming doors. Which step is **BEST** for getting usable data?

A. Average the two staff members' counts for each evening

B. Use the counts from the staff member with more experience

C. Switch to a standardized aggression rating scale

D. Write a shared definition with examples and train both staff

Why each option is right or wrong

A **Incorrect** (trap: Misuses the technique). Averaging blends two different measurements into a number that matches neither. It hides the disagreement instead of fixing its cause, so the data would still not reflect one consistent behavior.

B **Incorrect** (trap: Good but not best). Experience does not make one observer's definition correct. Dropping the other count leaves the team relying on a single unchecked observer, with no way to confirm the data are consistent.

C **Incorrect** (trap: Good but not best). A standardized scale can help, but staff would still need shared meanings and training to rate consistently. Switching tools also gives up behavior-specific counts that suit this plan.

D **Correct.** Interrater reliability depends on a clear operational definition of the target behavior and on training observers to apply it the same way (Bloom, Fischer & Orme, 2009). The gap here comes from the two staff counting different behaviors, so a written definition with examples, plus joint practice, addresses the cause. Agreement can then be checked. This beats C because a new instrument does not fix observers who apply different meanings unless they are trained together.

Takeaway: When observers disagree, define the target behavior precisely and train observers together before trusting the data.

Sources: Bloom, M., Fischer, J., & Orme, J. G. (2009). Evaluating Practice: Guidelines for the Accountable Professional (6th ed.). Pearson, reliability: interobserver reliability and operational definitions

Area III Intervention and Practice · **Sub-area** III.C Practice Evaluation and Research · **Topic** III.C.5 Research design, data collection/analysis, reliability, validity · **Qualifier** BEST · **Cognitive** application · **Difficulty** 2 of 3

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56. ASM-1143 (rev 1) · Best answer: C

A primary care physician refers a 76-year-old client after a full workup found no cause for months of fatigue, aches and poor appetite. The client has stopped gardening and no longer meets friends for cards but says, 'I'm not sad, just worn out.' What should the social worker do **NEXT**?

A. Encourage the client to rejoin a gardening group
B. Accept the denial of sadness and focus on coping with pain
C. Assess for depression, including lost interest and hopelessness

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). Reengaging in valued activities is a sound part of treating late-life depression. Suggesting it before assessing treats the withdrawal as a lifestyle issue and may miss depression and any suicide risk.

B **Incorrect** (trap: Good but not best). Denial of sadness does not rule out depression in later life. Focusing on pain coping accepts the physical framing and leaves the loss of interest unexamined.

C **Correct.** Older adults with depression often deny sadness and present instead with physical complaints, fatigue and loss of interest (Gallo & Rabins, 1999). DSM-5-TR allows loss of interest or pleasure, without depressed mood, to anchor a major depressive episode. With medical causes ruled out and clear withdrawal from valued activities, the social worker should assess for depression, including hopelessness and suicidal thoughts. It beats A because activity can help once an assessment guides the plan, but prompting first may miss a treatable and risky illness.

Takeaway: Older adults may show depression through physical complaints and lost interest without sadness; assess for depression once medical causes are excluded.

Sources: Gallo, J. J., & Rabins, P. V. (1999). Depression without sadness: Alternative presentations of depression in late life. American Family Physician, 60(3), 820-826, whole article; DSM-5-TR (APA, 2022), Depressive Disorders: Major Depressive Disorder
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.5 Signs of mental and emotional illness across the lifespan · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 1 of 3

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57. ASM-0005 (rev 1) · Best answer: B

During a routine session, a 70-year-old client says, 'Lately I've been thinking about ending my life.' The social worker thanks the client for sharing and stays calm. What should the social worker do **NEXT**?

A. Begin a written safety plan with coping strategies
B. Ask about a plan, intent and access to lethal means
C. Arrange an emergency psychiatric evaluation later today

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). A safety plan is a core intervention, but writing it before knowing whether there is a plan, intent or access to means skips the assessment it should rest on.

B **Correct.** Once a client discloses suicidal thoughts, the social worker completes the risk assessment by asking directly about a plan, intent, preparation and access to lethal means. The answers set the level of risk and the response (Posner et al., 2011). Asking

directly does not increase suicidal thinking (Dazzi et al., 2014). It beats A because safety planning is an important intervention, but it should be built on what the assessment shows. For example, an older adult's access to firearms or stored medications would shape the plan.

C **Incorrect** (trap: Skips assessment). Emergency evaluation is right if the assessment shows intent, a plan or imminent danger. Arranging it now escalates before the social worker knows the level of risk.

Takeaway: After a suicidal disclosure, finish the risk assessment, including plan, intent and means, before choosing safety planning or emergency action.

Sources: Posner, K., et al. (2011). The Columbia-Suicide Severity Rating Scale: Initial validity and internal consistency findings from three multisite studies with adolescents and adults. American Journal of Psychiatry, 168(12), 1266-1277, whole article; Dazzi, T., Gribble, R., Wessely, S., & Fear, N. T. (2014). Does asking about suicide and related behaviours induce suicidal ideation? What is the evidence? Psychological Medicine, 44(16), 3361-3363, whole article

Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.1 Assessing risk of harm to self or others · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 1 of 3

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58. ASM-0101 (rev 3) · Best answer: D

Parents in a high-conflict marriage each press their 14-year-old to agree that the other parent is at fault. Each parent also asks the teen to carry messages they refuse to say to each other. The teen agrees with whichever parent is talking and now has frequent stomachaches. Which family dynamic is **MOST** likely present?

A. Enmeshment
B. Cross-generational coalition
C. Emotional cutoff
D. Triangulation

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Enmeshment describes diffuse boundaries across a family, with little privacy or autonomy for its members. The stem shows a specific pattern of routing parental conflict through one child, which triangulation captures more precisely.

B **Incorrect** (trap: Wrong model or concept). In a cross-generational coalition, one parent and a child form a stable alliance against the other parent. Here both parents compete for the teen, who agrees with each in turn, so no lasting alliance with one side has formed.

C **Incorrect** (trap: Wrong model or concept). Emotional cutoff is managing unresolved family tension by distancing or ending contact. These family members are in close, intense contact through the teen, which is the opposite of cutoff.

D **Correct.** Triangulation, a core concept in Bowen theory, occurs when two people in conflict pull a third person in to lower the tension between them (Bowen, 1978). Both parents route their conflict through the teen by demanding agreement and using the teen as a messenger, and the stomachaches reflect the strain. It beats B because a cross-generational coalition is a stable alliance of one parent and a child against the other parent, while this teen is pulled by both sides and agrees with each in turn (Minuchin, 1974).

Takeaway: Triangulation is drawing a third person into a two-person conflict; a coalition is a parent and child allied against the other parent.

Sources: Bowen, M. (1978). Family Therapy in Clinical Practice. Jason Aronson, triangles; Minuchin, S. (1974). Families and Family Therapy. Harvard University Press, coalitions and boundaries
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.2 Family functioning and dynamics and their effects · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 2 of 3

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59. INT-1117 (rev 1) · Best answer: C

In an Illness Management and Recovery group at a community mental health center, the social worker has been reading each module's handout aloud. A member with schizophrenia who reads at a basic level says the material 'goes right over my head' and has started missing sessions. What is the **MOST** helpful adjustment?

A. Assign the full handout as homework between sessions
B. Refer the member to individual therapy instead
C. Teach interactively with pictures and frequent review
D. Give a printed summary written at a lower reading level

Why each option is right or wrong

A **Incorrect** (trap: Misuses the technique). Reading the full handout alone at home asks more of the member's literacy, not less, and removes the support of the group. Between-session practice in IMR is short and tailored, not extra reading.

B **Incorrect** (trap: Hands off too soon). Individual work may suit some people, but the problem here is how the material is taught, not the group itself. Moving the member out first skips a teaching change that could keep the member engaged with peers.

C **Correct.** Illness Management and Recovery delivers psychoeducation through interactive teaching combined with motivational and cognitive-behavioral strategies, tailored to each person's reading ability, prior knowledge and goals, with review of earlier material and visual supports (Mueser et al., 2006). Reading a handout aloud is passive and depends on literacy, and the member's missed sessions show it is not working. Teaching with pictures, frequent review and links to the member's own recovery goals makes the content usable. It beats D because an easier summary helps, but it keeps learning passive and print-based for a member who is already disengaging.

D **Incorrect** (trap: Good but not best). Matching written material to reading level is a sound adaptation and can be part of the fix. On its own, it keeps learning passive and print-based, so it does less to re-engage a member who is already missing sessions.

Takeaway: Psychoeducation for people with serious mental illness works best when interactive, reviewed often, visually supported and tailored to the person.

Sources: Mueser, K. T., Meyer, P. S., Penn, D. L., Clancy, R., Clancy, D. M., & Salyers, M. P. (2006). The Illness Management and Recovery program: Rationale, development, and preliminary findings. Schizophrenia Bulletin, 32(Suppl. 1), S32-S43, teaching strategies: educational, motivational, cognitive-behavioral
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.14 Delivering psychoeducation · **Qualifier** MOST · **Cognitive** application · **Difficulty** 3 of 3

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60. ETH-1178 (rev 3) · Best answer: C

A hospice patient with full decision-making capacity wants to ride 300 miles by car to see a sibling one last time. The nurse has told the patient that the drive will be exhausting and could worsen symptoms. The team asks the social worker, who has not yet met the patient, to talk the patient out of it. What should the social worker do **FIRST**?

A. Suggest a video visit instead, as a safer way to see the sibling
B. Help the patient plan the trip with the team's precautions
C. Explore with the patient what the visit means to them

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). A video visit can be meaningful and may be offered as one option. Proposing it before hearing what the in-person visit means steers the patient toward the team's preference instead of the patient's own goals.

B **Incorrect** (trap: Right action, wrong time). Helping a capable patient carry out a risky choice as safely as possible, with rest stops and symptom medication, fits hospice values and is likely to follow. It comes after the social worker has heard what the visit means to the patient and how the patient weighs the risks.

C **Correct.** In hospice, care follows what matters most to the patient, and a capable patient's choice deserves to be understood before it is supported or questioned (NASW Code 1.02; Ethical Principles: Dignity and Worth of the Person). The social worker has not yet spoken with the patient and the team wants persuasion, so learning what the visit means comes first; it also equips the worker to help the team see the choice clearly. It beats B because planning the trip is likely the next step, but planning before hearing the patient skips the conversation that grounds the plan in the patient's goals.

Takeaway: When a capable hospice patient makes a risky choice, first learn what it means to the patient, then help carry it out as safely as possible.

Sources: NASW Code of Ethics (2021), 1.02; NASW Code of Ethics (2021), Ethical Principles: Dignity and Worth of the Person
Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.3 Recognizing and working through ethical dilemmas · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 2 of 3

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61. ETH-0109 (rev 1) · Best answer: C

How should a social worker fix a mistake discovered in a completed progress note?

A. Replace the note with a corrected version
B. Mention the error in the next progress note entry
C. Add a dated, signed addendum that corrects it

Why each option is right or wrong

A **Incorrect** (trap: Misreads law or ethics). Replacing a note erases the original, which looks like alteration of the record and destroys the audit trail. Even well-meant changes made this way can be treated as falsification in an audit or legal review.

B **Incorrect** (trap: Good but not best). Noting the error elsewhere is honest, but the original entry stays unchanged and unlinked. A reader of the earlier note would still see the wrong information, so the record as a whole remains inaccurate.

C **Correct.** Records must be accurate and reflect the services actually provided (NASW Code 3.04(a)). Errors are corrected by a dated, signed addendum or late entry that leaves the original entry readable, so anyone reviewing the record can see what changed and when. HIPAA's amendment rule follows the same logic, appending or linking a correction rather than erasing the original (45 CFR 164.526(c)(1)). It beats B because a later note may never be read alongside the original, which would still appear uncorrected.

Takeaway: Correct records with a dated, signed addendum that leaves the original readable; never delete or overwrite entries.

Sources: NASW Code of Ethics (2021), 3.04(a); HIPAA Privacy Rule, 45 CFR 164.526(c)(1)
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.4 Legal and ethical record keeping and documentation · **Qualifier** none · **Cognitive** recall · **Difficulty** 1 of 3

Section 2

62. ASM-1063 (rev 2) · Best answer: A

An inpatient is asked how the morning went and answers rapidly: 'Grey morning, grey like the sea, I sailed once, sailors steer by the stars, I'm writing a book on astronomy.' The answer continues in the same way for several minutes. Which MSE finding does this answer **MOST** likely show?

A. Flight of ideas
B. Loose associations
C. Circumstantiality
D. Clang associations

Why each option is right or wrong

A **Correct.** Flight of ideas is a thought-process finding in which speech moves rapidly from topic to topic, each shift linked to the one before by an understandable association, as with grey, sea, sailing, stars and astronomy. It is typical of mania and usually comes with pressured speech. It beats B, loose associations, because in loosening of associations the connections between ideas cannot be followed, whereas here each link is clear even though the answer never returns to the question (Kaplan & Sadock).

B **Incorrect** (trap: Wrong model or concept). In loose associations, thoughts shift between topics with no understandable connection, as is often seen in schizophrenia. In this answer each phrase follows from the one before, so the links can be traced.

C **Incorrect** (trap: Wrong model or concept). Circumstantial speech takes a long, detailed detour but eventually returns to answer the question. This answer moves further away with each phrase and never gets back to the morning.

D **Incorrect** (trap: Wrong model or concept). Clang associations link words by sound, such as rhymes or puns, rather than by meaning. Here the links follow meaning, from sea to sailing to stars, not sound.

Takeaway: Rapid topic shifts with traceable links are flight of ideas; untraceable links are loose associations; detours that return are circumstantiality.

Sources: Boland, Verduin & Ruiz (Eds.). Kaplan & Sadock's Synopsis of Psychiatry (12th ed.), Mental status examination: thought process
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.10 Mental status exam: components and purpose · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 3 of 3

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63. INT-1609 (rev 2) · Best answer: D

An associate social worker at a substance use treatment program has two supervisors. The program manager provides administrative supervision, and an outside licensed social worker provides clinical supervision for licensure. For one client, the manager directs weekly drug screens, while the clinical supervisor advises pausing screens to rebuild trust. Neither supervisor knows of the other's direction. What should the associate do **FIRST**?

A. Follow the program manager, who represents the employer
B. Follow the clinical supervisor, who oversees licensure hours
C. Seek an outside consultant's opinion on drug screening
D. Ask both supervisors to resolve the conflict together

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). When no agreement exists, the employment supervisor may have the final say, so the screens may end up following the manager. Complying before either supervisor knows of the conflict skips that step and leaves the clinical supervisor unaware of a change in the client's care.

B **Incorrect** (trap: Good but not best). The clinical supervisor's advice carries weight, but the program manager has legitimate authority over program procedures. Setting aside the manager's direction without discussion bypasses the agreed channels.

C **Incorrect** (trap: Right action, wrong time). Outside consultation can help if the supervisors cannot agree. As a first step it adds a third voice before the two people responsible for the associate's work have tried to settle it.

D **Correct.** When a supervisee has more than one supervisor, best practice is an agreement that defines each role, information sharing and how conflicts will be resolved (NASW & ASWB supervision standards, 2013, Dual Supervision and Conflict Resolution). Neither supervisor knows of the conflict, so the associate's first step is to bring it to both so they can settle it through that process, rather than choosing a side alone. It beats A because, although the employment supervisor may have the final say when no agreement exists, that call belongs to the supervisors once both know of the conflict; silent compliance leaves the clinical supervisor unaware.

Takeaway: When two supervisors give conflicting directions, the supervisee brings the conflict to both to resolve rather than choosing one silently.

Sources: NASW & ASWB Best Practice Standards in Social Work Supervision (2013), Standard 1 (Context in Supervision): Dual Supervision and Conflict Resolution
Area III Intervention and Practice · **Sub-area** III.D Supervision and Administration · **Topic** III.D.1 Supervisee's role: learning needs, self-assessment, prioritizing · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 3 of 3

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64. INT-1189 (rev 1) · Best answer: A

In a hospital family room, a social worker meets the parent of a 19-year-old who died in a motorcycle crash an hour ago. The parent sits rigidly and says, 'I told them to sell that bike. I should have taken the keys.' What should the social worker do **FIRST**?

A. Invite the parent to keep talking about what they feel
B. Assure the parent that they are blameless for the crash
C. Provide information about bereavement support groups

Why each option is right or wrong

A **Correct.** In the first hour after a sudden death, the priority is a supportive presence that lets the bereaved person express what they feel. Guilt and self-blame are common early grief reactions, and Worden's counseling principles include helping survivors identify and experience such feelings (Worden, 2018). The parent has just voiced self-blame, so inviting more is the first step. It beats B because reassurance, though kind, argues with the parent's feelings before they are heard and can close the conversation; checking guilt against the facts comes later.

B **Incorrect** (trap: Good but not best). Reassurance feels kind, but offered now it dismisses the parent's guilt before it is fully spoken and can end the conversation. Gently testing guilt against the facts belongs later, once the feeling has been heard.

C **Incorrect** (trap: Right action, wrong time). Linking to support groups is a useful later step, often before the family leaves or at follow-up. An hour after the death, resources come before the parent has had any chance to be heard.

Takeaway: Right after a sudden death, let the bereaved voice grief and guilt before reassuring, problem-solving or offering resources.

Sources: Worden, J. W. (2018). Grief Counseling and Grief Therapy (5th ed.). Springer Publishing, Grief counseling principles: help the survivor identify and experience feelings, including guilt
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.10 Working with loss, separation, and grief · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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65. ETH-1569 (rev 1) · Best answer: B

A parent tells a school social worker that their 8-year-old is struggling with reading and asks for a special education evaluation. The school's practice is to complete 12 weeks of classroom reading intervention before considering a referral, and 4 weeks remain. What is the **MOST** helpful response by the social worker?

A. Tell the parent an evaluation can be requested once intervention ends
B. Help the parent submit a written request for an evaluation now
C. Suggest a private evaluation to avoid waiting for the school
D. Ask the teacher to increase the intensity of reading support

Why each option is right or wrong

A **Incorrect** (trap: Misreads law or ethics). Intervention data are useful, but federal guidance says an intervention process cannot delay or deny an evaluation a parent requests. Telling the parent to wait misstates the parent's rights (OSEP Memo 11-07).

B **Correct.** Under the Individuals with Disabilities Education Act, a parent may request an initial evaluation (34 CFR 300.301(b)), and the U.S. Department of Education has

stated that an intervention process cannot be used to delay or deny one (OSEP Memo 11-07, 2011). Putting the request in writing starts the school's required response: it must seek consent to evaluate or give written notice explaining a refusal (34 CFR 300.503). It beats D because stronger reading support helps in any case but does not act on the parent's request.

C **Incorrect** (trap: Hands off too soon). A private evaluation is the family's option, but it costs money and does not replace the school's duty to respond. The parent need not go outside the school to be heard.

D **Incorrect** (trap: Good but not best). More intensive reading support may help and can continue during an evaluation. It does not respond to the parent's request, which the school must act on.

Takeaway: School intervention routines cannot delay a parent's request for a special education evaluation; help families use their federal rights.

Sources: Individuals with Disabilities Education Act regulations, 34 CFR 300.301(b); Individuals with Disabilities Education Act regulations, 34 CFR 300.503; U.S. Department of Education, OSEP Memo 11-07 (2011), RTI process cannot delay or deny an evaluation

Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.8 How local, state, federal policy affects service delivery · **Qualifier** MOST · **Cognitive** application · **Difficulty** 2 of 3

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66. ETH-1016 (rev 3) · Best answer: C

During a video session, a client joins from a shared break room at work, and coworkers can be heard nearby. When the social worker asks whether the client can talk freely, the client says, 'It's fine, nobody pays attention.' The client then begins to describe a recent panic attack. What is the **MOST** helpful response?

A. Continue the session, since the client accepts the risk

B. End the session and require a private space next time

C. Explain the risk of being overheard and offer options

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Respecting the client's choice matters, but the choice is not yet informed: coworkers nearby could overhear a sensitive disclosure. Continuing would fit once the client understands the risk and still chooses to go on.

B **Incorrect** (trap: Overrides the client's choice). Ending the session and setting a rule overrides the client and withholds help in the moment. Offering options lets the client protect their privacy while keeping control of the decision.

C **Correct.** Confidential matters should be discussed only where privacy can be ensured, electronically or in person (NASW Code 1.07(i)), and remote services call for checking that the client understands their risks and limits (NASW Code 1.03(g)). A quick reassurance does not show that the client understands the risk, so the worker explains how the client could be overheard and offers choices, such as moving, using text chat or rescheduling, while leaving the decision with the client. C beats A because continuing honors the client's wish without making sure the choice is informed.

Takeaway: In remote sessions, make sure the client understands privacy risks in their setting and offer options before sensitive work continues.

Sources: NASW Code of Ethics (2021), 1.07(i); NASW Code of Ethics (2021), 1.03(g)
Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.1 Confidentiality law and ethics, its limits, HIPAA/PHI privacy · **Qualifier** MOST · **Cognitive** reasoning · **Difficulty** 3 of 3

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67. ETH-1127 (rev 2) · Best answer: A

PRETEST — unscored on this form; do not count it

A client of a community mental health agency died in an incident covered widely in the news. A local story claims the agency had refused the client services. A reporter calls the social worker who served the client and asks for comment, and the agency director wants the record set straight. How should the social worker respond to the reporter?

A. Decline to discuss whether or how the person received care
B. Share the dates of service to correct the false claim
C. Decline to give details but say the story's claim is untrue

Why each option is right or wrong

A **Correct.** Confidentiality continues after a client dies (NASW Code 1.07(w)), and social workers protect it when the media ask questions (1.07(k)). A false story that harms the agency does not create a compelling reason to disclose; the agency can speak generally about its intake policies without referring to any person. A beats B because dates of service, though limited, confirm that the person was a client and reveal details of care, which is exactly what the Code protects.

B **Incorrect** (trap: Misreads law or ethics). Wanting to correct a false claim is understandable, especially with the agency's reputation at stake. But dates of service confirm that the person was a client. Defending the agency is not a compelling reason to disclose.

C **Incorrect** (trap: Misreads law or ethics). Withholding details feels careful, but calling the claim untrue tells the reporter the agency knows this person's service history, which confirms a relationship. Any agency response must stay general, about intake policy, without reference to the individual.

Takeaway: Confidentiality survives death and covers media requests; agency or worker reputation is not a compelling reason to disclose.

Sources: NASW Code of Ethics (2021), 1.07(w); NASW Code of Ethics (2021), 1.07(k)
Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.1 Confidentiality law and ethics, its limits, HIPAA/PHI privacy · **Qualifier** none · **Cognitive** application · **Difficulty** 2 of 3

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68. INT-1039 (rev 2) · Best answer: A

In a residential program, a 15-year-old practices asking staff for an extra phone call instead of demanding one. In the first role-play, the teen uses a calm voice and says 'please,' but stands very close and talks over the staff member's reply. What should the social worker do **NEXT**?

A. Praise what worked, coach one change, and run it again
B. Model the whole request again with the right distance
C. Assign the teen to try the request with staff later today

Why each option is right or wrong

A **Correct.** In skills training, feedback after a role-play starts with specific praise for what the client did well, adds one or two concrete suggestions, and is followed by another role-play so the client can try the change right away (Bellack et al., 2004). The teen's calm voice and 'please' are real gains worth naming; standing close and interrupting are later targets, taken one at a time. It beats C because real-world practice is the final step, after repeated rehearsal in session has made the new behavior more reliable.

B **Incorrect** (trap: Good but not best). Repeating the demonstration helps a learner who missed the skill entirely. This teen already shows the calm voice and 'please', so a full re-demonstration skips the step that shapes the teen's own attempt: naming those gains, coaching one specific change and rehearsing it right away.

C **Incorrect** (trap: Right action, wrong time). Practice outside the session is the goal, but assigning it after one partly successful rehearsal comes too early. More role-plays with feedback come first, so the teen practices the improved version.

Takeaway: After a role-play, praise specific strengths, give one or two concrete suggestions, and rehearse again before assigning real-life practice.

Sources: Bellack, A. S., Mueser, K. T., Gingerich, S., & Agresta, J. (2004). Social Skills Training for Schizophrenia: A Step-by-Step Guide (2nd ed.). Guilford Press, steps of social skills training: feedback and repeated role-play
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.28 Using role-play in sessions · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 3 of 3

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69. INT-1525 (rev 1) · Best answer: C

A client at a family service agency agreed to an 8-session task-centered contract focused on conflict with a teenage child. At session 5, the client arrives with an eviction notice and says it is all they can think about. The original goals are about half met. What is the social worker's **BEST** response?

A. Keep the original contract and plan to address housing afterward
B. Explore how the eviction connects to the conflict at home
C. Revise the contract with the client to add the housing problem

Why each option is right or wrong

A **Incorrect** (trap: Misuses the technique). Focus matters in time-limited work, but an eviction is an urgent basic need that the client now ranks first. Waiting until the end risks losing housing and the client's engagement. Holding the agreed focus fits when a new issue is minor or can wait.

B **Incorrect** (trap: Good but not best). The stem already shows what the client needs: help with an eviction that dominates their thinking. Linking it to the family conflict may be useful later, but exploring meaning first delays a decision the client has clearly signaled.

C **Correct.** A contract is a working agreement that is revisited when the client's priorities or circumstances change (Hepworth et al.). An eviction notice is an urgent basic-needs problem, and the client says it now fills their attention, so worker and client revise the target problems and time frame together. It beats A because holding to the old focus ignores a pressing problem and the client's own ranking, which weakens engagement.

Task-centered work expects target problems to be added or reordered when a new problem becomes the client's priority (Reid & Epstein, 1972).

Takeaway: Contracts are living agreements: when an urgent new problem becomes the client's priority, renegotiate goals and time frame together.

Sources: Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, Negotiating goals and formulating a contract; Reid, W. J., & Epstein, L. (1972). Task-Centered Casework. Columbia University Press, target problems, contract and task implementation

Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.15 Contracting and goal setting: partializing, measurable objectives · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 3 of 3

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70. INT-1659 (rev 2) · Best answer: A

A 24-year-old client in outpatient therapy finishes graduate school next month and will start a part-time job with no health benefits. The client has been covered under a parent's employer plan and plans to stop therapy at graduation because the insurance will 'run out.' What is the social worker's **BEST** response?

A. Explain that the coverage can continue until the client turns 26

B. Help the client apply for Medicaid before graduation

C. Help the client compare marketplace plans before graduation

Why each option is right or wrong

A **Correct.** The Affordable Care Act requires plans that offer dependent coverage to keep it available until a child turns 26, whether or not the child is a student, married, or financially dependent (42 U.S.C. 300gg-14(a); 45 CFR 147.120). The client is 24, so graduation does not end coverage, and treatment can continue without a gap. Knowing how policy shapes access lets the social worker correct a mistaken belief that would end effective care. It beats B because Medicaid may or may not fit the client's income and could mean changing providers, while the current coverage already continues.

B **Incorrect** (trap: Good but not best). Medicaid can be a lifeline for clients who lose coverage, and eligibility could be checked if needed. Here the client keeps the parent's plan until 26, so applying solves a problem that does not exist yet and may disrupt care.

C **Incorrect** (trap: Good but not best). Marketplace coverage is a real option for young adults leaving a parent's plan, so this is a reasonable thought. It is not needed yet: the client can stay on the parent's plan until 26, which keeps the current coverage and provider without a gap or a new premium.

Takeaway: Know how laws shape access: young adults can stay on a parent's health plan until 26, regardless of student status.

Sources: Patient Protection and Affordable Care Act, 42 U.S.C. 300gg-14(a); Affordable Care Act regulations, 45 CFR 147.120

Area III Intervention and Practice · **Sub-area** III.A Practice Concepts · **Topic** III.A.7 Effects of policy, regulation, legislation on practice and services · **Qualifier** BEST · **Cognitive** application · **Difficulty** 2 of 3

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71. ASM-1179 (rev 2) · Best answer: B

For 2 years, a veteran with chronic back pain has described a passive wish not to wake up at most sessions. Today the social worker asks the standard risk questions, and the client again reports no plan and no intent to act. Firearms remain stored with a sibling under an earlier agreement, and the client used the safety plan during a hard night last week. A new team member urges an emergency evaluation. What is the **BEST** response?

A. Arrange an emergency evaluation for the client's safety
B. Review the safety plan together and continue current care
C. Ask the client to sign a no-suicide agreement
D. Reduce sessions to monthly check-ins

Why each option is right or wrong

A **Incorrect** (trap: Hands off too soon). Emergency evaluation fits an acute change: new intent, a plan, new access to means or a recent attempt. None is present. Using it for a stable chronic pattern is more restrictive than needed and may teach the client that honest reports bring a hold.

B **Correct.** Risk is judged against the client's own baseline. Chronic passive ideation without change, with means still secured and a safety plan the client actually uses, calls for continued outpatient care and a reviewed safety plan, the least restrictive effective response (Stanley & Brown, 2012). It beats A because nothing acute has changed: no plan, no intent and no new access to means. Emergency evaluation would become right if any of those changed.

C **Incorrect** (trap: Misuses the technique). No-suicide contracts have no evidence that they prevent suicide and can discourage honest disclosure (Rudd et al., 2006). A collaboratively written safety plan, which this client already uses, is the accepted alternative.

D **Incorrect** (trap: Ignores risk). Stable chronic ideation still carries elevated long-term risk, especially with chronic pain. Reducing contact removes monitoring and support when nothing suggests the client needs less.

Takeaway: Judge suicide risk against the client's own baseline; unchanged chronic ideation with secured means calls for safety planning, not emergency evaluation.

Sources: Stanley, B., & Brown, G. K. (2012). Safety Planning Intervention: A brief intervention to mitigate suicide risk. Cognitive and Behavioral Practice, 19(2), 256-264, whole article; Rudd, M. D., Mandrusiak, M., & Joiner, T. E. (2006). The case against no-suicide contracts. Journal of Clinical Psychology, 62(2), 243-251, whole article
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.1 Assessing risk of harm to self or others · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 3 of 3

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72. ETH-1153 (rev 2) · Best answer: D

When writing progress notes for members of a therapy group, how should a social worker handle information about the other group members?

A. Refer to other members by their initials
B. Keep one shared note in a group file
C. List every attendee in each member's note
D. Leave other members' identities out of each note

Why each option is right or wrong

A **Incorrect** (trap: Misuses the technique). Initials feel like de-identification, but in a small group with a known schedule they can easily point to specific people. Anyone who later reads the record, including the member, could identify peers.

B **Incorrect** (trap: Misreads law or ethics). A single shared note mixes every member's information, so releasing one person's record would expose everyone else's. Each member needs an individual record that supports their own care and billing.

C **Incorrect** (trap: Misreads law or ethics). An attendance list in every chart reveals who else is in treatment. That is not relevant to the individual member's services, and the group's confidentiality agreement (1.07(f)) cannot protect names placed in another member's record.

D **Correct.** Each member's record should hold only information relevant to that member's services, and privacy should be protected as far as possible (NASW Code 3.04(c)). Other members' names, initials or disclosures do not belong in a member's chart, because that member, an insurer or a court may later see it. The usual approach is a separate note per member describing group themes and that member's participation. It beats A because initials in a small, known group can still identify people, so they are not real protection.

Takeaway: Each group member's record stands alone: document the member's own participation and leave other members' identities out.

Sources: NASW Code of Ethics (2021), 3.04(c); NASW Code of Ethics (2021), 1.07(f)
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.4 Legal and ethical record keeping and documentation · **Qualifier** none · **Cognitive** recall · **Difficulty** 2 of 3

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73. INT-1065 (rev 1) · Best answer: B

PRETEST — unscored on this form; do not count it

A social worker is screening applicants for a 12-week interpersonal process group at a community mental health center. One applicant with schizophrenia stopped taking medication a month ago, hears critical voices most of the day, and believes neighbors are watching through the walls. The applicant hopes the group will help them make friends. What is the **MOST** appropriate action?

A. Admit the applicant and pair them with a peer buddy
B. Defer group entry and link the applicant to psychiatric care
C. Explore what the applicant hopes to gain from the group

Why each option is right or wrong

A **Incorrect** (trap: Ignores risk). A peer buddy cannot make an intense process group workable for someone with active psychosis, and the group's feedback could worsen distress. A structured, supportive group could be considered later as symptoms settle.

B **Correct.** Screening matches members to a group they can use. Active psychosis is a standard reason to exclude someone from an interpersonal process group, because its intensity and feedback can overwhelm a person who cannot yet use them (Yalom & Leszcz, 2020). The stopped medication and constant voices also need prompt psychiatric attention, so the worker links the applicant to care and offers the group

later. It beats C because the applicant's hope for friendship matters, but it does not change the fit decision now; exploring it fits once symptoms are stable.

C **Incorrect** (trap: Good but not best). Exploring goals is part of every screening interview, but this applicant's current symptoms already decide the fit question. The wish to make friends is worth carrying into psychiatric care and later group planning.

Takeaway: Pre-group screening defers people with active psychosis or acute crisis from process groups and links them to appropriate care first.

Sources: Yalom, I. D., & Leszcz, M. (2020). The Theory and Practice of Group Psychotherapy (6th ed.). Basic Books, selection of clients: criteria for exclusion
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.23 Group work: managing process and building cohesion · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 1 of 3

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74. INT-1662 (rev 2) · Best answer: A

A social worker is treating a 15-year-old for anxiety about attending school and tracks the teen's daily anxiety ratings on a 0-10 scale. Over 3 weeks the ratings fall from 8 to 2. In this session, the teen mentions that a parent recently arranged online classes so the teen can study at home most days. What should the social worker do **NEXT**?

A. Examine whether staying home accounts for the lower ratings
B. Praise the progress and continue the current treatment plan
C. Add a second anxiety scale to confirm the lower ratings

Why each option is right or wrong

A **Correct.** Progress data mean little until the social worker checks what produced the change; an outside event during the intervention is a classic rival explanation in an AB comparison (Bloom, Fischer & Orme, 2009). Anxiety ratings are expected to fall when a feared situation is simply avoided, and avoidance keeps school-related anxiety going (Kearney & Albano, 2007). The online classes began as the ratings dropped, so the next step is to compare the ratings with actual school attendance and discuss the pattern with the teen and parent. It beats B because carrying on as if treatment produced the drop would quietly reward the avoidance.

B **Incorrect** (trap: Good but not best). Recognizing progress is good practice, and continuing a plan that seems to work is usually sensible. Here the drop coincides with staying home, so the ratings may show relief from avoidance rather than less anxiety about attending school.

C **Incorrect** (trap: Misuses the technique). A second anxiety scale would measure the same feeling under the same avoidant conditions and would likely agree. What is missing is a measure of the behavior that matters, school attendance, not another rating of anxiety.

Takeaway: Before concluding a client improved, check whether the measured change reflects progress on the goal or a side effect such as avoidance.

Sources: Bloom, M., Fischer, J., & Orme, J. G. (2009). Evaluating Practice: Guidelines for the Accountable Professional (6th ed.). Pearson, AB design; threats to internal validity; Kearney, C. A., & Albano, A. M. (2007). When Children Refuse School: A Cognitive-Behavioral Therapy Approach, Therapist Guide (2nd ed.). Oxford University Press, functions of school refusal behavior

Area III Intervention and Practice · **Sub-area** III.C Practice Evaluation and Research · **Topic** III.C.1 Evaluating progress and effectiveness of intervention and service plans · **Qualifier** NEXT · **Cognitive** reasoning · **Difficulty** 3 of 3

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75. INT-1109 (rev 1) · Best answer: B

In a jail reentry program, a client says, 'Things got messed up with my probation officer last week.' The social worker responds, 'When you say things got messed up, what happened?' Which listening skill is the social worker using?

A. Paraphrase

B. Clarification

C. Interpretation

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). A paraphrase restates the content of what the client said in fresh words. The worker here does not restate anything; the worker asks the client to explain a vague phrase, which is clarification.

B **Correct.** Clarification asks the client to make a vague statement specific so the social worker understands it accurately. 'Messed up' could mean a missed meeting, an argument or a violation, so the worker checks what the phrase means before responding (Hepworth et al.). It beats A because a paraphrase would restate the content, such as 'Something went wrong with your probation officer,' without asking for the missing detail.

C **Incorrect** (trap: Wrong model or concept). Interpretation offers a new meaning or pattern, such as linking the problem to the client's feelings about authority. The worker adds no new meaning; the response only seeks the client's own account of what happened.

Takeaway: Clarification checks the meaning of vague words; paraphrase restates content; interpretation adds a new meaning.

Sources: Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, Verbal following, exploring, and focusing skills: seeking concreteness

Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.3 Active listening and observation skills · **Qualifier** none · **Cognitive** recall · **Difficulty** 1 of 3

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76. ETH-1093 (rev 1) · Best answer: C

On an inpatient psychiatric unit, a voluntary patient with schizophrenia who has decision-making capacity refuses a new antipsychotic. The psychiatrist proposes mixing it into the patient's juice without telling the patient. In the team meeting, the social worker objects, but the psychiatrist and nurse manager plan to start tomorrow. What should the social worker do **NEXT**?

A. Have the psychiatrist walk through the plan once more
B. Record the objection and follow the team's plan
C. Take the concern to the hospital ethics committee today
D. Tell the patient about the plan before tomorrow

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). Asking for the rationale fits when a concern is first raised. The objection has already been heard and the plan is set, so repeating the conversation only delays action before tomorrow.

B **Incorrect** (trap: Misreads law or ethics). Documenting disagreement while going along with covert medication would make the social worker part of deceiving the patient. The Code calls for pursuing the concern, not only recording it.

C **Correct.** When a team decision raises ethical concerns, social workers first try to resolve them through appropriate channels and then pursue other avenues consistent with client well-being (NASW Code 2.03(b)). Giving medication covertly to a capable voluntary patient overrides informed consent (1.03(a)), and objecting within the team has failed with a start date set. An ethics consultation is the hospital's next channel. C beats D because the formal channel can stop the plan without bypassing the team; telling the patient becomes an option if that channel fails.

D **Incorrect** (trap: Right action, wrong time). The patient's right to know matters, but going around the team before using the formal channel can fracture care and the patient's trust in staff. It may be warranted if the ethics consultation fails.

Takeaway: When a team proceeds with an ethically troubling plan after objection, use the next formal channel, such as an ethics consultation.

Sources: NASW Code of Ethics (2021), 2.03(b); NASW Code of Ethics (2021), 1.03(a)
Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.2 Legal-ethical duties to clients, colleagues, profession, and society · **Qualifier** NEXT · **Cognitive** reasoning · **Difficulty** 3 of 3

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77. ETH-1152 (rev 2) · Best answer: A

A parent in a contested custody case tells the social worker that their 6-year-old came back from the other parent's home with a burn on one hand. The parent says the child described the other parent holding the hand on a hot stove, and shows a dated photo of the burn. The two parents have accused each other of lying in court filings. The parent asks for a letter for the custody hearing. What should the social worker do **FIRST**?

A. Notify protective services about the burn
B. Write the letter describing what the parent reported
C. Ask to meet with the child to hear the account directly
D. Coach the parent on filing the hotline call themselves

Why each option is right or wrong

A **Correct.** Federal law requires states to mandate reports of known or suspected child abuse (CAPTA, 42 U.S.C. 5106a(b)(2)(B)(i)); reporters need reasonable suspicion, not proof, and judging credibility is the job of child protective services. A specific injury, the child's own account of how it happened and a dated photo create reasonable suspicion, even amid custody conflict. Possible motives are details to pass on, not reasons to wait. It beats C because meeting the child to test the story makes the social worker an investigator, risks shaping the child's account and delays protection. When safe, tell the parent about the report (NASW Code 1.07(d)).

B **Incorrect** (trap: Boundary slip or collusion). A letter for the hearing serves the custody case, not the child's safety, and it does not meet the reporting duty. Writing it could also pull the social worker into an advocacy role in the dispute. The report comes first.

C **Incorrect** (trap: Skips assessment). Hearing from the child seems careful given the custody conflict, but the duty rests on reasonable suspicion, which the injury, the child's account and the photo already supply. Questioning the child is investigative work that belongs to protective services and can taint later interviews.

D **Incorrect** (trap: Misreads law or ethics). The parent may also call, and supporting that is fine. But the reporting duty belongs to the social worker personally, and relying on the parent leaves the child's protection to someone in the middle of a court fight.

Takeaway: A litigant's possible motive does not raise the bar for reporting; pass a child's specific account to protective services to weigh.

Sources: Child Abuse Prevention and Treatment Act, 42 U.S.C. 5106a(b)(2)(B)(i); NASW Code of Ethics (2021), 1.07(d)

Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.3 Mandated reporting: abuse, harm threats, impaired colleagues, duty to warn · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 2 of 3

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78. ASM-1616 (rev 1) · Best answer: A

At a pediatric clinic, a social worker meets the parent of a child who turned 3 last month. The child uses only a few words, and the pediatrician recommends a developmental evaluation and possible services. The family has never received any early childhood services. Which resource is the **BEST** fit for the parent to contact?

A. The local school district's special education office

B. The state's Part C early intervention program

C. A local Head Start program

Why each option is right or wrong

A **Correct.** Under the Individuals with Disabilities Education Act, Part C early intervention serves children under age 3, and public school systems must make special education available from age 3 and find and evaluate children who may need it (34 CFR 303.21; 34 CFR 300.101; 34 CFR 300.111). This child just turned 3, so the school district is responsible for the evaluation and any preschool services, at no cost to the family. It beats B because Part C eligibility ends at the third birthday; that program would have been the right contact a few months earlier.

B **Incorrect** (trap: Wrong model or concept). Part C early intervention is for infants and toddlers under age 3. It was the right resource before the child's birthday, but a child who has turned 3 goes to the school district for evaluation.

C **Incorrect** (trap: Good but not best). Head Start serves many 3- to 5-year-olds and can enroll children with disabilities, but it does not replace the school district's duty to evaluate the child and provide special education. It may be a useful setting once eligibility is known.

Takeaway: Match resources to eligibility: IDEA Part C serves children under 3; from age 3, the public school district evaluates and serves.

Sources: Individuals with Disabilities Education Act regulations, 34 CFR 303.21; 34 CFR 300.101(a); 34 CFR 300.111

Area II Assessment and Planning · **Sub-area** II.C Assessment Practices · **Topic** II.C.1 Identifying available, appropriate community resources · **Qualifier** BEST · **Cognitive** recall · **Difficulty** 3 of 3

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79. INT-1591 (rev 2) · Best answer: C

A year-end evaluation of a home-based depression program shows strong overall results, and the board will vote next week on expanding it unchanged to three counties. When the social worker breaks the data down by age, clients 65 and older showed no improvement and dropped out at twice the rate of younger clients. What is the **MOST** appropriate action?

A. Support the board's plan, since overall results are strong

B. Hold the age results until a follow-up study confirms them

C. Present the age findings to the board before the vote

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Strong overall averages can hide groups a program does not help. Expanding unchanged would carry the same gap for older clients into three more counties.

B **Incorrect** (trap: Misreads law or ethics). Confirming a surprising subgroup result is wise, but the board votes next week. Holding the data back lets the expansion go forward on incomplete information, when the findings could be shared as preliminary with a plan to look into them.

C **Correct.** Evaluation findings should be reported accurately, and leaving out a known subgroup failure would misrepresent them (NASW Code 5.02(o)); and an evaluation earns its value when its results shape program decisions (Patton, 2008). An overall average here hides a group the program is failing. Sharing the age findings before the vote lets the board consider adapting the model for older clients. This beats B because waiting for a follow-up study lets a major decision rest on information the social worker already knows is incomplete.

Takeaway: Report subgroup findings that change the picture to decision makers before they act, so evaluation actually informs services.

Sources: NASW Code of Ethics (2021), 5.02(o); Patton, M. Q. (2008). Utilization-Focused Evaluation (4th ed.). SAGE, intended use by intended users

Area III Intervention and Practice · **Sub-area** III.C Practice Evaluation and Research · **Topic** III.C.3 Program evaluation and its effect on services · **Qualifier** MOST APPROPRIATE · **Cognitive** reasoning · **Difficulty** 2 of 3

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80. ASM-1552 (rev 1) · Best answer: B

A school social worker meets with a parent whose 10-year-old has missed 6 days this month. The family lost their apartment and is staying with relatives across town, about an hour away by bus. The parent wants the child to stay at the current school but assumes the move requires a transfer. The social worker has confirmed the family has no other housing. What should the social worker do **NEXT**?

A. Help the parent enroll the child at the school near the relatives
B. Connect the family with the district's homeless education liaison
C. Refer the family to a housing agency before addressing school
D. Start a truancy referral as the attendance rules require

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Enrolling near the relatives is a lawful option, but the parent prefers the current school and the law presumes that staying is in the child's best interest. A transfer fits if the family chooses it after learning the options.

B **Correct.** Under the McKinney-Vento Act, children who share others' housing because they lost their own are homeless (42 U.S.C. 11434a(2)(B)(i)). They may stay in their school of origin when it serves their best interest, with transportation provided, and each district has a liaison who arranges this (42 U.S.C. 11432(g)(1)(J), (g)(3)(A)). Connecting the family with the liaison addresses both the absences and the parent's wish. It beats A because no transfer is required, and changing schools would add another loss for a displaced child.

C **Incorrect** (trap: Right action, wrong time). Housing help matters and should follow, but it does not solve the immediate barrier: the child is missing school now because of the distance. School-stability rights apply right away, without waiting for new housing.

D **Incorrect** (trap: Misreads law or ethics). A truancy referral treats absences caused by homelessness as willful. The McKinney-Vento Act directs schools to remove barriers to attendance for homeless students; truancy steps fit only when absences have no such cause.

Takeaway: Children doubled up with others after losing housing count as homeless and may stay in their school of origin with transportation arranged.

Sources: McKinney-Vento Homeless Assistance Act, 42 U.S.C. 11434a(2)(B)(i); 42 U.S.C. 11432(g)(1)(J), (g)(3)(A)
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.20 Displacement effects: homelessness, immigration, refugee status · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 2 of 3

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81. ETH-1073 (rev 2) · Best answer: C

A group practice bills a client's health plan for weekly sessions. The client begins addressing gambling losses and asks that those sessions be kept from the plan, paying the full fee for each one at the visit. The social worker has confirmed that the client understands the cost. How should the social worker handle billing for those sessions?

A. Bill the plan but leave gambling off the claim
B. Explain that practice policy bills every session to the plan
C. Mark those sessions so nothing about them goes to the plan

Why each option is right or wrong

A **Incorrect** (trap: Misuses the technique). Billing the plan for sessions the client already paid for would collect twice and still disclose the services. Leaving gambling off the claim does not honor the restriction and makes the claim inaccurate (3.05).

B **Incorrect** (trap: Misreads law or ethics). Practice policy cannot override the client's federal right to restrict health plan disclosures for services paid in full. Telling the client otherwise misstates the client's rights.

C **Correct.** Under HIPAA, a provider must agree to a client's request not to disclose information to a health plan for payment or operations when it concerns services the client has paid for in full (45 CFR 164.522(a)(1)(vi)). This client pays in full for these sessions, so the practice must keep them off claims. Disclosure to payers also requires the client's authorization (NASW Code 1.07(h)). C beats B because an internal billing policy cannot override a restriction that federal law requires the practice to honor.

Takeaway: A client who pays in full out of pocket may keep those services from the health plan; providers must honor that restriction.

Sources: HIPAA Privacy Rule, 45 CFR 164.522(a)(1)(vi); NASW Code of Ethics (2021), 1.07(h); NASW Code of Ethics (2021), 3.05
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.5 Ethical billing: managed care, insurance, incident-to billing · **Qualifier** none · **Cognitive** application · **Difficulty** 2 of 3

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82. ETH-1081 (rev 1) · Best answer: D

A resident of a memory care unit who has major neurocognitive disorder asks staff every afternoon where their spouse is. The spouse died 2 years ago. Each time staff explain the death, the resident grieves as if hearing it for the first time. Staff ask the social worker how to answer. What is the **BEST** guidance?

A. Gently restate the fact of the death each time
B. Say the spouse is at work and will visit later
C. Ask the family to decide what staff should say
D. Acknowledge the longing and invite shared memories

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Truthfulness matters, but each disclosure here produces new grief the resident cannot retain. Repeating it ranks accuracy above harm, the reverse of the screen's order, without helping the resident.

B **Incorrect** (trap: Misreads law or ethics). A made-up story avoids distress but is deception, which 4.04 prohibits. It can also backfire when the promised visit never happens, adding to agitation.

C **Incorrect** (trap: Hands off too soon). Family knowledge of the resident's history is valuable, but the question is how to resolve a professional ethical conflict. Handing that decision to relatives sidesteps the social worker's own responsibility.

D **Correct.** This is a conflict between truthfulness and least harm. The Ethical Principles Screen ranks least harm above truthfulness (Dolgoff et al., 2012), and the stem shows that repeating the death causes fresh grief each time. Yet the Code bars taking part in deception (NASW Code 4.04). Responding to the feeling behind the question and inviting memories honors both: no false statement and no repeated injury. D beats A

because restating the death serves accuracy while ignoring documented harm to a resident who cannot retain the information.

Takeaway: When telling the truth causes repeated harm a person cannot process, respond to the underlying feeling without deceiving.

Sources: Dolgoff, R., Harrington, D., & Loewenberg, F. M. (2012). Ethical Decisions for Social Work Practice (9th ed.). Brooks/Cole Cengage, Ethical Principles Screen: least harm; truthfulness; NASW Code of Ethics (2021), 4.04
Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.3 Recognizing and working through ethical dilemmas · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

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83. ASM-1025 (rev 1) · Best answer: B

A 34-year-old client spends about 3 hours a day washing after touching doorknobs or mail. The client is completely sure that skipping the routine will give their children a fatal illness and cannot be talked out of it. Work and family life have narrowed around the washing. Outside of this topic, the client's conversation is organized and goal-directed. Which diagnosis is **MOST** likely?

A. Delusional disorder
B. Obsessive-compulsive disorder
C. Schizophrenia
D. Illness anxiety disorder

Why each option is right or wrong

A **Incorrect** (trap: Near-miss diagnosis). Delusional disorder involves a fixed false belief lasting at least 1 month without the full picture of schizophrenia. When the belief is an obsessional fear that drives compulsions, DSM-5-TR classifies it as OCD with absent insight instead.

B **Correct.** OCD is diagnosed even when the client fully believes the obsessional fear; DSM-5-TR adds the specifier 'with absent insight/delusional beliefs' instead of changing the diagnosis. Contamination fears that drive hours of washing to prevent a dreaded event are the core obsession and compulsion pattern (DSM-5-TR). It beats A because delusional disorder is not diagnosed when the false belief is better explained by OCD: this conviction drives hours of compulsive washing rather than standing alone as a fixed belief.

C **Incorrect** (trap: Near-miss diagnosis). Schizophrenia requires at least two core psychotic symptoms, such as hallucinations or disorganized speech, with signs lasting 6 months. One fixed belief tied to rituals, with organized conversation on other topics, does not fit that picture.

D **Incorrect** (trap: Near-miss diagnosis). Illness anxiety disorder is preoccupation with having or getting a serious illness oneself, usually without hours of rituals. This client fears harming the children through contamination, and the washing is compulsive.

Takeaway: Full conviction about an obsessional fear does not make it psychosis; OCD is diagnosed with the absent-insight specifier.

Sources: DSM-5-TR (APA, 2022), Obsessive-Compulsive and Related Disorders: Obsessive-Compulsive Disorder
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.9 DSM use in assessment; common mental and brain-related condition indicators · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 3 of 3

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84. INT-1155 (rev 1) · Best answer: D

In a family session, one parent makes every decision. This parent talks over the other parent, who rarely speaks. For a few minutes, the social worker takes the quiet parent's side in a fight about bedtime. The goal is to shift who holds power in the family. Which structural technique is the social worker using?

A. Joining
B. Enactment
C. Boundary making
D. Unbalancing

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Joining is how the social worker builds rapport and accommodates to the family's style, usually early, without challenging the structure. Taking one parent's side against the other is a deliberate challenge to the structure.

B **Incorrect** (trap: Wrong model or concept). In an enactment, the social worker has family members carry out their problem interaction with each other in the session, then observes and redirects. Here the social worker, not the family, is the one acting.

C **Incorrect** (trap: Wrong model or concept). Boundary making changes closeness and access between members or subsystems, for example through seating changes or blocking interruptions. The move described is a temporary alliance with one member, not a boundary.

D **Correct.** In unbalancing, the structural therapist temporarily allies with one family member to change the hierarchy or distribution of power within a subsystem (Minuchin & Fishman, 1981). The social worker here sides with the quieter parent on purpose and for a limited time to disrupt the other parent's dominance. It beats C, boundary making, because boundary making regulates closeness and distance between members or subsystems, such as blocking interruptions, rather than siding with one member to shift power.

Takeaway: Unbalancing means temporarily siding with one family member to shift power; boundary making regulates closeness between members and subsystems.

Sources: Minuchin, S., & Fishman, H. C. (1981). Family Therapy Techniques. Harvard University Press, Unbalancing
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.21 Family therapy models and interventions · **Qualifier** none · **Cognitive** recall · **Difficulty** 2 of 3

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85. INT-1045 (rev 2) · Best answer: C

On an inpatient unit, a 24-year-old patient with schizophrenia is in their second admission this year and will return home to live with their parents. The parents describe frequent criticism and say, 'We watch everything they do.' The patient agrees to family involvement. Which family intervention is **BEST** for reducing the risk of relapse?

A. Give the parents one education session before discharge
B. Refer the patient's parents to a family peer support group
C. Start months-long psychoeducation with the parents and patient
D. Coach the parents to examine patterns in their own families

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Brief education is better than none and often happens before discharge. But relapse reduction comes from multi-session programs that build skills over months, not from one meeting.

B **Incorrect** (trap: Good but not best). Family peer support groups ease caregiver burden and isolation and can complement treatment. They are not built to change the criticism and close monitoring that drive relapse, and they leave the patient out; structured psychoeducation with the patient included targets those patterns directly.

C **Correct.** Family psychoeducation for schizophrenia combines illness education, communication and problem-solving training, crisis planning, and support, usually over 9 months or more (Dixon, Adams & Lucksted, 2000). Meta-analyses show it cuts relapse by about half compared with usual care (McFarlane, 2016). Frequent criticism and intrusive monitoring are signs of high expressed emotion, which predicts relapse, and this patient has already been readmitted. It beats A because a single session can inform the parents but cannot build the skills that lower relapse over time.

D **Incorrect** (trap: Wrong model or concept). Examining multigenerational patterns is a Bowen approach. It does not target the current criticism and intrusiveness, or teach the skills that family psychoeducation uses to reduce relapse.

Takeaway: For schizophrenia with high expressed emotion at home, multi-month family psychoeducation reduces relapse more than brief education does.

Sources: McFarlane, W. R. (2016). Family interventions for schizophrenia and the psychoses: A review. Family Process, 55(3), 460-482, whole article; Dixon, L., Adams, C., & Lucksted, A. (2000). Update on family psychoeducation for schizophrenia. Schizophrenia Bulletin, 26(1), 5-20, whole article
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.21 Family therapy models and interventions · **Qualifier** BEST · **Cognitive** application · **Difficulty** 3 of 3

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86. ETH-1106 (rev 2) · Best answer: D

A client with panic disorder asks a social worker to use a 'one-session trauma release' technique that online videos say ends panic in a single visit. The social worker, who is trained in cognitive-behavioral therapy for panic, finds no published research supporting the technique. The client has not had any prior therapy for panic and says quick relief is the goal. What is the **BEST** response?

A. Ask what draws the client to this technique
B. Try the technique once, as the client prefers
C. Refer the client to a practitioner who offers the technique
D. Explain the evidence and offer an established treatment

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Understanding a request helps when its reasons are unclear. This client has explained the appeal (the videos' promise of one-visit relief) and named quick relief as the goal, so the useful next move is to share what the evidence shows and offer effective options.

B **Incorrect** (trap: Outside the social work role). Self-determination covers choices among legitimate options, but it does not oblige a social worker to deliver an

unsupported technique without training. Competent practice rests on recognized knowledge and training (4.01(c), 1.04(b)).

C **Incorrect** (trap: Hands off too soon). Referral fits when another professional's expertise is needed (1.16(a)). Here it would steer the client toward an unsupported approach before offering the effective treatment the worker can provide.

D **Correct.** Social workers base practice on recognized knowledge, including empirically based knowledge (NASW Code 4.01(c)), and informed consent includes discussing alternatives (1.03(a)). Explaining that the technique lacks research support while offering an established panic treatment, which the worker is trained to provide, respects the client's goal of quick relief and keeps the choice informed. Self-determination is honored by informing the client, not by delivering an unsupported technique (1.02). D beats A because the client has already named the reason for the request; more exploration only delays the information needed to choose.

Takeaway: Clients choose among informed options; explain the evidence and offer effective treatment rather than deliver an unsupported technique on request.

Sources: NASW Code of Ethics (2021), 4.01(c); NASW Code of Ethics (2021), 1.03(a); NASW Code of Ethics (2021), 1.02; NASW Code of Ethics (2021), 1.04(b); NASW Code of Ethics (2021), 1.16(a)
Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.5 Core values: competence, social justice, integrity, dignity and worth · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 3 of 3

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87. ETH-1708 (rev 2) · Best answer: D

A 68-year-old refugee from a rural farming region has lived in the United States for 3 years. The client works part-time and enjoys time with grandchildren. The client mourns that the family can no longer hold seasonal ceremonies for ancestors at the village shrine and dreams that the ancestors are hungry and calling. The client says, 'I left them with no one to care for them.' Which concept **MOST** likely describes this distress?

A. Prolonged grief disorder
B. Posttraumatic stress disorder
C. Acculturative stress
D. Cultural bereavement

Why each option is right or wrong

A **Incorrect** (trap: Near-miss diagnosis). Prolonged grief disorder follows the death of someone close at least 12 months earlier, with near-daily yearning or preoccupation and significant distress or impairment (DSM-5-TR). This client's loss is of ceremonies and ancestral duties, and work and family life continue.

B **Incorrect** (trap: Near-miss diagnosis). PTSD requires exposure to a traumatic event plus intrusion, avoidance, negative changes in mood and thinking, and arousal symptoms. These dreams express guilt about unmet duties to ancestors rather than reliving a traumatic event, and daily functioning is intact.

C **Incorrect** (trap: Wrong model or concept). Acculturative stress comes from the strain of adapting to a new culture, such as language barriers or discrimination. This client's pain points backward, toward lost practices and obligations, rather than toward difficulty adjusting to life here.

D **Correct.** Cultural bereavement describes a refugee's grief over the loss of homeland, community and cultural practices, including unmet obligations to ancestors, and it can be a meaningful, even constructive, response rather than a disorder (Eisenbruch, 1991). The distress centers on lost ceremonies and ancestors left without care, while work and family life continue. It beats A because prolonged grief disorder follows the death of a close person, with intense yearning and impairment; this grief is for a lost way of life and its duties.

Takeaway: Refugees' grief for lost homeland, rituals and ancestral duties may be cultural bereavement, a meaningful response, not automatically a psychiatric disorder.

Sources: Eisenbruch, M. (1991). From post-traumatic stress disorder to cultural bereavement: Diagnosis of Southeast Asian refugees. Social Science & Medicine, 33(6), 673-680, whole article; DSM-5-TR (APA, 2022), Trauma- and Stressor-Related Disorders: Prolonged Grief Disorder

Area I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.9 Globalization, immigration, refugee status effects on clients and services · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 3 of 3

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88. ETH-1041 (rev 1) · Best answer: C

At an opioid treatment program, a client says their partner burned their 6-year-old with a cigarette this morning, and the client saw the mark. The social worker knows federal rules strictly protect substance use treatment records and is unsure whether a report is allowed. What is the **BEST** action?

A. Ask the client to sign a release before reporting
B. Get guidance from the program's attorney before acting
C. Make the abuse report with just the facts it requires
D. Make the report and send the client's treatment file

Why each option is right or wrong

A **Incorrect** (trap: Misreads law or ethics). A release is the usual route for disclosing substance use treatment information, which makes this tempting. Abuse reports under state law are an express exception, and a mandated report cannot depend on the client's agreement.

B **Incorrect** (trap: Ignores risk). Legal advice helps with unclear questions, and the worker can consult later about any records request. The child abuse exception is clear, and waiting on an attorney delays a report that protects a child burned this morning.

C **Correct.** The federal confidentiality rules for substance use programs do not apply to reports of suspected child abuse made under state law (42 CFR 2.12(c)(6)). A burn the client witnessed gives reasonable suspicion, so the worker reports. The same rule keeps the original treatment records protected, so the report includes only what the law requires. C beats A because requiring a release would let a refusal block a mandated report, and asking for one delays protection.

D **Incorrect** (trap: Misreads law or ethics). The exception covers the report, not the treatment record. Sending the file discloses protected information beyond what reporting needs; disclosure should be limited to the least information required (NASW Code 1.07(c)).

Takeaway: Part 2 permits the initial child abuse report under state law, but treatment records stay protected; report only what is required.

Sources: 42 CFR Part 2, 42 CFR 2.12(c)(6); NASW Code of Ethics (2021), 1.07(c)
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.3 Mandated reporting: abuse, harm threats, impaired colleagues, duty to warn · **Qualifier** BEST · **Cognitive** application · **Difficulty** 3 of 3

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89. INT-1584 (rev 1) · Best answer: A

A board member of a rural senior center returns from a conference excited about a meal-delivery program that succeeded in a large city. The board asks the center's social worker to begin planning to launch the same program next year. What should the social worker do **FIRST**?

A. Gather data on unmet needs among local older adults
B. Write a grant proposal to fund a local version
C. Pilot the program in one town and track its use

Why each option is right or wrong

A **Correct.** A needs assessment comes before program design: it shows whether a problem exists locally, how large it is and what is already available (Kettner, Moroney & Martin, 2022). Success in a large city does not show that rural older adults share the same gaps, which might involve transportation, isolation or existing meal services. This beats C because a pilot tests the delivery of a program already chosen, and running one before knowing local needs risks building the wrong service.

B **Incorrect** (trap: Skips assessment). Funders expect proposals to document local need. Writing one before gathering that information skips the step that justifies the program and shapes its design.

C **Incorrect** (trap: Right action, wrong time). Piloting is a sound way to test delivery once a program is chosen. Doing it before assessing local needs puts resources into a model that may not fit the county.

Takeaway: Assess the local population's needs before designing or piloting a program, even one that has succeeded elsewhere.

Sources: Kettner, P. M., Moroney, R. M., & Martin, L. L. (2022). Designing and Managing Programs: An Effectiveness-Based Approach (6th ed.). SAGE, Ch. 4-5: Needs Assessment
Area III Intervention and Practice · **Sub-area** III.C Practice Evaluation and Research · **Topic** III.C.4 Agency program evaluation: needs assessment, formative/summative, cost analyses · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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90. ASM-1566 (rev 1) · Best answer: C

A social worker in private practice has seen a client for 8 sessions for work stress, based on a biopsychosocial assessment completed at intake. Today the client mentions, for the first time, an eating disorder in their teens and says they have started skipping meals again during a busy project. What should the social worker do **FIRST**?

A. Refer the client to an eating disorder treatment program
B. Continue the stress plan and monitor meals in later sessions
C. Ask about current eating patterns and physical health

Why each option is right or wrong

A **Incorrect** (trap: Hands off too soon). Specialized treatment may be needed, and referral becomes appropriate if assessment shows significant restriction or medical risk. Referring before learning the current pattern hands the client off without knowing what level of care fits.

B **Incorrect** (trap: Ignores risk). Continuing unchanged treats a possible relapse as background detail. A past eating disorder plus renewed restriction calls for prompt assessment, not watchful waiting.

C **Correct.** Biopsychosocial assessment is ongoing rather than a one-time intake task, and new information calls for updating it (Hepworth et al.). A past eating disorder with renewed meal skipping changes both the biological and psychological picture. Asking now about frequency, weight change, purging, physical symptoms and medical care updates that picture before the plan is revised. It beats A because the right level of care cannot be chosen until current severity is known; a specialized referral may well follow.

Takeaway: Biopsychosocial assessment is ongoing; when new history or symptoms emerge, update the assessment before revising the plan or referring.

Sources: Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, Assessment as an ongoing process
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.7 Biopsychosocial assessment framework and its components · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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91. INT-1062 (rev 3) · Best answer: A

In a residential treatment group for adolescents, the social worker announces that a planned outing is canceled because of staffing. Members begin blaming one quiet member, saying the outing was canceled because of that member's 'drama,' and the comments grow harsher. Which response is **BEST**?

A. Stop the blaming and name the group's shared frustration
B. Explain to the group the staffing reasons for canceling
C. Coach the blamed member privately on coping with peers

Why each option is right or wrong

A **Correct.** Scapegoating is a group process: members displace shared anger onto one person, often anger meant for the leader. The leader protects the targeted member and makes the hidden feeling speakable, here frustration about a cancellation the worker announced (SAMHSA TIP 41). Doing both in the group stops the harm and gives members a safer outlet. It beats C because private coaching may help the member later, but it leaves the group free to keep blaming and treats a group dynamic as one teen's problem.

B **Incorrect** (trap: Good but not best). Explaining the staffing problem corrects the false cause and may help once things settle. By itself it leaves the harsh comments unchallenged and the shared frustration unspoken, so the group can keep displacing it onto the quiet member.

C **Incorrect** (trap: Good but not best). Supporting the targeted member one-on-one can be a helpful follow-up, but it leaves the harsh comments unchallenged in the room. The pattern belongs to the group and needs to be worked on there.

Takeaway: When a group scapegoats one member, protect that member and redirect the displaced feeling to its real source within the group.

Sources: SAMHSA TIP 41 (2005), Ch. 6: Group Leadership, Concepts, and Techniques (scapegoating)
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.23 Group work: managing process and building cohesion · **Qualifier** BEST · **Cognitive** application · **Difficulty** 2 of 3

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92. ETH-1075 (rev 2) · Best answer: A

An outpatient clinic ran a quality-improvement project tracking depression scores for clients in a new group program. The results look strong, and the program director asks the social worker to prepare the client data for a journal article. The clinic has a research partnership with a nearby university. What should the social worker do **FIRST**?

A. Submit the project for IRB review before using the data
B. Remove client identifiers from the data set
C. Ask former group members for permission to publish
D. Draft the article, since the project used routine care data

Why each option is right or wrong

A **Correct.** Planning to publish does not by itself turn quality improvement into research, but analyzing client data for a journal article may be a systematic investigation designed to contribute to generalizable knowledge (45 CFR 46.102(l)). Social workers engaged in research follow participant-protection guidelines and consult institutional review boards (NASW Code 5.02(d)). The IRB, not the social worker, decides whether the work is research, whether it is exempt, what consent is needed and how data must be handled. A beats B because removing identifiers may well be part of the approved plan, but the social worker does not decide alone that the data need no review.

B **Incorrect** (trap: Good but not best). De-identifying data is often required and may support an exemption. Doing it first and treating the review question as settled skips the step that decides how the data may be used; it belongs within IRB review.

C **Incorrect** (trap: Right action, wrong time). Seeking permission sounds respectful, but the IRB determines whether consent is needed and what it must say. Contacting former clients without an approved procedure can itself raise privacy concerns.

D **Incorrect** (trap: Misreads law or ethics). Data gathered in routine care can still become research when they are used to produce generalizable knowledge. Publishing without review skips the protections that 5.02(d) expects.

Takeaway: Before using practice data for publication, let the IRB decide whether it is research and what review applies; don't decide alone.

Sources: NASW Code of Ethics (2021), 5.02(d); 45 CFR Part 46 (Common Rule), 45 CFR 46.102(l)
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.11 Research ethics: IRBs, human subjects protections, consent · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 3 of 3

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93. INT-1560 (rev 1) · Best answer: A

A parent in a home-visiting program made early progress with a 2-year-old's bedtime routine. For the past 6 weeks, the parent has skipped practice, sleeps much of the day and says nothing feels worth doing. A depression screen today falls in the moderately severe range, and the parent denies thoughts of self-harm when asked directly. Which action is **MOST** appropriate?

A. Refer the parent for depression treatment and keep visiting
B. Pause home visits until the parent's depression improves
C. Increase practice assignments to rebuild the bedtime routine now
D. Explore with the parent what has lowered their motivation

Why each option is right or wrong

A **Correct.** Parental depression is a major barrier to building parenting skills, and treating it helps children too: children's symptoms fell when their parents' depression remitted (Weissman et al., 2006). The symptoms and screen show a need beyond the home visitor's role, so a referral is warranted, and continuing visits keeps the child's support in place (NASW Code 1.16(a)). It beats B because pausing removes support from the parent and child at a hard time instead of adding treatment.

B **Incorrect** (trap: Hands off too soon). Pausing seems kind, but it withdraws support from a struggling parent and a young child. Visits can continue, perhaps in a lighter form, while the parent starts treatment.

C **Incorrect** (trap: Good but not best). More assignments add pressure on a parent whose energy and interest are already low. Skill practice is likely to work better once the depression is being treated.

D **Incorrect** (trap: Right action, wrong time). Exploring motivation is a sound step when the cause of a stall is unclear. Here the symptoms and the screen already point to depression, so further exploration delays the help the parent needs.

Takeaway: When parental depression stalls parenting work, refer the parent for treatment while continuing parenting support rather than pausing or pushing harder.

Sources: Weissman, M. M., Pilowsky, D. J., Wickramaratne, P. J., Talati, A., Wisniewski, S. R., Fava, M., et al. (2006). Remissions in maternal depression and child psychopathology: A STAR*D-child report. JAMA, 295, 1389-1398, whole article; NASW Code of Ethics (2021), 1.16(a)
Area III Intervention and Practice · **Sub-area** III.A Practice Concepts · **Topic** III.A.6 Building parenting capacity and skills · **Qualifier** MOST APPROPRIATE · **Cognitive** reasoning · **Difficulty** 2 of 3

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94. INT-1009 (rev 2) · Best answer: B

PRETEST — unscored on this form; do not count it

A hospital social worker meets with a patient admitted for heart failure who keeps adding salt to meals brought from home. The patient asks, 'Why does salt even matter for my heart?' Which response is **MOST** consistent with motivational interviewing?

A. Explain how salt causes fluid buildup and strains the heart
B. Ask what the patient already knows about salt and the heart
C. Reflect that the diet limits seem frustrating to the patient

Why each option is right or wrong

A **Incorrect** (trap: Misuses the technique). Accurate information can help, and MI allows giving it. Leading with a full explanation, though, skips the first step of asking what the patient knows, which makes the information feel imposed and invites pushback.

B **Correct.** When a client asks for information, motivational interviewing uses elicit-provide-elicit: first ask what the person already knows or wants to know, then offer information, then ask what they make of it (SAMHSA TIP 35, 2019). Starting with a question respects the patient's expertise and lets the worker fill real gaps. It beats A because delivering the explanation first, however accurate, casts the worker as the expert and often prompts the patient to defend current habits.

C **Incorrect** (trap: Good but not best). Reflections are central to MI, but the patient asked a direct question. Reflecting a feeling the patient has not expressed sidesteps the request; elicit-provide-elicit answers it while keeping the patient engaged.

Takeaway: When a client asks for information, MI uses elicit-provide-elicit: ask what they know, offer information, then ask their reaction.

Sources: SAMHSA TIP 35 (2019), Ch. 3: Motivational Interviewing as a Counseling Style
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.7 Evidence-based treatments: CBT, DBT, MBSR, MI, EMDR · **Qualifier** MOST · **Cognitive** application · **Difficulty** 3 of 3

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95. ASM-1545 (rev 2) · Best answer: A

A school social worker learns that a 7-year-old often arrives hungry and wears the same clothes several days in a row. The parent, who works two part-time jobs, explains that the family's food benefits stopped 2 months ago after a missed renewal appointment. The child is clean and up to date on checkups, and the parent attends every school conference. What should the social worker do **FIRST**?

A. Assist with restoring the lapsed aid and arranging free meals
B. Notify child protective services of possible neglect
C. Interview the child privately about conditions at home

Why each option is right or wrong

A **Correct.** Poverty is not neglect. Child welfare guidance stresses separating the two and meeting families' concrete needs, because neglect involves a caregiver's failure to provide care, not a temporary lack of resources (Child Welfare Information Gateway, 2023). This parent is working, engaged and seeking help, and the hunger traces to a lapsed benefit. Restoring the lapsed food benefits and arranging free school meals addresses the cause directly. It beats B because a report would bring an investigation to a family whose problem is a benefits gap; a report becomes appropriate if needs go unmet after help is offered or other signs of harm appear.

B **Incorrect** (trap: Misreads law or ethics). Mandated reporters report reasonable suspicion of maltreatment, but these facts point to a temporary loss of income support, not a caregiver's failure to provide. Reporting first treats poverty as neglect and can harm a family that is already seeking help.

C **Incorrect** (trap: Good but not best). Hearing from the child matters when the cause of hunger is unclear. Here the parent has explained the lapse in benefits and other care is consistent, so a private interview delays the practical help the child needs now.

Takeaway: Distinguish poverty from neglect: when a family's unmet needs stem from lack of resources, connect them to concrete supports first.

Sources: Child Welfare Information Gateway (2023). Separating Poverty From Neglect in Child Welfare, bulletin for professionals

Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.9 Poverty's impact; social determinants of health · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 3 of 3

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96. ASM-1091 (rev 1) · Best answer: B

An adult child calls a geriatric care program about a parent with moderate Alzheimer's disease who keeps a loaded handgun by the bed. Last week the parent woke at night, did not recognize the adult child, and pointed the gun at them. The parent insists the gun is needed for protection. What is the social worker's **BEST** response?

A. Suggest unloading the gun and hiding the ammunition

B. Help the family remove the gun from the home now

C. Wait for a capacity evaluation before changing anything

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Unloading and separate storage can reduce risk in milder impairment with supervision. With moderate dementia and an episode of aiming the gun at a family member, the parent may find or reload it, so partial steps fall short.

B **Correct.** Dementia impairs memory, judgment and recognition, and guidance on firearms holds that people with moderate to severe dementia should have no access; families should remove firearms (Betz et al., 2018). An episode of pointing the gun at an unrecognized family member shows immediate danger to others, which outweighs the parent's wish to keep it. It beats A because unloading and hiding ammunition can suit early stages, but a confused person who has already aimed the gun may find or reload it.

C **Incorrect** (trap: Ignores risk). A capacity evaluation can help with longer-term decisions. Waiting for one leaves a loaded gun with a person who has already pointed it at a family member during nighttime confusion.

Takeaway: In moderate dementia, firearm access endangers others and the person; families should remove firearms rather than rely on partial safeguards.

Sources: Betz, M. E., McCourt, A. D., Vernick, J. S., Ranney, M. L., Maust, D. T., & Wintemute, G. J. (2018). Firearms and dementia: Clinical considerations. Annals of Internal Medicine, 169(1), 47-49, whole article

Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.1 Assessing risk of harm to self or others · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 3 of 3

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97. ETH-0115 (rev 1) · Best answer: C

An outpatient clinic closes the case of any client who misses 2 appointments in a row. A social worker reviews closure data and finds that most closed cases involve Black clients from a neighborhood whose bus route was recently cut. What is the **BEST** way for the social worker to address this pattern?

A. Help each affected client appeal the closure of their case
B. Keep affected clients' cases open without telling managers
C. Present the data to leaders and propose changing the rule
D. Complete training on implicit bias in clinical decisions

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Helping clients appeal is useful advocacy for those already harmed, and it can accompany policy work. On its own, it leaves the rule in place, so new closures keep happening along the same racial lines.

B **Incorrect** (trap: Misreads law or ethics). Quietly ignoring the rule protects a few clients but bypasses the agency and cannot last. It leaves the unfair policy unchanged for other workers' clients and puts the social worker in an ethics conflict with the employer.

C **Correct.** Anti-racist practice looks beyond individual cases to policies that produce unequal outcomes, and the Code calls on social workers to hold institutions accountable and act against discrimination (NASW Code 1.05(c), 6.04(d)). A neutral-looking rule is falling hardest on Black clients who lost transit access. Bringing the data to leaders with a proposed change addresses the cause through proper channels. It beats A because appeals help one client at a time, while the rule keeps closing new cases; the best response targets the policy itself.

D **Incorrect** (trap: Good but not best). Examining one's own bias is part of anti-racist practice, but the disparity here comes from a closure rule and a transit cut, not from one worker's decisions. Training alone leaves the policy unchanged.

Takeaway: When a neutral rule produces racially unequal outcomes, bring the evidence to agency leaders and advocate changing the rule.

Sources: NASW Code of Ethics (2021), 1.05(c); NASW Code of Ethics (2021), 6.04(d)
Area I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.1 Practicing from anti-oppressive, anti-racist frameworks · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

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98. INT-0109 (rev 1) · Best answer: B

A client in a primary care clinic says, 'Drinking is the only thing that helps me sleep. But my liver tests are getting worse, and that scares me.' The client has not decided whether to cut back. Which response by the social worker is **MOST** appropriate?

A. For your liver's sake, it's time to find another way to sleep.
B. Drinking gets you to sleep, and your liver results worry you.
C. Let's set a date to cut back on drinking, and I can help.

Why each option is right or wrong

A **Incorrect** (trap: Too directive). Telling the client what to do is the righting reflex: well-meant advice that tends to draw out more arguments for drinking. Advice can come later, with permission, using elicit-provide-elicit.

B **Correct.** The client voices both a reason to keep drinking and a reason to change, which marks the contemplation stage (Prochaska & DiClemente, 1983). The stage-matched response explores ambivalence, and a double-sided reflection does this by acknowledging the sustain talk and pairing it with the client's own change talk (SAMHSA TIP 35, Ch. 3). Hearing both sides in their own words helps the client weigh them. It beats C because setting a start date is a preparation-stage task, and pushing a plan before the client decides tends to provoke more sustain talk.

C **Incorrect** (trap: Right action, wrong time). Setting a date fits a client who has decided to change, which is the preparation stage. This client is still undecided, so moving to planning skips the work of resolving ambivalence.

Takeaway: For a contemplative client, reflect both sides of the ambivalence; save advice and action plans for clients who have decided to change.

Sources: SAMHSA TIP 35 (2019), Ch. 3, Motivational Interviewing as a Counseling Style: double-sided reflections; Prochaska, J. O., & DiClemente, C. C. (1983). Stages and processes of self-change of smoking: Toward an integrative model of change. Journal of Consulting and Clinical Psychology, 51(3), 390-395, contemplation stage **Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.17 Intervening with addiction · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 2 of 3

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99. ASM-1126 (rev 2) · Best answer: D

PRETEST — unscored on this form; do not count it

A 27-year-old is admitted to an inpatient unit after 3 weeks of near-daily methamphetamine use. On admission, the patient believes neighbors are pumping gas through the vents and hears them plotting. With no drug use on the unit, these beliefs fade by day 5, and the patient returns to their usual self. Which diagnosis is **MOST** likely?

A. Brief psychotic disorder
B. Schizophreniform disorder
C. Psychotic disorder due to another medical condition
D. Stimulant-induced psychotic disorder

Why each option is right or wrong

A **Incorrect** (trap: Near-miss diagnosis). Brief psychotic disorder also lasts less than 1 month with a return to baseline, which makes it tempting. It cannot be diagnosed when the symptoms are attributable to a substance, as they are here.

B **Incorrect** (trap: Near-miss diagnosis). Schizophreniform disorder requires at least 1 month of symptoms such as delusions and hallucinations, and it also excludes substance effects. Five days of symptoms that ended with abstinence fall short on both counts.

C **Incorrect** (trap: Near-miss diagnosis). This diagnosis requires evidence that a medical illness, such as a seizure disorder or brain infection, directly causes the psychosis. Here the timing points to the drug: symptoms began during heavy use and cleared with abstinence.

D **Correct.** DSM-5-TR diagnoses a substance/medication-induced psychotic disorder, here a stimulant-induced one, when psychotic symptoms arise during or soon after use of a substance able to cause them and are not better explained by an independent psychotic disorder. The persecutory beliefs and voices began during a methamphetamine binge and cleared within days of abstinence. It beats A because brief psychotic disorder also lasts under a month but is excluded when a substance accounts for the symptoms. Psychosis persisting about a month after stopping would point toward an independent disorder (DSM-5-TR).

Takeaway: Psychosis that starts during stimulant use and clears soon after abstinence points to a substance-induced disorder, not a primary psychotic disorder.

Sources: DSM-5-TR (APA, 2022), Schizophrenia Spectrum and Other Psychotic Disorders: Substance/Medication-Induced Psychotic Disorder
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.9 DSM use in assessment; common mental and brain-related condition indicators · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 2 of 3

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100. ASM-1101 (rev 1) · Best answer: A

A 12-year-old with type 1 diabetes is admitted with diabetic ketoacidosis for the second time in 3 months. The parent missed 5 of 6 clinic visits despite transportation vouchers and home nurse visits, and insulin is supplied free. The parent can describe the insulin plan but says the child 'is old enough to handle it.' After meeting with the parent, what should the social worker do **NEXT**?

A. Report suspected medical neglect to child welfare authorities
B. Arrange another round of diabetes teaching for the parent and child
C. Coach the child to take over the daily insulin doses

Why each option is right or wrong

A **Correct.** Medical neglect is considered when a child is harmed by missed care, effective care is accessible, and the caregiver understands the advice but does not use it. Less intrusive steps come first, and a protective services report follows when they fail (Jenny & AAP, 2007). Vouchers and nurse visits have already been tried, the parent understands the plan, and the child has had two dangerous admissions. It beats B because more teaching repeats a support that has not worked while the danger grows. The report rests on reasonable suspicion, not proof.

B **Incorrect** (trap: Good but not best). Teaching is the right step when a family does not understand the care plan. This parent can describe the plan, and the family has already had home nurse visits, so another round of teaching delays protection after a second dangerous admission.

C **Incorrect** (trap: Ignores risk). Children build self-care skills gradually, but a 12-year-old still needs an adult to supervise insulin. Shifting full control to the child after two crises moves the parent's duty onto the child and raises the risk of another emergency.

Takeaway: When a child is harmed by missed care that the caregiver understands and can access, and supports have failed, report suspected medical neglect.

Sources: Jenny, C., & AAP Committee on Child Abuse and Neglect (2007). Recognizing and responding to medical neglect. Pediatrics, 120(6), 1385-1389, Diagnosis of medical neglect; responding to medical neglect
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.4 Physical, sexual, psychological abuse and neglect: signs, dynamics, impact · **Qualifier** NEXT · **Cognitive** reasoning · **Difficulty** 2 of 3

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101. ASM-1115 (rev 2) · Best answer: B

After a stroke, a patient with left-side weakness wants to go home alone. The patient correctly repeats the team's explanation that the weakness makes walking unsafe without help. The patient then says, 'That's true for other stroke patients, but my leg is fine,' yet needs two staff to stand. Which decision-making ability is **MOST** likely impaired?

A. Understanding
B. Appreciation
C. Reasoning

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Understanding means grasping and restating the relevant information. The patient repeats the team's explanation correctly, which shows this ability is intact.

B **Correct.** Capacity rests on four abilities: communicating a choice, understanding the information, appreciating how it applies to oneself, and reasoning about options (Appelbaum, 2007). The patient restates the risk accurately, so understanding is intact, but denies that it applies to their own leg despite needing two staff to stand. That gap is appreciation. It beats A because understanding is shown by restating the facts, which the patient does. Capacity is decision-specific, so this finding concerns the discharge decision, not every choice.

C **Incorrect** (trap: Wrong model or concept). Reasoning means comparing options and their consequences logically. The core problem comes earlier: the patient does not accept that the risk applies to them, so the reasoning starts from a false premise.

Takeaway: A person who can restate risks but denies they apply to themselves lacks appreciation, one of the four capacity abilities.

Sources: Appelbaum, P. S. (2007). Assessment of patients' competence to consent to treatment. New England Journal of Medicine, 357(18), 1834-1840, Legally relevant criteria for decision-making capacity
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.23 Client capacity, self-determination, self-monitoring (consent age, emancipation, permanency) · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 2 of 3

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102. ETH-1693 (rev 2) · Best answer: C

A client who resettled in the United States 8 months ago brings a small gift to every home visit. The client presses each gift into the social worker's hands at the door and seems anxious when the worker hesitates. The two have not yet talked about the gifts. What response is **MOST** helpful?

A. Explain that services continue with or without gifts
B. Accept each gift to honor the client's custom
C. Ask the client what the gifts are meant to express
D. Decline the gifts, citing the agency's no-gifts policy

Why each option is right or wrong

A **Incorrect** (trap: Skips assessment). This reassurance is exactly right if the client fears that benefits depend on gifts, a fear common among people who have dealt with officials who expected them. Offered before the client explains anything, it guesses at the motive and may confuse or embarrass a client who gives out of courtesy or gratitude.

B **Incorrect** (trap: Boundary slip or collusion). Small gifts with cultural meaning can sometimes be accepted. Accepting every gift without knowing why it is given, though, could reinforce an exchange the client feels obliged to keep up, especially if the client believes help depends on it.

C **Correct.** A gift's meaning shapes the right response: it may be courtesy, gratitude or a sign that the client fears services depend on pleasing the worker (Reamer, 2020). The client's anxiety at the worker's hesitation suggests something important rides on the gifts, but nothing yet shows what. Asking openly lets the client explain and treats the client as the expert on their own experience and culture (NASW Code 1.05(c)). It beats A because reassuring that services continue answers a fear the client has not voiced and may miss the real meaning.

D **Incorrect** (trap: Good but not best). Citing policy may be accurate, but it ignores the client's evident anxiety and may sound like a warning. Any limit on gifts lands better once the worker understands what the gifts mean to the client.

Takeaway: Before accepting or declining a client's repeated gifts, learn what they mean to the client; the meaning shapes the response.

Sources: NASW Code of Ethics (2021), 1.05(c); Reamer, F. G. (2020). Boundary Issues and Dual Relationships in the Human Services (3rd ed.). Columbia University Press, gifts and favors
Area I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.5 Power differentials, racial privilege, internalized inferiority/superiority · **Qualifier** MOST · **Cognitive** application · **Difficulty** 2 of 3

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103. ETH-1014 (rev 3) · Best answer: A

A state law limits disclosure of client information more than HIPAA does. Which rule generally applies?

A. The stricter state law
B. HIPAA, since federal law takes priority
C. Whichever law the state licensing board adopts

Why each option is right or wrong

A **Correct.** HIPAA sets a federal floor for privacy. A state law that is more stringent, meaning it gives individuals greater privacy protection or greater access to their own records, is not preempted and governs (45 CFR 160.203(b)). Social workers therefore follow whichever rule protects the client more. A beats B because federal law usually controls conflicts, but HIPAA itself expressly leaves stricter state privacy laws in place.

B **Incorrect** (trap: Misreads law or ethics). Federal law often overrides state law, which makes this tempting. HIPAA is different: it preempts state laws that are less protective and leaves more stringent ones in force.

C **Incorrect** (trap: Misreads law or ethics). Licensing boards enforce practice rules, but they do not decide whether HIPAA preempts a state law. HIPAA's own preemption rule settles it: a more stringent state privacy law stays in force and applies.

Takeaway: HIPAA is a floor: a more stringent state privacy law governs.

Sources: HIPAA Privacy Rule, 45 CFR 160.203(b)
Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.1 Confidentiality law and ethics, its limits, HIPAA/PHI privacy · **Qualifier** none · **Cognitive** recall · **Difficulty** 2 of 3

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104. ASM-1525 (rev 2) · Best answer: B

In the family life-cycle framework, what is the central emotional task for families in later life?

A. Accepting many exits from and entries into the family
B. Accepting the shift in generational roles
C. Committing to a newly formed family system
D. Loosening boundaries to allow children's independence

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). This is the key task of the launching stage, when grown children leave home and partners and grandchildren join. Later-life families may still see these changes, but their central task concerns how roles shift between generations as the oldest members age.

B **Correct.** In the family life-cycle framework, each stage has a key emotional process. For families in later life it is accepting the shift in generational roles: older adults keep up their own functioning and interests while the middle generation takes a more central role, and the family makes room for elders' experience without overfunctioning for them (Carter & McGoldrick, 2005). It beats A, the launching-stage task, because exits and entries describe children leaving and in-laws and grandchildren joining, not the reshuffling of roles as parents age.

C **Incorrect** (trap: Wrong model or concept). Commitment to a new system is the task of the new couple stage, when partners form their relationship and realign ties with their families of origin. It describes the start of a family, not its later years.

D **Incorrect** (trap: Wrong model or concept). Increasing the flexibility of boundaries belongs to families with adolescents, who need room to move in and out of the family. That stage also begins to account for grandparents' frailties, which is why it can tempt, but it is not the later-life task.

Takeaway: In later life, families accept shifting generational roles: elders keep their functioning while the middle generation takes on a more central role.

Sources: Carter, B., & McGoldrick, M. (Eds.) (2005). The Expanded Family Life Cycle: Individual, Family, and Social Perspectives (3rd ed., classic ed.). Allyn & Bacon, Ch. 1, Table 1.1 (p. 2): families in later life
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.18 Aging's impact on individuals and family systems · **Qualifier** none · **Cognitive** recall · **Difficulty** 2 of 3

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105. ETH-1689 (rev 1) · Best answer: C

A social worker notices that the clinic's intake coordinator routes every Spanish-speaking caller to case management only, noting 'not suited for therapy,' even when callers ask for counseling. The clinic has bilingual therapists with openings. The social worker raised the pattern privately with the coordinator, who replied, 'It's what works for those clients.' What should the social worker do **NEXT**?

A. Talk with the coordinator again about the routing pattern
B. Contact the affected callers to offer clinic therapy directly
C. Bring the routing pattern to the clinic's program director

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). Talking with the colleague is the right first step, and it has been taken. The coordinator dismissed the concern, so repeating it delays action while more callers are steered away from therapy.

B **Incorrect** (trap: Good but not best). Reaching out to affected callers may become part of the fix, but doing it alone bypasses intake and leaves the biased routing in place for future callers. The program director can correct the process and arrange outreach.

C **Correct.** When a colleague acts unethically, social workers first discuss it with the colleague when that is likely to help, then take action through appropriate formal channels (NASW Code 2.10(c)-(d)). Routing callers by language rather than request denies Spanish speakers the therapy others receive, a biased practice social workers should not condone (4.02). The private talk failed, so the next step is the program director, who oversees intake. It beats A because a second talk repeats a step the coordinator already rejected.

Takeaway: If a colleague's biased practice continues after a direct conversation, take it to the appropriate formal channel, such as the program director.

Sources: NASW Code of Ethics (2021), 2.10(c); NASW Code of Ethics (2021), 2.10(d); NASW Code of Ethics (2021), 4.02
Area I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.6 Privilege, implicit bias, microaggressions and their client impact · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 1 of 3

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106. ASM-1151 (rev 1) · Best answer: D

A client with schizophrenia has worked part-time stocking shelves for 4 months through a supported employment program. The supervisor reports that the client arrives early, chats easily with coworkers, and calls the job the best part of the week. However, the client cannot hold multistep instructions in mind and gets lost when a task changes midway. Which problem is **MOST** likely affecting job performance?

A. Negative symptoms
B. Disorganized thinking
C. Depressive symptoms
D. Cognitive impairment

Why each option is right or wrong

A **Incorrect** (trap: Near-miss diagnosis). Negative symptoms include reduced motivation, pleasure and expression, and they often hurt job performance in schizophrenia. This client is eager, socially engaged and finds the work rewarding, which points away from them.

B **Incorrect** (trap: Near-miss diagnosis). Disorganized thinking shows up in speech that derails or loses coherence. The client chats easily with coworkers; the trouble lies in holding and updating instructions, not in how thoughts connect in speech.

C **Incorrect** (trap: Near-miss diagnosis). Depression can slow concentration, but it comes with low mood or loss of interest. The client calls the job the best part of the week, which does not fit a depressive picture.

D **Correct.** Cognitive deficits, such as problems with working memory, attention and executive function, are common in schizophrenia and are closely tied to work and daily functioning, even when other symptoms are controlled (DSM-5-TR). Following multistep instructions and adjusting when a task shifts depend on working memory and mental flexibility. It beats A because negative symptoms appear as low drive, little pleasure or reduced speech, while this client arrives early, enjoys the job and talks easily with coworkers.

Takeaway: Working-memory and executive deficits in schizophrenia can impair work even when the client is motivated and other symptoms are controlled.

Sources: DSM-5-TR (APA, 2022), Schizophrenia Spectrum and Other Psychotic Disorders: Schizophrenia (Associated Features)
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.5 Signs of mental and emotional illness across the lifespan · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 2 of 3

107. ASM-1682 (rev 1) · Best answer: C

A school social worker receives a referral for a 14-year-old whom teachers describe as defiant because the student 'ignores directions.' With a parent's consent, the worker reviews school records. A third-grade evaluation identified a language disorder, and speech-language services stopped after a school transfer in sixth grade, with no reevaluation on file. Teacher notes say most problems occur with multistep spoken instructions. What is the **BEST** action?

A. Interview the student about trouble following directions
B. Start a school behavior plan rewarding prompt compliance
C. Ask the student's school team to revisit the earlier evaluation
D. Refer the student for a psychiatric evaluation for defiance

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Hearing the student's view is a reasonable part of any assessment and may add useful detail. On its own, it leaves the record's strongest lead, an untreated language disorder, unexamined by the team that can evaluate it.

B **Incorrect** (trap: Skips assessment). A reward plan assumes the problem is willful noncompliance. The records point instead to trouble processing spoken directions, which rewards cannot fix and which may leave the student punished for a disability.

C **Correct.** Integrating records means letting prior evaluations shape the current formulation. The file shows a diagnosed language disorder, services that ended without the reevaluation federal rules require before ending eligibility (34 CFR 300.305(e)), and current problems clustered on multistep spoken directions. Together these suggest a language-processing need rather than defiance. Asking the student's school team to revisit the earlier evaluation brings the question to the group responsible for revising services (34 CFR 300.324(b)). It beats A, the strongest competitor, because interviewing the student is useful but cannot settle a language-processing question the team must evaluate.

D **Incorrect** (trap: Hands off too soon). A psychiatric referral for defiance accepts the teachers' label before the documented language disorder is reconsidered. It would fit if a review found no language explanation and serious behavior concerns remained.

Takeaway: Integrate prior evaluations into the current picture; a documented disability in the records can reframe a behavior referral and should prompt team review.

Sources: IDEA Part B regulations, 34 CFR Part 300, 34 CFR 300.305(e)(1); IDEA Part B regulations, 34 CFR Part 300, 34 CFR 300.324(b)(1)
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.11 Integrating records and evaluations (medical, school, psychological, employment) · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 3 of 3

108. INT-1157 (rev 2) · Best answer: C

A social worker at a family services agency is helping separated co-parents settle a dispute over their child's weekday schedule. In the second joint session, one parent agrees to each proposal within seconds whenever the other parent's voice rises and keeps glancing at the door. After the session, that parent asks whether anything said privately would be shared with the other parent. What should the social worker do **FIRST**?

A. Ask both parents in the next joint session whether either feels unsafe
B. Switch to separate-room sessions with each parent from now on
C. Meet alone with that parent to screen for abuse and fear

Why each option is right or wrong

A **Incorrect** (trap: Ignores risk). Asking about safety is the right instinct, but not with the other parent in the room. A frightened parent is unlikely to answer honestly in front of the person they fear, and the question itself may raise the risk. Abuse screening is done privately.

B **Incorrect** (trap: Right action, wrong time). Separate sessions are a recognized safeguard when abuse is present, and they may well follow. Chosen before screening, they could leave a parent in danger and miss that the process should be suspended entirely.

C **Correct.** Family mediation standards call for screening for domestic abuse, including control and intimidation, and continuing to assess throughout the process (AFCC Model Standards, Standard X). Instant agreement whenever the other parent's voice rises, glances at the door, and a question about privacy suggest fear that could make any agreement unsafe and not truly voluntary. A private screening tells the social worker whether to adapt the process or stop it. It beats B because separate sessions are one possible safeguard, but choosing a safeguard before learning what is happening may leave danger unaddressed.

Takeaway: Signs of fear or intimidation in mediation call for private abuse screening before deciding how, or whether, joint problem solving continues.

Sources: AFCC, ABA & ACR (2000). Model Standards of Practice for Family and Divorce Mediation, Standard X: domestic abuse
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.19 Resolving conflict · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 3 of 3

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109. ASM-1507 (rev 1) · Best answer: B

In a court-ordered program, a client says they follow every rule, even ones they think are unfair, because 'if everyone picked which laws to obey, society would fall apart.' The client adds that changing a bad law is up to lawmakers, not individuals. In Kohlberg's theory, this reasoning **MOST** likely reflects which level?

A. Preconventional
B. Conventional
C. Postconventional

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Preconventional reasoning centers on avoiding punishment or gaining rewards. The client is not focused on personal consequences; the concern is keeping society orderly, which marks a later level.

B **Correct.** Kohlberg's conventional level judges right and wrong by social approval and by upholding laws and social order; stage 4 is the law-and-order orientation (Kohlberg, 1981). This client obeys even unfair rules because order would collapse otherwise and leaves change to lawmakers, the stage 4 view. It beats C because postconventional reasoning treats laws as social contracts that can be challenged when they violate rights or universal principles, a view this client rejects. Knowing the level can help the social worker frame program expectations.

C **Incorrect** (trap: Wrong model or concept). Postconventional reasoning sees laws as agreements that serve rights and justice and supports changing or resisting unjust laws on principle. The client's insistence on obeying unfair rules for the sake of order fits the conventional level instead.

Takeaway: Kohlberg's conventional level upholds rules and social order for their own sake; postconventional reasoning weighs laws against rights and principles.

Sources: Kohlberg, L. (1981). Essays on Moral Development, Vol. 1: The Philosophy of Moral Development. Harper & Row, stages of moral development: stage 4

Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.16 Typical vs. atypical development across the lifespan · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 2 of 3

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110. ASM-1636 (rev 2) · Best answer: B

A 20-year-old college student lost a soccer scholarship after a knee injury ended the student's playing career. Within a semester, the student joined the debate team and now prepares 30 hours a week. In counseling, the student says, 'I'm not the strong one anymore, so I have to be the smartest one in every room.' Which defense mechanism does this **MOST** likely reflect?

A. Sublimation

B. Compensation

C. Rationalization

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Sublimation redirects an unacceptable impulse, such as aggression or sexual energy, into a valued outlet, like turning rage into competitive boxing. The student's drive comes from feeling diminished after the injury, not from an impulse that needs a safer outlet.

B **Correct.** Compensation is making up for a real or perceived weakness by striving to excel in another area, an idea rooted in Adler's work on feelings of inferiority. The student's own words tie the intense debate effort to no longer being 'the strong one,' which shows an effort to offset a felt deficit. It beats A because sublimation channels an unacceptable impulse, such as aggression, into a socially valued activity; nothing here suggests a forbidden impulse being redirected, only a loss being made up for (Adler, 1927).

C **Incorrect** (trap: Wrong model or concept). Rationalization offers a plausible excuse for behavior or a disappointment, such as claiming soccer never mattered. The student openly names the loss and its effect rather than explaining it away.

Takeaway: Compensation offsets a felt weakness by excelling elsewhere; sublimation channels an unacceptable impulse into a socially valued activity.

Sources: Adler, A. (1927). Understanding Human Nature. Greenberg, feelings of inferiority and compensation
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.11 Defense mechanisms and how they shape behavior and relationships · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 2 of 3

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111. INT-1152 (rev 1) · Best answer: A

In a hospital support group for adults newly diagnosed with cancer, a member in the second week of chemotherapy listens as another member describes finishing treatment 5 years ago and returning to work. Afterward the newer member says, 'If they got through it, maybe I can too.' Which therapeutic factor is **MOST** likely at work for the newer member?

A. Instillation of hope
B. Universality
C. Imitative behavior
D. Imparting information

Why each option is right or wrong

A **Correct.** Yalom describes instillation of hope as the encouragement members gain from seeing others who have coped with, or improved from, the same problem (Yalom & Leszcz, 2020). The newer member's words look forward: a peer's survival and return to work make recovery seem possible. It beats B, universality, because universality is the relief of learning that others share one's problem or feelings. Here the member draws belief from another person's progress, not from shared suffering.

B **Incorrect** (trap: Wrong model or concept). Universality would fit if the member said they felt less alone or less abnormal on hearing others describe the same fears. The statement here is about a possible future, which marks hope.

C **Incorrect** (trap: Wrong model or concept). Imitative behavior means taking on another member's or the leader's way of acting or coping. The newer member expresses a belief about the future, not an adopted behavior.

D **Incorrect** (trap: Wrong model or concept). Imparting information covers teaching or advice about an illness or about coping. The survivor shared a personal story, and the newer member's response centers on possibility rather than new facts.

Takeaway: Instillation of hope comes from seeing peers who are further along improve; universality comes from discovering that others share the same problem.

Sources: Yalom & Leszcz (2020). The Theory and Practice of Group Psychotherapy (6th ed.). Basic Books, Ch. 1, The therapeutic factors: instillation of hope; universality
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.23 Group work: managing process and building cohesion · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 2 of 3

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112. ETH-1711 (rev 2) · Best answer: C

A 26-year-old gay Muslim client tells a social worker that the local LGBTQ center's events, often held in bars, feel foreign to their faith, while sermons at their mosque make them hide their sexuality. The client says clearly that they want to keep both their faith and their relationships, and asks where they might find belonging. The social worker had suggested the center's support group. What should the social worker do **NEXT**?

A. Urge the client to give the LGBTQ group a few more tries
B. Explore whether the client's faith reflects internalized homophobia
C. Help the client find LGBTQ groups within their faith, including online

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). The suggestion was reasonable at first, but the client has explained why that space feels foreign to their faith. Pressing it again asks the client to set aside one identity to fit a service.

B **Incorrect** (trap: Too directive). Exploring internalized stigma can help a client who rejects their own sexuality. This client wants both faith and relationships, so treating religious commitment as a possible symptom misreads the problem and can harm trust.

C **Correct.** Intersectionality holds that identities combine to create distinct experiences that cannot be served one identity at a time (Crenshaw, 1989; NASW Code 1.05(d)). The client wants to keep both faith and relationships and has explained why each single-identity space falls short. Linking the client with LGBTQ Muslim communities, including online networks, meets the stated goal of belonging as a whole person. It beats B because the client has described a mismatch of settings, not self-rejection, and probing for internalized homophobia would cast the client's faith as a symptom.

Takeaway: Serve clients with intersecting identities as whole people; when single-identity services fall short, seek communities that honor those identities together.

Sources: Crenshaw, K. (1989). Demarginalizing the intersection of race and sex: A Black feminist critique of antidiscrimination doctrine, feminist theory and antiracist politics. University of Chicago Legal Forum, 1989(1), 139-167, whole article; NASW Code of Ethics (2021), 1.05(d)
Area I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.3 Culture, race, gender, sexuality, intersectionality shaping identity and behavior · **Qualifier** NEXT · **Cognitive** reasoning · **Difficulty** 2 of 3

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113. ETH-1537 (rev 2) · Best answer: D

A hospital oncology social worker meets for the first time with a client newly diagnosed with early-stage colon cancer. The oncologist has twice explained that surgery offers a strong chance of cure, and the client can restate the risks accurately. The client wants to try herbal treatment from a trusted practitioner for 3 months first. The oncologist asks the social worker to help. What should the social worker do **FIRST**?

A. Request a capacity evaluation before honoring the choice
B. Respect the choice and keep follow-up care open
C. Ask the client's family to encourage the surgery
D. Ask about the client's reasons and concerns

Why each option is right or wrong

A **Incorrect** (trap: Skips assessment). Capacity should be assessed when confusion, psychosis or poor reasoning raises doubt. Ordering an evaluation only because the client disagrees with the physician treats an unwelcome choice as a symptom and can damage trust.

B **Incorrect** (trap: Right action, wrong time). Supporting a competent client's informed refusal while keeping follow-up open is right once the social worker understands the decision; competence is presumed, and patients may make choices others consider unreasonable (Appelbaum, 2007). Moving straight to it at a first meeting skips learning what lies behind the choice, which may include fears the client would want addressed.

C **Incorrect** (trap: Overrides the client's choice). Recruiting family to push surgery pressures a competent adult and may share health information without consent. Family can be involved if the client wants that, but not to override the decision.

D **Correct.** A competent adult may decline recommended treatment (NASW Code 1.02), and the social worker's first task is to understand the decision. The client already knows the medical facts, so the open questions are what draws the client to the herbal plan and what the client fears about surgery; the answers may reveal worries that can be addressed or values that should shape the plan (Miller & Rollnick, 2023). It beats B because respecting the choice may be where this ends, but doing so before hearing the client's reasons skips the conversation the decision deserves.

Takeaway: When a competent client declines recommended treatment, first explore the reasons and fears behind the choice, then support the informed decision.

Sources: NASW Code of Ethics (2021), 1.02; Appelbaum, P. S. (2007). Assessment of patients' competence to consent to treatment. New England Journal of Medicine, 357(18), 1834-1840, presumption of competence; right to unreasonable choices; Miller & Rollnick (2023). Motivational Interviewing (4th ed.). Guilford Press, engaging: understanding the client's perspective before planning

Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.1 Protecting and promoting client self-determination, including right to refuse · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 3 of 3

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114. ASM-1162 (rev 1) · Best answer: B

Long-term antipsychotic use can lead to involuntary lip smacking, chewing motions, and tongue movements. Which movement disorder does this describe?

A. Acute dystonia
B. Tardive dyskinesia
C. Akathisia
D. Parkinsonism

Why each option is right or wrong

A **Incorrect** (trap: Near-miss diagnosis). Acute dystonia means sudden, sustained muscle spasms, such as a twisted neck or eyes rolled upward. It usually appears within days of starting an antipsychotic, not after years.

B **Correct.** Tardive dyskinesia is a late-appearing movement disorder linked to long-term use of antipsychotics and other dopamine-blocking drugs. It causes involuntary movements, often of the mouth, lips and tongue, and may persist after the drug is stopped (DSM-5-TR). A social worker who notices these signs alerts the prescriber. It

beats D because parkinsonism brings tremor, stiffness and slowed movement, usually within weeks of starting or raising a dose, rather than repetitive mouth movements after years.

C **Incorrect** (trap: Near-miss diagnosis). Akathisia is an inner restlessness with an urge to keep moving, such as pacing or shifting in a chair. It typically begins within weeks of starting or raising a dose.

D **Incorrect** (trap: Near-miss diagnosis). Drug-induced parkinsonism causes tremor, rigidity, slowed movement and a mask-like face. It tends to develop within weeks of treatment and is not marked by repetitive mouth and tongue movements.

Takeaway: Repetitive involuntary mouth and tongue movements after long-term antipsychotic use suggest tardive dyskinesia; report them to the prescriber.

Sources: DSM-5-TR (APA, 2022), Medication-Induced Movement Disorders and Other Adverse Effects of Medication: Tardive Dyskinesia

Area II Assessment and Planning · **Sub-area** II.C Assessment Practices · **Topic** II.C.5 Prescription, OTC, alternative medications: uses and side effects · **Qualifier** none · **Cognitive** recall · **Difficulty** 1 of 3

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115. ETH-1533 (rev 2) · Best answer: A

PRETEST — unscored on this form; do not count it

A child protective services investigation closed with no findings, and the agency offered the family voluntary in-home parenting support. At the first home visit, before the social worker can describe the program, the parent says sharply, 'We were cleared. We don't need anybody else checking up on us.' What should the social worker do **FIRST**?

A. Acknowledge the frustration behind the comment
B. Notify the investigator that the family refused services
C. Accept the refusal and explain how to return later
D. Explain that the service is voluntary and what it offers

Why each option is right or wrong

A **Correct.** A parent just cleared by an investigation may meet any follow-up visit with anger and fear of continued surveillance. Acknowledging that frustration first lowers defensiveness and makes room to explain that the service is voluntary, so whatever the parent decides is informed and freely chosen (NASW Code 1.02; Hepworth et al.). It beats D because explaining the program is the right next step, but launching into it over the parent's anger can sound like the agency is pressing on regardless.

B **Incorrect** (trap: Misreads law or ethics). Refusing voluntary services is not a new safety concern and is not reportable on its own. Telling the investigator implies a suspicion the closed investigation did not support; a report would be required only if new signs of abuse or neglect emerged.

C **Incorrect** (trap: Right action, wrong time). If the parent declines after learning that the service is voluntary, the social worker accepts the refusal and explains how to return (NASW Code 1.02, 1.03(a)). Treating an angry first remark as a final, informed refusal skips the chance to clarify what is being offered.

D **Incorrect** (trap: Right action, wrong time). Explaining that the service is voluntary and what it offers is the right next step, and it makes any refusal an informed one.

Delivered over the parent's anger, before the feeling is acknowledged, it can sound like a sales pitch from the agency that just investigated the family.

Takeaway: When a reluctant client opens with anger, acknowledge the feeling first, then clarify choices so any decision is informed and free.

Sources: NASW Code of Ethics (2021), 1.02; NASW Code of Ethics (2021), 1.03(a); Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, engaging involuntary and reluctant clients
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.1 Protecting and promoting client self-determination, including right to refuse · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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116. INT-1627 (rev 2) · Best answer: A

At the second session in a community clinic, a client with a history of brief, intense relationships says the social worker is 'the first one who really gets me.' The client describes two former workers as cold and useless. What is the **MOST** appropriate response?

A. Thank the client and invite feedback when the worker gets it wrong
B. Accept the praise warmly and move on to the client's goals
C. Point out that the client may be idealizing the worker

Why each option is right or wrong

A **Correct.** Early idealization, especially with a history of intense relationships that end badly, often swings to disappointment. Safran and Muran (2000) describe building an alliance in which ruptures can be named and worked through; inviting feedback now turns a future letdown into something to discuss rather than a reason to leave. Accepting the thanks keeps the bond warm in session two. It beats C because confronting idealization before trust exists is likely to feel like rejection and repeat the pattern the client fears; confrontation fits after rapport is established (Hepworth et al.).

B **Incorrect** (trap: Good but not best). Accepting the thanks keeps the bond warm in session two, and returning to goals is reasonable. Moving on without preparing for future disappointment, though, misses the chance to make ruptures discussable, which matters most for a client whose relationships tend to swing from ideal to useless and end abruptly.

C **Incorrect** (trap: Too directive). Naming idealization can be useful later, once the alliance can hold it. In the second session it is likely to land as criticism and may push away a client whose relationships tend to end abruptly.

Takeaway: Early idealization often precedes disappointment; build an alliance where future ruptures can be voiced instead of colluding with or confronting it.

Sources: Safran, J. D., & Muran, J. C. (2000). Negotiating the Therapeutic Alliance: A Relational Treatment Guide. Guilford Press, Ruptures and metacommunication; Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, Confrontation: timing after rapport
Area III Intervention and Practice · **Sub-area** III.A Practice Concepts · **Topic** III.A.2 Building and sustaining the helping relationship (empathy, rapport, acceptance) · **Qualifier** MOST APPROPRIATE · **Cognitive** reasoning · **Difficulty** 3 of 3

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117. ASM-0114 (rev 1) · Best answer: A

PRETEST — unscored on this form; do not count it

Which mental status exam term describes laughing while recounting a sad event, such as a recent death?

A. Inappropriate affect
B. Labile affect
C. Restricted range of affect
D. Euphoric mood

Why each option is right or wrong

A **Correct.** Affect is the observed emotional expression, and it is called inappropriate when it clashes with the content being discussed, such as laughing while describing a death (Kaplan & Sadock's Synopsis). The finding calls for further assessment, with attention to context and cultural norms, because it can occur in psychotic and other disorders. It beats B because labile affect refers to rapid, abrupt shifts in emotional expression, not a steady mismatch between expression and content.

B **Incorrect** (trap: Wrong model or concept). Labile affect means rapid, abrupt swings in emotional expression, such as moving from tears to laughter within moments. A single expression that conflicts with the topic is better described as inappropriate.

C **Incorrect** (trap: Wrong model or concept). A restricted range of affect means emotional expression varies less than expected across topics. Laughter about a sad event is a mismatch between expression and content, not a narrowing of range.

D **Incorrect** (trap: Wrong model or concept). Mood is the client's own report of their emotional state, while affect is what the interviewer observes. Laughter seen during the interview is recorded under affect, and nothing indicates a euphoric mood.

Takeaway: Affect is observed expression: inappropriate affect clashes with content, labile affect shifts rapidly. Mood is the client's self-report.

Sources: Boland, R., Verduin, M. L., & Ruiz, P. (Eds.). Kaplan & Sadock's Synopsis of Psychiatry (12th ed.). Wolters Kluwer, mental status examination: affect
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.10 Mental status exam: components and purpose · **Qualifier** none · **Cognitive** recall · **Difficulty** 2 of 3

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118. ETH-1645 (rev 2) · Best answer: D

A social worker at a housing program learns that a landlord refused to show a vacant apartment to a client after hearing the client has three children, saying the unit 'isn't right for kids.' The client is upset that other families may be turned away too, but does not want to file a complaint, saying the housing search is exhausting enough. Which response is **BEST**?

A. Respect the decision and focus on the housing search
B. File a fair housing complaint on the client's behalf
C. Encourage the client to reconsider filing a complaint
D. With permission, report the practice to a fair housing group

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Supporting the client's search and choice is necessary, and it may be all the client wants at first. Stopping there ignores the client's stated worry about other families and misses a way to address the landlord's practice without burdening the client.

B **Incorrect** (trap: Overrides the client's choice). Filing without consent overrides a competent client's decision and discloses private information. A complaint can be powerful when the client chooses it; here the client has declined.

C **Incorrect** (trap: Too directive). Giving information about filing is fine, but urging the client after a clear, reasoned refusal adds pressure during a stressful search. The client can revisit the choice later.

D **Correct.** Refusing to show housing because a family has children is familial status discrimination under the Fair Housing Act (42 U.S.C. 3604(a)). The client's choice not to file is respected (NASW Code 1.02), and the Code also calls for social action so that all people have equal access to the resources they need (NASW Code 6.04(a)). Sharing the landlord's practice with a fair housing organization, with the client's permission, can prompt testing or outreach without making the client a complainant. It beats A because focusing on the search alone honors autonomy but leaves a discriminatory practice in place for the next family, the very concern the client raised.

Takeaway: When a client declines to act on discrimination, respect the choice and, with permission, use channels that address the practice without burdening the client.

Sources: Fair Housing Act, 42 U.S.C. 3604(a); NASW Code of Ethics (2021), 1.02; NASW Code of Ethics (2021), 6.04(a)
Area I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.2 Principles of social, racial, environmental, economic justice · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 3 of 3

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119. INT-1686 (rev 1) · Best answer: C

A child welfare associate is completing a self-assessment before a 6-month evaluation. The associate feels confident about court reports, but the supervisor returned three of the last five reports for missing safety details. The agency form asks for a rating and an example for each skill. Which approach to the self-assessment is **MOST** useful?

A. Rate court reports a strength, based on the associate's confidence
B. Give every skill a middle rating to avoid seeming overconfident
C. Rate court reports a growth area, noting the specific feedback

Why each option is right or wrong

A **Incorrect** (trap: Misuses the technique). Confidence matters, but self-ratings that ignore concrete feedback signal a gap in self-awareness. Most recent reports came back for revision, so calling them a strength would steer supervision away from a real need.

B **Incorrect** (trap: Misuses the technique). Uniform middle ratings look modest but give the supervisor no usable information about strengths or needs. Self-assessment should distinguish among skills using evidence.

C **Correct.** Self-assessment is part of the supervisee's role, and it is useful only when grounded in evidence rather than feelings. Three of five reports came back for missing

safety details, so an honest rating names this a growth area and ties it to the feedback, which lets supervision target it (Kadushin & Harkness, 2014; NASW & ASWB, 2013). It beats A because confidence is not evidence; rating the skill a strength despite returned reports suggests limited insight and delays help with a safety-related task.

Takeaway: Base self-assessment for supervision on concrete feedback and work samples, not on confidence, so learning goals target real needs.

Sources: Kadushin & Harkness (2014). Supervision in Social Work (5th ed.). Columbia University Press, Ch. 5: Principles and Problems in Implementing Educational Supervision; NASW & ASWB Best Practice Standards in Social Work Supervision (2013), Overview of Supervision (shared responsibility)
Area III Intervention and Practice · **Sub-area** III.D Supervision and Administration · **Topic** III.D.1 Supervisee's role: learning needs, self-assessment, prioritizing · **Qualifier** MOST · **Cognitive** application · **Difficulty** 1 of 3

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120. ETH-1504 (rev 1) · Best answer: B

At a neighborhood street fair, a social worker runs into a former client whose services ended 3 years ago. After a friendly conversation, the former client asks the social worker out on a date. How should the social worker respond?

A. Accept, since over 2 years have passed
B. Decline, citing the lasting professional duty
C. Offer to meet as friends instead of dating

Why each option is right or wrong

A **Incorrect** (trap: Misreads law or ethics). The 2-year idea comes from some state laws and other professions, not from the NASW Code, which sets no waiting period. The national standard for social workers is to avoid romantic or sexual relationships with former clients.

B **Correct.** The NASW Code directs social workers to avoid sexual activity or contact with former clients and sets no time limit; a worker who claims an exception bears the full burden of showing no exploitation (NASW Code 1.09(c)). The power and personal knowledge gained in the helping relationship persist after services end, so a clear, kind decline is correct. It beats C because a friendship is also a dual relationship with a former client, carrying the same risk of exploitation (NASW Code 1.06(c)).

C **Incorrect** (trap: Boundary slip or collusion). Offering friendship sounds like a gentle middle ground, but it creates a new dual relationship with a former client, which the Code cautions against when there is a risk of exploitation or harm.

Takeaway: The NASW Code sets no time limit after which a romantic relationship with a former client becomes acceptable.

Sources: NASW Code of Ethics (2021), 1.09(c); NASW Code of Ethics (2021), 1.06(c)
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.2 Boundaries: power, conflicts of interest, self-disclosure, dual relationships · **Qualifier** none · **Cognitive** application · **Difficulty** 1 of 3

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121. ASM-1038 (rev 1) · Best answer: C

On an inpatient unit, a 24-year-old patient with autism spectrum disorder diagnosed in early childhood is admitted after a family conflict. The team notes flat facial expression, few words, rigid routines and little interest in peers, and a resident suggests adding schizophrenia. The patient's parent says these traits have always been present. What is the **MOST** appropriate input from the social worker?

A. Support adding schizophrenia based on the negative symptoms
B. Gather a detailed developmental history from the parent
C. Explain that prominent delusions or hallucinations are also required

Why each option is right or wrong

A **Incorrect** (trap: Near-miss diagnosis). Negative-like features such as flat expression and few words are part of this patient's lifelong autism. Without prominent delusions or hallucinations for a month, DSM-5-TR does not support adding schizophrenia.

B **Incorrect** (trap: Right action, wrong time). Developmental history is important when autism is uncertain. Here the diagnosis dates to early childhood and the parent confirms the traits are lifelong, so more history would not answer the team's question.

C **Correct.** With a history of autism spectrum disorder, DSM-5-TR allows an added schizophrenia diagnosis only when prominent delusions or hallucinations are present for at least a month, along with the other required symptoms. Flat expression, few words, rigid routines and limited interest in peers are lifelong autism features here and overlap with negative symptoms. It beats B because the parent has already described these traits as lifelong; the decisive question for the team is whether prominent delusions or hallucinations are present.

Takeaway: In autism, add schizophrenia only if prominent delusions or hallucinations last a month; lifelong flat affect and few words don't count.

Sources: DSM-5-TR (APA, 2022), Schizophrenia Spectrum and Other Psychotic Disorders: Schizophrenia
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.9 DSM use in assessment; common mental and brain-related condition indicators · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 3 of 3

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122. ETH-1029 (rev 1) · Best answer: B

A couple receives services at a university training clinic. At intake, both partners consented in writing to a faculty team observing sessions through a one-way mirror. Midway through the fifth session, one partner says they no longer want the team watching. What should the social worker do **FIRST**?

A. Continue, since both partners consented in writing at intake
B. Stop the team's observation from this point on
C. Ask the team lead to explain why observation helps
D. Explore the partner's concerns before making any change

Why each option is right or wrong

A **Incorrect** (trap: Misreads law or ethics). Signed consent at intake does not bind a client for the rest of treatment. Consent is ongoing and can be withdrawn, and continuing after a clear withdrawal ignores it.

B **Correct.** Third-party observation requires informed consent (NASW Code 1.03(h)), and consent includes the right to withdraw it at any time (NASW Code 1.03(a)). The partner's statement withdraws consent now, so the observation stops first. B beats D because exploring the concern is useful, but doing it while the team keeps watching continues observation the client has just refused. Afterward, the worker can discuss what prompted the change and how the couple wants to proceed.

C **Incorrect** (trap: Overrides the client's choice). Having the team argue for observation pressures a client who has already said no. Information about benefits belongs in the original consent discussion, not in an effort to reverse a withdrawal.

D **Incorrect** (trap: Right action, wrong time). Understanding the partner's concerns is a good next step and may lead to a new agreement. Doing it before stopping the observation lets the team keep watching after consent was withdrawn.

Takeaway: Consent to observation or recording can be withdrawn at any time; stop when a client withdraws, then discuss the concern.

Sources: NASW Code of Ethics (2021), 1.03(h); NASW Code of Ethics (2021), 1.03(a)
Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.4 Informed consent: principles and how it is obtained · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 3 of 3